

काय चिकित्सा 2 (B)

गलगण्ड	मूत्राघात	Syphilis
गण्डमाला	मूत्रकृच्छ्र	पूयमेह
अपची	अश्मरी	Gonorrhoea
अर्बुद	Cystitis	मनस
प्रमेह	Nephritis	मनोभाव
Diabetes mellitus	Nephrotic syndrome	उन्माद
स्थौल्य	Renal failure	अपस्मार
काश्यं	BPH	अतत्त्वाभिनिवेश
Dyslipidaemia	अतिसार	Anxiety disorders
अस्थिमज्जा विद्रधि	प्रवाहिका	Stress induced disorders
Osteomyelitis	अर्श	
अस्थिसौशीर्यं	पुरीषज कृमि	Depression
Osteoporosis	IBS	Somatoform and mood disorders
अस्थिक्षय	Ulcerative colitis	
Osteopenia	उपदंश	Psychosexual disorders
संधिगत वात	Chancroid	
Osteoarthritis	Lymphogranuloma	भूत विद्या
क्लैब्य	inguinale	रसायन
शुक्रदोष	फिरंग	वाजीकरण

Diseases of मांस & मेदोवह स्रोतस

गलगण्ड (Goitre)

निरुक्ति →

गलस्य पार्श्वे गलगण्ड एकः। (च.चि.12/79)

Single swelling in the neck-region is called galaganda.

निदान & सम्प्राप्ति →

Due to मिथ्या आहार-विहार (unwholesome diet & improper activities) the vitiated dosha & dushya- वात, कफ & मेद in neck region (outside the throat) produce a swelling which grows in course of time. It hangs like the scrotum and causes severe pain.

भेदानुसार लक्षण →

1. वातज गलगण्ड → तोदयुक्त गण्ड – कृष्णराजिमान – कृष्ण अरुण वर्ण – तालु गल शोष
2. कफज गलगण्ड → स्थिर गण्ड – शीतस्पर्शी – महान – त्वकसमानवर्ण – अल्पवेदना – अतिकण्डू – चिराभिवृद्धि – चिरपाक
3. मेदोज गलगण्ड → वेदनारहित – अतिकण्डूयुक्त – पाण्डुवर्ण – अल्पमूल – कफज लक्षण – देहानुरूप क्षयवृद्धि युक्त

चिकित्सा →

1. वातज गलगण्ड चिकित्सा →

- नाडीस्वेद with एरण्डादि वातहर औषध
- रक्तमोक्षण (blood letting)
- उपनाह with तिल, अतसी, मूलक, शिगू, किण्व, प्रियाल, अर्क, गजपिप्पली, कांजी etc.

2. कफज गलगण्ड चिकित्सा →

- स्वेदन
- विम्लापन
- लेप with अजगंधा, अतिविषा, कुष्ठ, गुंजा etc.
- पिप्पली आदि कफहर द्रव्यसिद्ध पंचलवणयुक्त तैलपान

3. मेदोज गलगण्ड चिकित्सा →

- सिराव्यध (रक्तमोक्षण)
- कफघ्न चिकित्सा
- गोमूत्र पान with असनादि चूर्ण

गण्डमाला (Lymphadenitis)

निरुक्ति →

गलस्य पार्श्वे गलगण्ड एकः, स्याद् गण्डमाला बहुभिश्च गण्डैः। (च.चि.12/79)

Multiple swellings in the neck-region are called gandamala.

निदान & सम्प्राप्ति →

The aggravated doshas- वात, पित्त & कफ along with vitiated मेद get स्थानसंश्रय in कण्ठ (throat), मन्या (neck), अक्ष (clavicular-region), अंस (scapular-region), कक्षा (axilla) & वंक्षण-प्रदेश (inguinal region) and produce multiple swellings → त्वकसमान वर्ण, कठिन, स्निग्ध, वार्ताक – आमलक फल समान आकृति, गम्भीर, अवगाढ, चिरपाकी गण्ड.

भेदानुसार लक्षण →

1. वातज गण्डमाला → रूक्ष, वेदनायुक्त
2. पित्तज गण्डमाला → दाह, क्षिप्रपाक
3. कफज गण्डमाला → कठिन, गूढ, स्निग्ध, अल्पवेदना, मेदोधिक्य

चिकित्सा →

- सिराव्यध (रक्तमोक्षण)
- विरेचन कर्म
- नस्य कर्म
- औषध प्रयोग → कांचनार गुग्गुलु – चक्रमर्द तैल – गुंजा तैल – गुंजाद्य तैल

अपची

चयप्रकर्षाद् अपचीमिति निरुक्तिः । (डल्हण)

चय प्रकर्ष (accumulation of granthis) → swelling is more.

These granthis are आमलकास्थि or मत्स्याण्डाकार in shape and size.

अर्बुद (Tumour)**निरुक्ति →**

अर्बुदोऽयं ग्रथितो महान् भवति ।

When granthi becomes too large, it is called arbuda.

The new and abnormal growth, or swelling, resembling a knot, is called granthi. The aggravated dosha invades मेद, मांस and रक्त धातु to produce a round, raised swelling (new growth) which resembles a knot is ग्रन्थि, and when a granthi is too large (महान्) it is known as अर्बुद.

सामान्य लक्षण →

गात्रप्रदेशे क्वचिदेव दोषाः संमूर्च्छिता मांसमभिप्रदूष्य ।

वृत्तं स्थिरं मन्दरुजं महान्तमनल्पमूलं चिरवृद्धयपाकम् ॥

कुर्वन्ति मांसोपचयं तु शोफं तमर्बुदं शस्त्रविदो वदन्ति ॥ (सु.नि.11/13)

Aggravated doshas causing vitiation of the dhatus like mamsa etc. produce mamsa-shophya and the following symptoms (लक्षण) anywhere in the body →

Rabin Singh

- वृत्तम् – Round shape.
- स्थिरम् – Fixed to the surrounding tissue.
- मन्दरुजम् – Less painful in the initial stage or pain is conspicuously absent in initial stage.
- महान्तम् – Very large in size or rapid growth.
- अनल्पमूलम् – Deep rooted due to infiltration to the surrounding tissues.
- चिरवृद्धि – Chronic in nature (longer pathogenesis to manifest the disease).
- अपाकम् – Fewer tendencies towards suppuration (ulceration).

अर्बुद भेद → 6

1. वातार्बुद → In Vatarbuda, there are different types of pain such as stretching, pricking, cutting and tearing in nature. The swelling is black in colour, rough in nature and enlarged like the bladder. It secretes clear bloody discharge if bursted.
2. पित्तार्बुद → Pittarbuda produces severe burning sensations along with symptoms like fuming, sucking, throbbing, churning in nature etc. And red or yellow coloured discharge or warm blood if it is bursted or opened.
3. कफार्बुद → It is a stone like cold swelling same as that of body colour. It is associated with mild pain and excessive itching. It develops slowly, and discharges white thick pus like material, if burst.
4. रक्तार्बुद → Due to mithya-ahara-vihara, vatadi doshas get aggravated and vitiate the rakta that results in contraction of the blood vessels and siras and develops a non-suppurating bleeding mass.

This mass is projected with fleshy buds, which increases in size rapidly and bleeds excessively. This raktribuda is incurable, Panduroga and its associated complications may occur due to severe blood loss. Raktribuda is correlated with malignant tumour.

5. मांसार्बुद → Due to fist-blows and other repeated traumas on mamsa, affected part gets vitiated and shophya is developed at the site. It presents as painless, unctuous, same colour of the body, stony hard in nature and fixed, non-suppurating mass. It is incurable.
6. मेदोर्बुद → Medoarbuda is smooth, big in size with mild pain and excessive itching. It increases or decreases according to Sharira-vridhi or kshaya respectively. If bursts, it secretes oil cake or ghee like material.

अध्यर्बुद → When a second arbuda develops over an already existing one, it is called Adhyarbuda. It can be considered as recurrence of a tumour after excision or malignant transformation of a benign one.

द्विर्बुद → When two or more arbudas develop in different sites simultaneously or one after, the condition is known as Dvirarbuda. It is mainly due to the vitiation of Vyanavayu. It can be understood as metastatic spreading of the disease.

अर्बुद चिकित्सा →

अर्बुदे ग्रन्थिवत् कुर्यात् यथास्वं सुतरां हितम् । (अ.ह.उ.30/8)

Treatments for अर्बुद are same as the treatments advocated for ग्रन्थि.

In अपक्वावस्था treatment of ग्रन्थि is like that of any शोफ (swellings).

1. वातज अर्बुद चिकित्सा →

- उपनाह स्वेद with seeds of कर्करुक, एव्रारुक, नारिकेल, प्रियाल, पंचांगुल or poultice made of मांस or वेशवार
- रक्तमोक्षण (with शृंग)
- त्रिवृत स्नेहपान (घृत+तैल+वसा) with वातहर द्रव्य

2. पित्तज अर्बुद चिकित्सा →

- उपनाह स्वेद & विरेचन
- रक्तमोक्षण (by लेखन with गोजिह्वापत्र)
- After rubbing the tumour with rough leaves of udumbara, shaka, gojihva → fine paste of sarjarasa, priyangu, lodhra, srotonjana, yashtika, aragvadha etc. mixed with honey should be applied.

3. कफज अर्बुद चिकित्सा →

- शोधन (वमन-विरेचन etc.) & रक्तमोक्षण
- उपनाह with वामक & विरेचक द्रव्य or prepared from excreta of dove and pigeon, kamsya nila (bronze slag), suka, and langalaka mixed with gomutra.
- क्षार कर्म or अग्नि कर्म

4. रक्तज अर्बुद चिकित्सा →

- असाध्य → Treatment can be tried with शस्त्रकर्म (excision)

5. मांसज अर्बुद चिकित्सा →

- असाध्य → Treatment can be tried with शस्त्रकर्म (excision)

6. मेदोज अर्बुद चिकित्सा →

- स्वेदन & भेदन (पाटन क्रिया)
- व्रण रोपण (Wound is sprinkled with powder of haridra, grihadhuma, lodhra, pattanga, manahshila etc. → Next karanja taila added with more of honey and should be applied to clean the wound.)

प्रमेह (Diabetes)

निरुक्ति →

प्रकर्षेण मेहति इति प्रमेह ।

Prameha is characterized by increased frequency & quantity of urination.

निदान →

आस्यासुखं स्वप्नसुखं दधीनि ग्राम्यौदकानूपरसाः पयांसि ।

नवान्नपानं गुडवैकृतं च प्रमेहहेतुः कफकृच्च सर्वम् ॥ (च.चि.6/4)

There are two types of प्रमेहरोगी → (i) स्थूल प्रमेही & (ii) कृश प्रमेही. Hence प्रमेह-हेतु can also be classified as (i) संतर्पक हेतु & (ii) अपतर्पक हेतु →

संतर्पक हेतु		अपतर्पक हेतु	
आहारज	विहारज	आहारज	विहारज
- अतिदधि सेवन - ग्राम्यौदक आनूप-मांस सेवन - इक्षुविकार अतिसेवन - मेदकफ वर्धक आहार - नव अन्नधान्य सेवन - मद्यपान	- आस्यासुख - स्वप्न सुख - स्नान त्याग - व्यायाम त्याग - दिवास्वप्न - आलस्य	- कटु-तिक्त कषाय रस अतिसेवन - कर्षण प्रयोग - अनशन - विषमाशन	- काम-क्रोध-शोक-भय-चिन्ता - वमन-विरेचन-आस्थापन अतिसेवन - आतप अतिसेवन

संतर्पण जन्य प्रमेह (स्थूल प्रमेही) is correlated with Non-Insulin Dependent Diabetes mellitus (NIDDM).

अपतर्पण जन्य प्रमेह (कृश प्रमेही) is correlated with Insulin Dependent Diabetes mellitus (IDDM).

पूर्वरूप →

स्वेद – अंगगन्ध – शिथिल अंग – केशनख अतिवृद्धि – गलतालु शोष – करपाद दाह

सम्प्राप्ति (Pathogenesis) →

- कफज प्रमेह सम्प्राप्ति – प्रकुपित कफ → मेदोधातु-मांसधातु-बस्तिस्थित(मूत्राशय) क्लेद दुष्टी → कफज प्रमेह
- पित्तज प्रमेह सम्प्राप्ति – प्रकुपित पित्त → मेद-मांस-क्लेद दुष्टी → पित्तज प्रमेह
- वातज प्रमेह सम्प्राप्ति – प्रकुपित वात → वसा-मज्जा-ओजादि धातु दुष्टी & स्थानसंश्रय in बस्ति → वातज प्रमेह

दश दूष्य → रस – रक्त – मांस – मेद – मज्जा – शुक्र – ओज – अम्बु – वसा – लसीका

प्रमेह सामान्य-लक्षण →

प्रकर्षेण प्रभूतं=प्रचुर बारं-बारं वा मेहति मूत्र त्याग करोति यस्मिन् रोगे स प्रमेहः ।

General symptoms of all types of prameha are increased frequency and increased quantity of urination with turbid urine, resulting in excess thirst.

प्रमेह भेद →

(i) According to cause →

- सहज प्रमेह (जातप्रमेह)
- अपथ्य निमित्तज प्रमेह

(ii) According to doshas → (20 types)

1. कफज प्रमेह (10 types)

उदकमेह – इक्षुमेह – सान्द्रमेह – सान्द्रप्रसादमेह – शुक्लमेह – शुक्रमेह – शीतमेह – शनैर्मेह – सिकतामेह – लालामेह

2. पित्तज प्रमेह (6 types)

क्षारमेह – कालमेह – नीलमेह – हारिद्रमेह – मांजिष्ठामेह – रक्तमेह

3. वातज प्रमेह (4 types)

मज्जामेह – वसामेह – लसीकामेह – मधुमेह

कफज प्रमेह	10	उदकमेह – इक्षुमेह – सान्द्रमेह – सान्द्रप्रसादमेह – शुक्लमेह – शुक्रमेह – शीतमेह – शनैर्मेह – सिकतामेह – लालामेह
पित्तज प्रमेह	6	क्षारमेह – कालमेह – नीलमेह – हारिद्रमेह – मांजिष्ठामेह – रक्तमेह
वातज प्रमेह	4	मज्जामेह – वसामेह – लसीकामेह – मधुमेह
1	उदकमेह	The urine resembling water
2	इक्षुमेह	The urine resembling sugarcane juice
3	सान्द्रमेह	The urine having high density (thick urine)
4	सान्द्रप्रसादमेह	Precipitating (high density in lower layers of urine)
5	शुक्लमेह	The urine having white colour
6	शुक्रमेह	The urine containing seminal fluid
7	शीतमेह	The urine with cold touch
8	शनैर्मेह	The urine passing out slowly
9	सिकतामेह	The urine containing sand like particles
10	लालामेह	The urine containing slimy material like saliva
11	क्षारमेह	The urine resembling the solution of alkalies
12	कालमेह	The urine having black colour
13	नीलमेह	The urine having blue colour
14	हारिद्रमेह	The urine having yellow colour like haridra
15	मांजिष्ठामेह	The urine having reddish colour like manjishtha
16	रक्तमेह	The urine having blood in it
17	मज्जामेह	The urine mixed with majja or bone marrow
18	वसामेह	The urine mixed with vasa or muscle fat
19	लसीकामेह or हस्तिमेह	The urine mixed with lasika or lymph. Or the patient urinates more in quantity like an elephant
20	मधुमेह	The urine mixed with ojas

साध्यासाध्यता → कफज is साध्य, पित्तज is याप्य, & वातज is असाध्य.

प्रमेह is incurable (असाध्य) in following conditions →

प्रमेहपिडका युक्त प्रमेह – उपद्रव युक्त प्रमेह – जातप्रमेह – मधुमेह

प्रमेह-चिकित्सा सूत्र →

स्थूलः प्रमेही बलवानिहैकः कृशस्तथैकः परिदुर्बलश्च ।

सबृंहणं तत्र कृशस्य कार्यं संशोधनं दोषबलाधिकस्य ॥ (च.चि.6/15)

- स्थूल प्रमेही (बलवान) should be treated with शोधन चिकित्सा
- कृश प्रमेही (दुर्बल) should be treated with शमन & बृंहण चिकित्सा

➤ **संशोधन चिकित्सा in स्थूल and बलवान प्रमेही** →

→ निदान परिवर्जन

→ संशोधन → आभ्यंतर स्नेहन with त्रिफला घृत or पिप्पली घृत

स्वेदन निषेध (fomentation should not be done in prameha)

तीक्ष्ण वमन-विरेचन

तीक्ष्ण-उष्ण-रुक्ष बस्ति acc. to dosha

(मुस्तादि यापनाबस्ति / मधुतैलिक बस्ति / प्रमेहहर आस्थापन बस्ति)

अनुवासन बस्ति निषेध (contraindicated in स्थूल प्रमेही because it is nourishing type of basti)

→ कफ-मेदोहर चिकित्सा – तक्रारिष्ट – मुस्तादि क्वाथ – मधूदक

→ व्यायाम – जागरण – चिन्तन – मानसिक & शारीरिक श्रम

➤ **संशमन चिकित्सा in कृश and दुर्बल प्रमेही** →

→ निदान परिवर्जन

→ संतर्पण चिकित्सा (बृंहण) – रसायन & वाजीकरण योग

→ मन्थ – कषाय – अवलेह – यवान्न with मधु – सालसरादिगण क्वाथ भावित शिलाजतु

→ अभ्यंग – उन्मर्दन – प्रदेह – परिषेक – मानसिक & शारीरिक विश्राम

अर्थ (साधन) रहित प्रमेही चिकित्सा →

- Those patients, who don't have money to afford the treatment, should be advised following regimens →
- Without footwears, patient should travel like a 'भिक्षुक'. He should travel or walk 100 योजन.
- आमलकी, कपित्थ, तिन्दुक, अशमन्तक फल, गोमूत्र, गोमय etc. should be used.
- ब्राह्मण should read वेद while sitting on stones.
- Except ब्राह्मण, patients should forrow the fields and dig the well.

1. कफज प्रमेह चिकित्सा →

- अपतर्पण should be done in सन्तर्पणजन्य प्रमेह and then after वमन, सन्तर्पण should be done
- मन्थ, कषाय or लेह of यवचूर्ण (जौ का सत्तु)
- हरिद्राचूर्ण and मधु with आमलकी स्वरस
- कफ प्रमेहनाशक हरीतक्यादि दश कषाय योग should be used
- In मेदवृद्धि → त्रिफला, गुडूची, शिलाजतु, मधु etc. should be used

2. पित्तज प्रमेह चिकित्सा →

- After विरेचन, सन्तर्पण should be done
- पित्तज प्रमेहनाशक उशीर-लोध्रादि दश कषाय योग should be used

3. वातज प्रमेह चिकित्सा →

- स्नेह (तैल & घृत) processed with कफ-पित्त प्रमेहनाशक औषध

प्रमेह पिडका (7) → शराविका – कच्छपिका – जालिनी – सर्षपी – अलजी – विनता – विद्रधि

प्रमेह पिडका चिकित्सा → निदान परिवर्जन – शस्त्रकर्म – रोपणकारक औषध प्रयोग

Medicines (शमनौषधी) used in प्रमेह → बसन्तकुसुमाकर रस – त्रिवंग भस्म –

शिलाजतु – चन्द्रप्रभा वटी – शिलाजत्वादि वटी – त्रिफला चूर्ण – निशामलकी चूर्ण – फलत्रिकादिक्वाथ – लोधासव – दन्त्यासव – मधूदक – सारोदक – कुशोदक etc.

पथ्य आहार-विहार → यव – यवचूर्ण (सत्तु) – मुद्ग यूष – पुराण शालि – चणक – कोद्रव – अतसी – सर्षप तैल – सैन्धव – मधु – पटोल – कारवेल्लक – आमलकी – हरिद्रा – मरिच – चक्रमण – लघु व्यायाम – रूक्ष उद्वर्तन – क्रीडा etc.

अपथ्य आहार-विहार → दुग्ध – दधि – तक्र – तैल – गुड – इक्षुविकार – पिष्टान्न – आनूप मांस – माष – अध्यशन – दिवास्वप्न – अतिमैथुन – स्वेदन – धूमपान – वेगधारण etc.

मधुमेह

It is called मधुमेह, क्षौद्रमेह or ओजोमेह. The patient passes urine mixed with ojas frequently and the urine is kashaya, madhura, pandu varna & ruksha in nature.

Two types → (1) धातुक्षय जन्य मधुमेह & (2) दोषावृत जन्य मधुमेह

पूर्वरूप → मुखमाधुर्य – कर पाद दाह & सुप्तता – मुख तालु कण्ठ शोष – पिपासाधिक्य

रूप (लक्षण) → प्रभूत मूत्रता – आविल मूत्रता – शरीर गौरवता – विबन्ध – शरीर जाड्यता – तृष्णाधिक्य – क्षुधाधिक्य – स्वेदाधिक्य – दौर्बल्य – चिरकालीन व्रण रोपण – कण्डू etc.

मधुमेह चिकित्सा → (i) संशोधन चिकित्सा :

- (a) स्थूल प्रमेही → संशोधन पञ्चाद संतर्पण & संशमन चिकित्सा
- (b) कृश प्रमेही → संशमन & बृंहण चिकित्सा

(ii) संशमन चिकित्सा :

- (a) निदान परिवर्जन
- (b) व्यायाम
- (c) रूक्ष अन्नपान
- (d) औषध व्यवस्था

प्रमेह उपद्रव (Complications) → तृष्णा – दाह – अरोचक – अविपाक – पूतिमांस – प्रमेह पिडका – अलजी – विद्रधि – कण्डू – छर्दि – अतिसार – उदावर्त – मक्षिकोपसर्ग etc.

Diabetes mellitus

Diabetes mellitus (DM) is a group of metabolic disorder (syndrome) characterized by high blood sugar level (hyperglycemia) due to absolute or relative deficiency of insulin.

There are mainly three types of DM →

1. **Type-1 DM** → It results from the pancreas's failure to produce enough insulin. This form was previously referred to as "Insulin Dependent Diabetes Mellitus" (IDDM) or "juvenile diabetes". The cause is unknown.
2. **Type-2 DM** → It begins with insulin resistance, a condition in which cells fail to respond to insulin properly. As the disease progresses a lack of insulin may also develop. This form was previously referred to as "Non Insulin Dependent Diabetes Mellitus" (NIDDM) or "adult-onset diabetes". The most common cause is excessive body weight and insufficient exercise.
3. **Gestational DM** → When pregnant women without a previous history of diabetes develop high blood sugar levels (hyperglycemia).

Clinical Features →

- Classic hyperglycemic triad → Polyuria – Polydipsia – Polyphagia
- Weight loss
- Tiredness / debility
- Blurred vision
- Genital itching
- Delayed wound-healing

- Symptoms may develop rapidly (weeks or months) in type 1 DM, while they usually develop much more slowly and may be subtle or absent in type 2 DM.

Complications →

- Diabetic ketoacidosis (DKA)
- Cardiovascular disease (angina, atherosclerosis etc.)
- Diabetic neuropathy (Nerve damage)
- Diabetic nephropathy (Kidney damage)
- Diabetic retinopathy (Eye damage)
- Diabetic foot ulcers
- Skin lesions etc.

Investigations →

- Fasting blood glucose level (FBS) > 126 mg/dL
(FBS less than 100 mg/dL is normal. FBS from 100 to 125 mg/dL is considered prediabetes. If it's 126 mg/dL or higher on two separate tests, diabetes is confirmed.)
- Post-prandial blood glucose level (PPBS) > 200 mg/dL
- Random blood glucose level (RBS) > 200 mg/dL & symptoms
- Urinary glycosuria – present

Treatment →

- Diet control
- Daily exercise & Life style modifications
- Medications – Insulin injections for Type-1 DM
Metformin / Sulphonylureas / Insulin for Type-2 DM
- Mass Education / Counselling to the patient

स्थौल्य (Obesity)

निरुक्ति →

“स्थूलस्य भावः स्थौल्यम् ।” The word स्थौल्य is derived from “स्थूल”.

“स्थूल परिवृंहणे” The word स्थूल denotes the meaning of परिवृंहण or वृद्धि. Hence स्थौल्य means over-weight (अति स्थूल) or excess accumulation of body fat (मेदो वृद्धि).

निदान →

आहारज निदान	गुरु स्निग्ध आहार अतिसेवन – अतिभोजन – अध्यशन – माहिष दुग्ध, घृत आदि अतिसेवन – नवान्न – पिष्टान्न – आनूप मांस – अति मद्यपान – गुड & गुडविकार – वारुणी मद्य सेवन
विहारज निदान	अव्यायाम (अचेष्टा) – अव्यवाय – दिवास्वप्न – सुख स्वप्न – अतिनिद्रा – सुख आसन
मानसिक निदान	अचिन्ता (निश्चिन्तता) – हर्ष – सन्तोष
अन्य निदान	सहज or कुलज कारण

लक्षण →

स्थौल्य अष्टदोष (Acharya Charaka has explained eight defects of Sthaulya)

1. आयु ह्रास – Short life span
2. जवापरोध – Inability to work
3. कृच्छ्रव्यवायता – Loss of libido
4. दौर्बल्य – Weakness
5. दौर्गन्ध्य – Foul odour of the body
6. अतिस्वेद – Excess sweating
7. अतिक्षुदा – Excess appetite
8. अतितृष्णा – Excess thirst

Rabin Singh

स्थौल्य जन्य रोग (or उपद्रव) → प्रमेह – अर्श – भगन्दर – ज्वर – कामला – अतिसार – विसर्प – क्षीपद – अपची – उच्च रक्तचाप – सन्धिवात – कटिग्रह etc.

चिकित्सा सिद्धान्त →

गुरुचापतर्पणं चेष्टं स्थूलानां कर्षणं प्रति । (च.सू.21/20)

- निदान परिवर्जन
- गुरु अपतर्पण चिकित्सा
- कर्षण

1. संशोधन →

- वमन
- विरेचन
- निरूह वस्ति
- कर्षण नस्य प्रयोग

2. संशमन →

पारद भस्म – वडवाग्नि रस – भेदनी वटी – मेदोहर वटी – त्रिफला चूर्ण – त्रिकटु चूर्ण – विडंगादि चूर्ण – फलत्रिकादि क्वाथ – मुस्तादि क्वाथ – लोहासव – मेदोहर गुग्गुलु – त्रयोदशांग गुग्गुलु – गुग्गुलु रसायन – शिलाजतु रसायन – आमलकी रसायन

स्वेदहर & दौर्गन्ध्य नाशक लेप → शिरीषादि लेप – हरितक्यादि लेप – वसादि लेप – शैलेयाद्य लेप etc.

योगासन → सूर्य नमस्कार – सर्वांगासन – हलासन – मयूरासन – प्राणायाम etc.

पथ्य आहार-विहार → पुराण शालि – मुद्ग – यव – कुलत्थ – कोद्रव – चणक – मसूर – आढकी – पटोल – वृन्ताक – शिगु – तक्र – एला – सर्षप तैल – उष्णोदक – लंघन – उद्वर्तन – चिन्ता – श्रम – व्यायाम – चक्रमण – आतपसेवन

अपथ्य आहार-विहार → नव धान्य – गोधूम – माष – आलूक – दुग्ध – दधि – मांस – मत्स्य – गुड – तैलाभ्यंग – शीतजल स्नान – दिवास्वप्न – सुखशय्या – निश्चिन्ता

काश्य

निरुक्ति →

The word काश्य is derived from “कृश”. It is a disease characterized by अतिकृशता that means excess leanness or emaciation (मांस क्षय or मेदो क्षय).

निदान →

आहारज निदान	रूक्ष अन्नपान सेवन – लंघन (उपवास) – प्रमिताशन (अल्प भोजन) – वातवर्धक आहार सेवन – कषाय रस अतिसेवन
विहारज निदान	अतिव्यायाम – अतिव्यवाय – रात्रिजागरण – मल मूत्र आदि वेगधारण – तृष्णा क्षुधा वेगधारण – रूक्ष उद्वर्तन – वमन विरेचनादि पंचकर्म अतियोग
मानसिक निदान	चिन्ता – शोक – भय – क्रोध
अन्य निदान	सहज or कुलज कारण – अत्यधिक शारीरिक & मानसिक चेष्टा – चिरकालीन व्याधिग्रस्त

लक्षण →

शुष्कस्फिगुदरग्रीवो धमनी जाल सन्ततः।

त्वगस्थिशेषोऽतिकृशः स्थूलपर्वा नरो मतः ॥ (च.सू.21/15)

- शुष्क स्फिक्-उदर-ग्रीवा – Thinner thighs, abdomen and neck
- धमनीजाल सन्तत – Appearance of vascular network on skin
- त्वक् अस्थि शेष – Skinny and skeletal appearance of body
- स्थूल पर्व – Swollen joints
- अरुचि – Anorexia
- आलस्य – Laziness
- श्रम – Tiredness
- शीत-उष्ण-असह्यत्व – Intolerable to heat and cold
- सर्वदा रोग-ग्रस्त – Always suffering from diseases

उपद्रव → प्लीहा वृद्धि – कास – श्वास – गुल्म – अर्श – क्षय (राजयक्ष्मा) – उदर – ग्रहणी

काश्य चिकित्सा →

कृशानां बृंहणार्थं च लघु संतर्पणं च यत् । (च.सू.21/20)

- निदान परिवर्जन
- आवश्यकतानुसार संशोधन
- बल्य बृंहण चिकित्सा
- लघु संतर्पक आहार सेवन

1. संशोधन →

बाह्य & आभ्यन्तर स्नेहन – अनुवासन बस्ति – बृंहण बस्ति – बृंहण नस्य प्रयोग

2. संशमन →

क्षुधासागर रस – प्रवाल पंचामृत – अग्निपुण्ड्री वटी – चित्रकादि वटी – चन्द्रप्रभा वटी – आरोग्यवर्द्धिनी वटी – आमलकी चूर्ण – शतावरी चूर्ण – अश्वगन्धा चूर्ण – अश्वगन्धारिष्ट – द्राक्षासव – बला घृत – बला तैल – महानारायण तैल – च्यवनप्राश – हरीतक्यादि अवलेह – अमृतप्राश

बल्य & बृंहणद्रव्य → दुग्ध – घृत – मांसरस – बला – अतिबला – अश्वगन्धा – मूसली – शालपर्णी – लशुन – विदारीकन्द – आमलकी – द्राक्षा – मृद्विका – खर्जूर

योगासन → शवासन – सुखासन – वज्रासन – प्राणायाम – योग साधना etc.

पथ्य आहार-विहार →

नवान्न – नवनिर्मित मद्य – ग्राम्य आनूप & जांगल मांसरस – दुग्ध – दधि – घृत – इक्षु – षष्टिक शालि – गोधूम – स्निग्ध & मधुर द्रव्य – रसायन & वाजीकरण द्रव्य

अपथ्य आहार-विहार →

कटु तिक्त कषायरस प्रधान द्रव्य – रूक्षान्न – यव – चणक – मसूर – शिग्रू – कोद्रव – लंघन – अतिव्यायाम – अति परिश्रम – अति संशोधन etc.

Dyslipidaemia

Dyslipidemia is an abnormal amount of lipids in the blood. Lipids, or fats, are building blocks of life and provide energy to cells. Lipids include:

LDL cholesterol, which is considered bad because it can cause plaques to form in the blood vessels.

HDL cholesterol, which is regarded as good because it can help to remove LDL from the blood.

Triglycerides, which develop when calories are not burned right away and are stored in fat cells.

Types of Dyslipidemia →

Types	Increases	Decreases
Lipid	Hyperlipidemia: lipids → Hypercholesterolemia: cholesterol. Familial hypercholesterolemia. → Hyperglyceridemia: glycerides. → Hypertriglyceridemia: triglycerides	Hypolipidemia → Hypocholesterolemia: cholesterol
Lipoprotein	Hyperlipoproteinemia: lipoproteins (usually LDL unless otherwise specified) → Honemia: chylomicrons	Hypolipoproteinemia: lipoproteins → Abetalipoproteinemia: beta lipoproteins → Tangier disease: high density lipoprotein
Both	Combined hyperlipidemia: both LDL and triglycerides	

Healthy blood lipid levels naturally vary from person to person. However, people with high levels of LDL and triglycerides or very low HDL levels tend to have a higher risk of developing atherosclerosis.

Atherosclerosis develops when hard, fatty deposits called plaques accumulate in blood vessels, making it difficult for blood to flow. Over time, these plaques can build up and cause major circulation problems, such as heart attacks and strokes.

Signs & Symptoms of Dyslipidemia →

Severe or untreated dyslipidemia can lead to other conditions, including coronary artery disease (CAD) and peripheral artery disease (PAD). Both CAD and PAD can cause serious health complications, including heart attacks and strokes. Common symptoms of these conditions include:

- Leg pain, especially when walking or standing
- Chest pain, tightness or pressure in the chest and dyspnoea
- Pain, tightness, and pressure in the neck, jaw, shoulders, and back
- Sleep problems and daytime exhaustion
- Dizziness
- Fainting

Investigations →

Lipid profile → Testing the blood cholesterol level →

- Normal blood level of total cholesterol is <200 mg/dL, between 200 and 239 mg/dL is considered border-line high. A reading of 240 mg/dL or higher is considered high.

- LDL cholesterol levels should be <100 mg/dL. Levels of 100 to 129 mg/dL are acceptable for people with no health issues but may be of more concern for those with heart disease or heart disease risk factors. A reading of 130 to 159 mg/dL is borderline high and 160 to 189 mg/dL is high. A reading of 190 mg/dL or higher is considered very high.
- A normal level of HDL cholesterol is between 40-50 mg/dL.
- Triglyceride levels should be <150 mg/dL, between 150 and 199 mg/dL is considered border-line high. A reading of 200 mg/dL or higher is considered high.

Treatment →

Lifestyle changes such as exercising and eating a healthy diet are the first line of defense against high cholesterol. But, if cholesterol levels remain high with these important lifestyle changes, medication is required which depends on various factors, including individual risk factors, age, current health and possible side effects. Common drugs of choices include:

- **Statins** → Atorvastatin, Fluvastatin, Lovastatin, Pitavastatin, Pravastatin, Rosuvastatin and Simvastatin lower the level of cholesterol in the blood by reducing the production of cholesterol by the liver.
- **Bile-acid-binding resins** → Cholestyramine, Colesevelam and Colestipol lower cholesterol indirectly by binding to bile acids.

- **Cholesterol absorption inhibitors** → The drug ezetimibe helps reduce blood cholesterol by limiting the absorption of dietary cholesterol. Ezetimibe can be used in combination with a statin drug.
- **Fibrates** The medications fenofibrate and gemfibrozil decrease triglycerides by reducing liver's production of very-low-density lipoprotein (VLDL) cholesterol and by speeding up the removal of triglycerides from blood. VLDL cholesterol contains mostly triglycerides.

Diet → Eating a healthy, balanced diet low in saturated fats can reduce level of "bad cholesterol" (LDL). Avoid or cut down on the following foods, which are high in saturated fat →

- fatty cuts of meat and meat products, such as sausages and pies
- butter, ghee and lard
- cream, soured cream, and ice cream
- cheese, particularly hard cheese
- cakes, biscuits and milk chocolate
- coconut oil, coconut cream and palm oil
- Many experts believe that the omega-3 fatty acids found in soybeans, walnuts, flaxseeds, chia seeds, oily fish such as mackerel, sardine, salmon and tuna, are good for cholesterol.
- Herbal drugs → Garlic (लशुन), Ginger (आर्द्रक), Turmeric (हरिद्रा), and Guggulipid (गुग्गुलु) are useful for lowering blood cholesterol level.

Diseases of अस्थि & मज्जावह स्रोतस

अस्थिमज्जा विद्रधि (Osteomyelitis)

अथमज्जपरिपाको घोरः ससमुपजायते ।

सोस्थिमांसनिरोधेन द्वारं न लभतेयदा ॥

ततः स व्याधिना तेन ज्वलनेनेव दह्यते ॥ (सु.नि.9/33,34)

The aggravated doshas vitiate the त्वक्, रक्त, मांस, and मेद getting स्थानसंश्रय in अस्थि. When there is suppuration process (पाको घोरः) in the अस्थि & मज्जा, the pus fails to find an outlet and thus results in severe burning pain (ज्वलनशील वेदना).

चिकित्सा → It is प्रत्याख्येय that means the physician can treat it without holding any definite hope of recovery. With स्नेहन-स्वेदन & रक्तमोक्षण, विद्रधि चिकित्सा should be followed. Incision (भेदन) and drainage of pus (पूयनिर्हरण), followed by व्रण शोधन & रोपण.

Osteomyelitis →

Osteomyelitis is an infection of bone. Infection can be primary (haematogenous) or secondary (fracture or bone operation).

Another classification is → (a) Acute pyogenic osteomyelitis

(b) Subacute osteomyelitis

Brodie's abscess

(c) Chronic osteomyelitis

Chronic pyogenic osteomyelitis

Tuberculous osteomyelitis

Syphilitic osteomyelitis etc.

Aetiopathogenesis → Staphylococcus aureus is common causative organism. These organisms reach bone via blood circulation (hematogenous– primary infection). The bacteria as they pass through bone, get lodged in the metaphysis. The host bone initiates an inflammatory reaction in response to the bacteria. This leads to bone destruction and production of an inflammatory exudate & cell (pus). Once sufficient pus forms in the medullary cavity, it spreads to sub periosteally, muscle or subcutaneous plane, where it can be felt as an abscess. The abscess, if untreated then bursts out of skin forming discharging sinus.

Clinical Features → Pain & swelling at the end of a bone (usually long bones affected), painful movements, fever with chills, dehydration, signs of inflammation.

Complications → Septicaemia, Pyaemia, Toxaemia, Amyloid disease, Pathological fracture, Chronic sinus formation etc.

Investigations →

- Routine blood test, Blood culture
- Aspiration of the bone using a thick needle
- X-ray

Treatment →

- incision and drainage of pus (I&D)
- Antibiotics
- analgesics & anti-inflammatory
- use of splint
- rest

अस्थिसौशीर्य (Osteoporosis)

Osteoporosis →

Osteoporosis, which literally means porous bone, is a disease in which the density and quality of bone are reduced. It is characterized by thinning of the bones, with reduction in bone mass, due to depletion of calcium and bone protein. Osteoporosis predisposes a person to fractures, which are often slow to heal and heal poorly.

Etiology → Osteoporosis becomes more common with old age. Bone loss increases after menopause due to lower levels of estrogen. Osteoporosis may also occur due to a number of diseases or treatments including alcoholism, anorexia, hyperthyroidism, kidney disease, and surgical removal of the ovaries. Certain medications increase the rate of bone loss including some antiseizure medications, chemotherapy, proton pump inhibitors, selective serotonin reuptake inhibitors, and glucocorticosteroids. Smoking and too little exercise are also risk factors.

Clinical Features → Bones may weaken to such a degree that a break may occur with minor stress or spontaneously. Chronic pain and a decreased ability to carry out normal activities may occur following a broken bone. Backache, fatigue, brittle fingernails, spine deformities like kyphosis, bone pain and tenderness, cramps in the legs at nights.

Investigation → X-ray or Dual-energy X-ray absorptiometry (DEXA scan)

Treatment → Medications → Bisphosphonates (such as Alendronate, ibandronate, risedronate, and zoledronic acid) & SERMs such as raloxifene; estrogen; calcitonin; and teriparatide.

Aerobics, weight bearing, and resistance exercises.

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अस्थिक्षय (Osteopenia)

Osteopenia →

Osteopenia is decreased bone density but not to the extent of osteoporosis. This decreased bone density leads to bone fragility and an increased chance of breaking a bone (fracture).

Etiology → Like osteoporosis, osteopenia occurs more frequently in post-menopausal women as a result of the loss of estrogen. Risk factors are – lack of exercise, excess consumption of alcohol, smoking or prolonged use of glucocorticoid medications.

Osteopenia vs. Osteoporosis → Osteopenia and osteoporosis are related conditions. The difference between osteopenia and osteoporosis is that in osteopenia the bone loss is not as severe as in osteoporosis.

Osteopenia vs. Osteomalacia → Osteopenia is a bone condition characterized by decreased bone density, which leads to bone weakening and an increased risk of bone fracture. Osteomalacia is a bone disorder characterized by decreased mineralization (demineralization) of newly formed bone. Osteomalacia is caused by severe vitamin D deficiency and by conditions that cause very low blood phosphate levels.

Treatment →

An adequate intake of calcium and vitamin D, avoiding excessive alcohol, not smoking, and getting plenty of exercise can help prevent osteopenia. Commonly used drugs are → (a) Bisphosphonates, and (b) Selective estrogen receptor modulators (SERMs) such as – raloxifene; estrogen; calcitonin; and teriparatide.

संधिगत वात (Osteoarthritis)

वातपूर्ण दृतिस्पर्शः शोथः सन्धिगतेऽनिले ।

प्रसारणाकुञ्चनयोः प्रवृत्तिश्च सवेदना ॥ (च.चि.28/37)

लक्षण → On palpation- swelling over joint feels like air-filled bag. There will be pain and difficulty in movement of joints in Sandhigata vata. (It is a vatavyadhi so nidana & samprapti include aggravated vata, especially)

चिकित्सा →

कुर्यात् सान्धिगते वाते दाह स्नेहोपनाहम् । (भा.प्र.म.ख.24/259)

→ दाह कर्म

→ स्नेहकर्म (स्नेहपान/अभ्यंग/जानुबस्ति/अनुवासनबस्ति)

→ उपनाह चिकित्सा

- वातहर संशोधन चिकित्सा (e.g. स्नेह with महानारायण तैल – नाडी स्वेद with दशमूल क्वाथ – अनुवासन बस्ति with महानारायण तैल or दशमूल तैल, निरूह बस्ति with एरण्डमूलादि क्वाथ or दशमूलादि क्वाथ etc.)
- वातहर संशमन चिकित्सा (e.g. वातविध्वंसक रस, त्रयोदशांग गुग्गुलु, रास्नादि गुग्गुलु, पंचकोल चूर्ण, हरीतकी चूर्ण, दशमूलारिष्ट etc.)

Osteoarthritis →

Osteoarthritis (OA) is the most common form of arthritis that results from breakdown of joint cartilage and underlying bone. It is characterized by progressive degenerative changes in the articular cartilages, and commonly affects the hands, feet, spine, and the large weight-bearing joints, such as the hips and knees

Types → Two main types →

1. Primary osteoarthritis
2. Secondary osteoarthritis

Rabin Singh

1. Etiological factors of primary osteoarthritis →

Genetic factors – Metabolic disorders – Old age – Idiopathic avascular necrosis – Endocrinal factors – Obesity etc.

2. Etiological factors of secondary osteoarthritis →

Trauma – Mal-alignments – Inadequate blood supply – Nutritional bone diseases – Inflammatory diseases – Infections of the joints e.g. pyogenic, tuberculosis etc.

Clinical Features →

- Pain – It is initial and leading symptom.
- Stiffness – It follows pain.
- Deformity due to shrinkage of capsule, fibrosis, muscle imbalance.
- Limping – due to pain, stiffness and deformity of the joint.
- Synovial thickening and effusion resulting in swelling of the joints.
- Osteophytes may be felt on palpation.

Investigations →

CBC – ESR – X ray of affected joints (AP and Lateral view) – RA factor to exclude Rheumatoid arthritis

Treatment →

- Lifestyle modification (such as weight loss and exercise) and analgesics are the mainstays of treatment.
- Paracetamol with NSAIDs such as aspirin or naproxen or diclofenac.
- Topical ointments with diclofenac.
- If conservative management is ineffective → Joint replacement surgery or resurfacing.

Diseases of शुक्रवह स्रोतस

क्लैब्य (Impotency)

A man, who is incapable of sexual intercourse, is called क्लीब or नपुंसक, and such disease is called क्लैब्य (impotency or erectile dysfunction).

सामान्य निदान →

अतिमैथुन – अकाल मैथुन – अयोनि मैथुन – कटु तिक्त कषाय अम्ल लवण उष्ण आहार अतिसेवन – भय चिन्ता शोक क्रोध – वेगधारण – शुक्रक्षय – शुक्रवह स्रोतो दुष्टी – वृद्धावस्था – शस्त्र क्षार अग्निकर्म मिथ्यायोग etc.

भेद → Klaihya is classified on the basis of etiology →

1. बीजोपघात जन्य क्लैब्य → due to pathology of spermatogenesis → Patient is weak, depressed and unhappy with his relation with women. He can be affected by पाण्डुरोग, हृद्रोग, तमकश्वास, कास, ज्वर etc.
2. ध्वजोपघात जन्य क्लैब्य → due to pathology of genital (e.g. STD) → Genital organ is having pain, swelling, redness, boils or abscess. It can discharge like rice-water and undergo necrosis.
3. जरा जन्य क्लैब्य → due to old age → The patient is weak, getting tired without doing any work, affected by various diseases.
4. शुक्रक्षय जन्य क्लैब्य → due to depletion of shukra dhatu → Symptoms of Shukradhatu kshaya – Pratiloma dhatu kshaya leads to death.

क्लैब्य चिकित्सा सूत्र →

शुक्रदोषेषु निर्दिष्टं भेषजं यन्मयाऽनघ।

क्लैब्योपशान्तये कुर्यात् क्षीणक्षतहितं च यत् ॥ (च.चि.30/192)

Impotency should be treated with the drugs and therapies useful in शुक्रदोष चिकित्सा and क्षतक्षीण चिकित्सा.

Rabin Singh

सामान्य चिकित्सा →

- संशोधन – बस्ति कर्म – रसायन & वाजीकरण प्रयोग
- दैवव्यपाश्रय चिकित्सा (मणिधारण-यज्ञ-स्तुति-बलि-होम-ब्रह्मचर्य-जप-तप etc.)

1. बीजोपघातज क्लैब्य चिकित्सा →

- वाजीकरण प्रयोग

2. ध्वजोपघात or ध्वजभंग जन्य क्लैब्य चिकित्सा →

- प्रदेह → परिषेक → रक्तमोक्षण
- स्नेहपान → स्नेहयुक्त विरेचन
- अनुवासन or निरूह बस्ति
- व्रण चिकित्सा

3. & 4. जराजन्य & शुक्रक्षयज क्लैब्य चिकित्सा →

- स्नेहन → स्वेदन → स्नेहयुक्त विरेचन
- यापना बस्ति
- रसायन – वाजीकरण प्रयोग
- दग्ध & घृत सेवन

रसायन योग →

शिलाजतु रसायन – अश्वगन्धा रसायन – भल्लातक रसायन – आमलकी रसायन – च्यवनप्राश – अमृतप्राश घृत

वाजीकरण योग →

वानरि गुटिका – बृंहणी गुटिका – कामेश्वर मोदक – वाजीकरण पिण्डरस – मकरध्वज – अश्वगन्धा घृत – वृष्य शतावरी घृत – वृष्य रस

वाजीकरण द्रव्य →

दुग्ध – घृत – मांसरस – कपिकच्छू – अश्वगन्धा – शतावरी – शिलाजतु – मुसली

बल्य & बृंहण द्रव्य →

बला – अतिबला – अश्वगन्धा – शालपर्णी – लशुन – विदारीकन्द – द्राक्षा – खर्जूर

शुक्रदोष

फेनिलं तनु रुक्षं च विवर्णं पूति पिच्छिलम् ।
अन्यधातूपसंसृष्टमवसादि तथाऽष्टमम् ॥

According to Acharya Charaka, due to mithya ahara vihara, 8 types of shukradosha can be manifested which can result in impotency or /and infertility →

- | | |
|-----------------------------|--|
| 1. फेनिल (frothy) | 5. पूति (foul smell) |
| 2. तनु (thin) | 6. पिच्छिल (slimy) |
| 3. रुक्ष (rough) | 7. अन्यधातूपसंसृष्ट (other dhatus mixed) |
| 4. विवर्ण (abnormal colour) | 8. अवसादि (precipitating) |

शुक्रदोष चिकित्सा सूत्र →

- निदान परिवर्जन
- रक्तपित्त नाशक चिकित्सा
- रसायन-वाजीकरण प्रयोग (जीवनीय घृत, च्यवनप्राश, शुद्ध शिलाजतु)
- In वातप्रधान शुक्रदोष → अनुवासन & निरूह बस्ति
- In पित्तप्रधान शुक्रदोष → अभयामलकी रसायन
- In कफप्रधान शुक्रदोष → भल्लातक रसायन, पिप्पली रसायन, त्रिफला रसायन

पथ्य आहार-विहार → दुग्ध – घृत – मधु – षष्टिक शालि – माष – मांसरस – मृद्विका – पिण्डखर्जूर – ताम्बूल – पूग – अभ्यंग – उन्मर्दन – स्नान – गीत संगीत श्रवण – सुगंध धारण – स्त्रीस्पर्श – वाजीकरण मित्र मण्डली

अपथ्य आहार-विहार → कटु अम्ल लवण उष्ण तीक्ष्ण आहार – विदाही अन्न – सर्षप – विरूद्धाशन – वेगधारण – रात्रिजागरण – अतिमैथुन

Diseases of मूत्रवह स्रोतस

मूत्राघात (Urinary retention)

Retention of urine due to obstruction is called Mutraghata. It is characterized by oliguria or anuria.

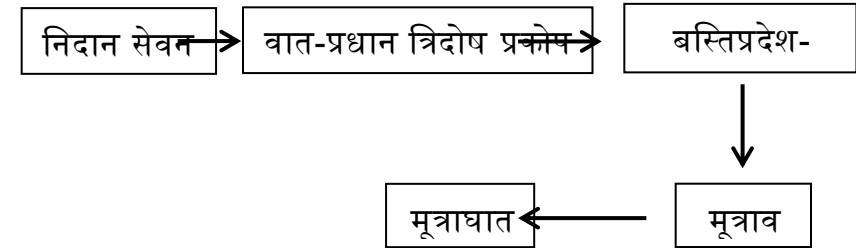
निदान →

जायन्ते कुपितैर्दोषैः मूत्रघातास्त्रयोदश ।

प्रायो मूत्रविघाताद्यैर्वात कुण्डलिकादयः ॥ (मा.नि.31/1)

- मल मूत्र आदि वेगधारण
- रूक्षादि अन्न सेवन
- वातप्रकोपक आहार-विहार

सम्प्राप्ति →



मूत्राघात भेद (Types) →

According to Sushruta → 12

According to Charaka → 13

वातकुण्डलिका – अष्टीला – वातबस्ति – मूत्रातीत – मूत्रजठर – मूत्रोत्संग – मूत्रक्षय – मूत्रग्रंथि – मूत्रशुक्र – उष्णवात – मूत्रौकसाद – विड्विघात – बस्तिकुण्डल

1. वातकुण्डलिका → Due to suppression of natural urges or intake of ruksha ahara, vata gets aggravated. Vitiated vata obstructs the bladder and causes – (i) painful & (ii) slow micturition.
2. अष्टीला (वाताष्टीला) → Vitiated vata forms immovable hard gland (tumour) and obstructs micturition. There will be distension of abdomen. It is correlated with BPH.
3. वातबस्ति → Due to suppression of urge of urination vitiated vata gets sthanasamshraya at basti-pradesha (bladder) and obstruct the micturition.
4. मूत्रातीत → Suppression of urge of urination for a long time causes retention of urine. There is pain and dribbling micturition.
5. मूत्रजठर (मूत्रोदर) → The vitiated apana-vayu causes painful distension of abdomen below the umbilicus.
6. मूत्रसंग → Obstruction in urethra and then forceful micturition leads to urine mixed with blood, scanty & frequent urination.
7. मूत्रक्षय → Vitiated vata & pitta causes painful & burning micturition, and the quantity of urine gets reduced.
8. मूत्रग्रंथि → Small, circular tumour (like a calculus) near the urethral opening of bladder (बस्तिमुख) causes pain & retention of urine.
9. मूत्रशुक्र → If the sexual intercourse is carried out when there is urge of urination, then vata gets aggravated and causes urine mixed with semen.

10. उष्णवात → Vitiated pitta and vata causes burning sensation in bladder, penis, and anal canal. The urine is yellow, red or mixed with blood.
11. मूत्रसाद → Pitta and kapha either separately or together get solidified by aggravated vata.
12. विडविघात → Due to emaciation (क्षय), urine gets smell of faeces.
13. बस्तिकुण्डल → Due to excess physical exercise or injury, the bladder is affected which causes pain and burning.

मूत्राघात चिकित्सा →

स्नेहस्वेदोपपन्नस्य हितं स्नेहविरेचनम् ।
दद्यादुत्तरबस्तिं च मूत्राघाते सवेदने ॥ (यो.र.)

- आभ्यन्तर & बाह्य स्नेहन
- सर्वदैहिक (सर्वांग) or स्थानिक (एकांग) स्वेदन
- स्निग्ध विरेचन
- उत्तर बस्ति
- मूत्रल औषध & आहार सेवन

[Mutraghata should be treated like Mutrakricchra]

Retention of urine → It may be acute / chronic. Acute retention is painful while chronic retention is painful & occurs due to chronic obstruction, ultimately resulting in acute retention.

Oliguria or hypouresis is the low output of urine. It is defined as a urine output that is less than 1 mL/kg/h in infants, less than 0.5 mL/kg/h in children, and less than 400 mL or 500 mL per 24h in adults.

Anuria or anuresis is non-passage of urine, in practice is defined as passage of less than 100 mL of urine in a day.

मूत्रकृच्छ्र (Dysuria)

निरुक्ति →

मूत्रस्य कृच्छ्रेण महता कष्टेन प्रवृत्तिः । (मधुकोष)

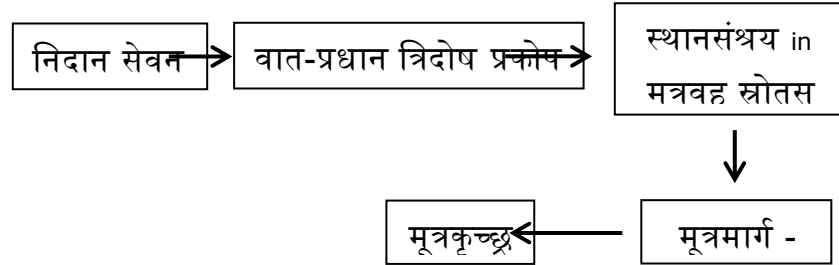
Mutrakrichchhra refers to difficult and painful micturition (i.e. dysuria).

निदान →

व्यायाम तीक्ष्णौषध रूक्ष मद्य प्रसंग नित्य द्रुत पृष्ठयानात् ।
आनूप मत्स्याध्यशनादजीर्णात् स्यु मूत्रकृच्छ्राणि नृणामिहाद्यौ ॥ (च.चि.26/32)

व्यायाम – तीक्ष्णौषध सेवन – रूक्ष मद्यपान – अतिमैथुन – नित्य द्रुत पृष्ठयान – आनूप मांस,
मत्स्य अतिसेवन – अध्यशन – अजीर्ण etc.

सम्प्राप्ति →



मूत्रकृच्छ्र भेद → 8

वातज – पित्तज – कफज – सन्निपातज – अभिघातज – शकृतज – अश्मरीज / शर्कराज –
शुक्रज मूत्रकृच्छ्र

क्र.	मूत्रकृच्छ्र भेद	लक्षण
1	वातज मूत्रकृच्छ्र	Elimination of little quantity of urine with difficulty. Pain in groin, scrotum, penis and urinary bladder.
2	पित्तज मूत्रकृच्छ्र	Elimination of yellowish, blood-stained & hot urine. Burning sensation in scrotum, penis and bladder.

3	कफज मूत्रकृच्छ्र	Elimination of pale, unctuous and slimy urine. Heaviness & swelling in scrotum, penis, bladder.
4	सन्निपातज मूत्रकृच्छ्र	Mixed symptoms with severe pain & burning while micturition.
5	अभिघातज मूत्रकृच्छ्र	Due to injury to urinary tract, or due to foreign bodies, acute pain in bladder with distension and heaviness.
6	शकृतज मूत्रकृच्छ्र	Due to retention of faeces or constipation, the apana-vayu gets aggravated and results in distension, pain and retention of urine.
7	अश्मरीज मूत्रकृच्छ्र	Aggravated vata dries up the pitta & kapha in the basti pradesha to form stones (calculus). Elimination of little quantity of urine with difficulty. Pain in groin, scrotum, penis and urinary bladder. When stones injure the bladder, urine with blood.
	शर्कराज मूत्रकृच्छ्र	When ashmari gets disintegrated into small particles by vata and comes out through urinary passage, it is known as sharkara.
8	शुक्रज मूत्रकृच्छ्र	Due to suppression of shukra-vega, pain & swelling in scrotum, penis and urinary bladder.

मूत्रकृच्छ्र चिकित्सा सिद्धान्त →

→ निदान परिवर्जन

→ अभ्यंग (स्नेहन) & स्वेदन (उपनाह/अवगाह/परिषेक)

→ अनुवासन & निरुह बस्ति – उत्तर बस्ति

→ मूत्रल द्रव्य प्रयोग (e.g. इक्षुरस, गोक्षुर, पुनर्नवा, उशीर, शतावरी, तृणपंचमूल, रक्तचन्दन etc.)

वातज मूत्रकृच्छ्र चिकित्सा →

अभ्यंजन स्नेह निरूहवस्ति स्नेहोपनाहोत्तरवस्ति सेकान् ।
स्थिरादिभिर्वातहरैश्च सिद्धान् दद्याद्रसांश्चानिलमूत्रकृच्छ्रे ॥ (च.चि.26/45)

- अभ्यंग with वातनाशक तैल
- अनुवासन & निरूह वस्ति
- स्निग्ध उपनाह
- स्निग्ध उत्तरवस्ति
- परिषेक with वातनाशक तैल or क्वाथ
- शालिपर्णि आदि लघुपंचमूल क्वाथ + मांसरस प्रयोग

पित्तज मूत्रकृच्छ्र चिकित्सा →

सेकावगाहाः शिशिराः प्रदेहा ग्रैष्मो विधिर्बस्तिपयोविरेका ।
द्राक्षाविदारिक्षुरसैर्वृतैश्च कृच्छ्रेषु पित्तप्रभवेषु कार्याः ॥ (च.चि.26/49)

- परिषेक with शीतद्रव्य-क्वाथ
- अवगाह with शीत जल
- शीत प्रदेह
- ग्रीष्म ऋतुचर्या पालन
- बस्ति – विरेचन – दुग्धपान
- द्राक्षा – विदारीकंद – इक्षुरस – घृत सेवन

कफज मूत्रकृच्छ्र चिकित्सा →

क्षारोष्णतीक्ष्णौषधमन्नपानं स्वेदो यवान्नं वमनं निरूहाः ।
तक्रं सतिक्तौषधसिद्धतैलमभ्यंगपानं कफमूत्रकृच्छ्रे ॥ (च.चि.26/54)

- क्षार-उष्ण-तीक्ष्ण औषध & आहार
- स्वेदन – वमन – निरूह वस्ति
- तक्र प्रयोग & अभ्यंग with तित्कद्रव्य सिद्ध तैल

सन्निपातज मूत्रकृच्छ्र चिकित्सा →

सर्वत्रिदोषप्रभवे तु वायोः स्थानानुपूर्व्या प्रसमीक्ष्य कार्यम् ।
त्रिभ्योऽधिके प्राग्वमनं कफे स्यात् पित्ते विरेकः पवने तु वस्तिः ॥ (च.चि.26/58)

- वातस्थानानुसार चिकित्सा
(First treat वातस्थान, then पित्तस्थान, and then कफस्थान)

- In कफाधिक्य त्रिदोष प्रकोप → वमन
- In पित्ताधिक्य त्रिदोष प्रकोप → विरेचन
- In वाताधिक्य त्रिदोष प्रकोप → बस्ति

अश्मरीज & शर्कराज मूत्रकृच्छ्र चिकित्सा →

क्रिया हिता साऽश्मरी शर्कराभ्यां कृच्छ्रे यथैवह कफानिलाभ्याम् । (च.चि.26/59)

- अश्मरी भेदन-पातन क्रिया
- कफज & वातज मूत्रकृच्छ्रवत चिकित्सा

रक्तज मूत्रकृच्छ्र चिकित्सा →

पिबेत् सिताक्षौद्रयुतानि खादेदिक्षुं विदारी त्रपुपाणि चैव । (च.चि.26/73)

- मधुरवर्ग द्रव्य सिद्ध क्वाथ + मधु + मिश्री
- इक्षुरस – विदारीकंद – त्रपुष बीज सेवन

शुक्रज मूत्रकृच्छ्र चिकित्सा →

रेतोऽभिघातप्रभवे तु कृच्छ्रे समीक्ष्य दोषं प्रतिकर्म कुर्यात् । (च.चि.26/68)

- प्रधान दोषानुसार चिकित्सा

शमनौषधि →

1. रसौषधि → मात्रा = 125 - 250 mg. अनुपान = मधु / जल

चन्द्रकला रस – श्वेत पर्पटी - मूत्रकृच्छ्रान्तक रस

2. वटी → मात्रा = 250 - 500 mg. अनुपान = ऊष्णोदक / मधु / घृत

चन्द्रप्रभा वटी – हिंवादि वटी – गोक्षुरादि गुग्गुलु

3. चूर्ण → मात्रा = 3 - 6 g. अनुपान = ऊष्णोदक / मधु

यवक्षार चूर्ण – एर्वारुबीज योग – एलादि चूर्ण

4. क्वाथ/आसव/अरिष्ट → मात्रा = 20 - 40 ml. अनुपान = जल

गोक्षुरादि क्वाथ – पुनर्नवादि क्वाथ – तृणपंचमूलादि क्वाथ – पुनर्नवासव

5. घृत → मात्रा = 10 - 30 ml. अनुपान = उष्ण जल / दुग्ध

श्वदंष्ट्रा घृत – त्रिकण्टकाद्य घृत – शतावरादि घृत

अश्मरी (Urolithiasis)

निरुक्ति →

अश्मानं राति इति अश्मरी ।

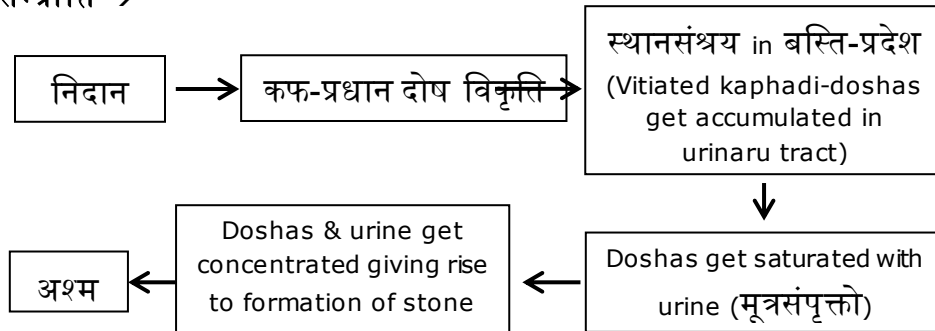
Ashmari is so named because of its similarity with stone. It is hard like a stone that can be correlated with urinary lithiasis or calculus.

निदान →

तत्रासंशोधनशीलस्यापथ्यकारिणः प्रकुपितः क्षेष्मा
मूत्रसंपृक्तोऽनुप्रविश्य बस्तिमश्मरीं जनयति ॥ (सु.नि.3/4)

- असंशोधनशील →
Not cleansing the body → unhygiene or uncleanliness, or not undergoing purificatory therapies like panchakarma etc.
- अपथ्य कारिणः →
(Unwholesome food intake & improper activities)
→ कफवर्धक आहार-विहार
→ दुष्ट & संकीर्ण अन्नपान, अल्प जलपान
→ मूत्रादि वेगधारण
→ अव्यायाम or अतिव्यायाम
→ दिवास्वप्न etc.

सम्प्राप्ति →



भेद (Types) → 4

वातज – पित्तज – कफज – शुक्रज अश्मरी

सामान्य लक्षण →

- Pain in lower abdomen & penis
- Dribbling urination
- Haematuria
- Turbid urine
- Pain & burning get relieved after passage of calculus
- Other symptoms are according to predominant dosha

अश्मरी चिकित्सा सूत्र →

क्रिया हिता साऽश्मरी शर्कराभ्यां कृच्छे यथैवह कफानिलाभ्याम् । (च.चि.26/59)

- अश्मरी भेदन-पातन क्रिया (पाषाणभेदादि अश्मरीघ्न द्रव्य प्रयोग)
- कफज & वातज मूत्रकृच्छ्रवत चिकित्सा

शमनौषधि →

पाषाण भिन्न रस – पाषाण वज्र रस – वरुणाद्य लौह – चन्द्रप्रभा वटी – गोक्षुरादि गुग्गुलु – गोक्षुरादि चूर्ण – अश्मरीभेदन योग – गोक्षुरादि क्वाथ – पुनर्नवादि क्वाथ – तृणपंचमूलादि क्वाथ – पुनर्नवासव – वरुणक गुड – कुलत्थाद्य घृत
पाषाणभेदादि-चूर्ण → पाषाणभेद+गोक्षुर+पाठा+हरीतकी+त्रिकटु+शटी+दन्तीमूल+हिंस्रा+शितिवारक+एवार्क बीज+त्रपुष बीज+हिंस्र+बृहती+कण्टकारी+हपुषा+वचा

पथ्य आहार-विहार (for मूत्राघात, मूत्रकृच्छ्र, अश्मरी) → पुराण शालि – मुद्ग – शर्करा – मिश्री – तक्र – गोदुग्ध – पटोल – त्रपुष – एवार्क – खर्जूर – नारिकेल – आमलकी – जांगल मांसरस – स्वेदन – विरेचन – उत्तरबस्ति – अति अम्बुपान

अपथ्य आहार-विहार (for मूत्राघात, मूत्रकृच्छ्र, अश्मरी) → उष्ण तीक्ष्ण विदाही अन्नपान – लवण – आर्द्रक – तिल – माष – विषमाशन – मद्यपान – अति श्रम – अति मैथुन

Cystitis

Inflammation of the urinary bladder is mostly caused by a bacterial infection, and is also known as urinary tract infection (UTI).

Cystitis is much more common in women than men, probably because urethra of women is much shorter and the anus is closer to the urethral opening.

Etiology →

- Ascending infection from faecal and perineum through urethral route.
- Causative organisms are E.coli, gram -ve bacteria like klebsiella & proteus species.

Clinical Features →

- Fever with chills, pain, and burning micturition
- Frequency & urgency of urination
- Cloudy or strong-smelling urine
- Blood in urine
- Suprapubic tenderness

Investigations →

Blood – CBC (leucocytosis), Raised ESR

Urine – (In microscopic urine examination → more than 5 WBCs/hpf in females and 2-3 WBCs/hpf in males)

Urine culture to identify causative organism

Treatment →

- Depending upon culture & sensitivity report, antibiotic therapy for 10 days. Commonly used antibiotics in cystitis →
 - Trimethoprim + sulfamethoxazole (Bactrim)
 - Ciprofloxacin / Levofloxacin / Cephalexin
 - Metronidazole etc.
- Symptomatic treatment with → analgesics, antipyretics, alkalinizing agent (NaHCO_3) & more water intake.

Nephritis

Nephritis is inflammation of the kidneys and may involve the glomeruli, tubules, or interstitial tissue surrounding the glomeruli and tubules.

Etiology →

Nephritis is often caused by infections, and toxins, but is most commonly caused by autoimmune disorders that affect the major organs like kidneys.

- **Pyelonephritis** is inflammation that results from a urinary tract infection that reaches the renal pelvis of the kidney.
- **Glomerulonephritis** is inflammation of the glomeruli. The main cause of this type of kidney infection is unknown. However, some conditions may encourage an infection, including: (i) autoimmune disease, (ii) a history of cancer, and (iii) an abscess that breaks and travels to kidneys through blood.

- **Interstitial nephritis** (or tubulo-interstitial nephritis) is inflammation of the spaces between renal tubules. This type often results from an allergic reaction to a medication or antibiotic. Taking medications for long periods of time may damage the tissues of the kidneys and lead to interstitial nephritis.

Clinical Features →

- Pain in the pelvis, abdomen and lumbar region
- Painful and/or burning urination
- A frequent need to urinate
- Cloudy urine
- Blood or pus in the urine
- Swelling of the body, commonly in the face, legs, and feet
- Vomiting
- Fever
- Hypertension

Investigations →

Blood – CBC, Serum Creatinine, Uric acid etc.

Urine – Routine & Microscopic (blood and pus may be present)

USG KUB, CT scan, Renal biopsy etc.

Treatment →

Treatment (or management) of nephritis depends on what has provoked the inflammation of the kidney(s). e.g. in infection, antibiotics are used, while in the case of lupus nephritis, hydroxychloroquine could be used.

- To reduce inflammation – Corticosteroids can be used.
- To reduce hypertension – ACE inhibitors such as Captopril, Lisinopril, Ramipril etc. and/or Angiotensin receptor blockers such as Valsartan, Telmisartan, Olmesartan etc. can be used.
- Diet should be low in certain electrolytes like potassium and sodium. Bananas, potatoes, peas, mushrooms, pumpkins, spinach etc. are rich in potassium those should be avoided, or the vegetables can be used by soaking in water for few hours and then drained which is known as leaching. Less or no salt or low sodium-intake is beneficial in hypertension.
- Intake of plenty of water.
- **Dialysis** → Dialysis is a treatment that filters and purifies the blood using a machine. This helps keep the fluids and electrolytes in balance when the kidneys can't do their function properly.

Nephrotic syndrome

Nephrotic syndrome is a collection of symptoms due to kidney damage. This includes → protein in the urine (proteinuria), low blood albumin levels (hypoalbuminemia – a type of hypoproteinemia), high blood lipids (hyperlipidemia), and significant swelling (oedema). Other symptoms may include weight gain, feeling tired, and foamy urine. Complications may include blood clots (thrombosis), infections (sepsis), and high blood pressure (hypertension).

Nephrotic syndrome is characterized by large amounts of proteinuria (>3.5 g per 1.73 m² body surface area per day, or > 40 mg per square meter body surface area per hour in children), hypoalbuminemia (< 2.5 g/dl), hyperlipidaemia, and edema that begins in the face.

Etiology →

Nephrotic syndrome has many causes and may either be the result of a glomerular disease that can be either limited to the kidney, called primary nephrotic syndrome (primary glomerulonephrosis), or a condition that affects the kidney and other parts of the body, called secondary nephrotic syndrome.

Many diseases and conditions can cause glomerular damage and lead to nephrotic syndrome, including:

- **Diabetic kidney disease.** Diabetes can lead to kidney damage (diabetic nephropathy) that affects the glomeruli.
- **Minimal change disease.** This is the most common cause of nephrotic syndrome in children. Minimal change disease results in abnormal kidney function, but when the kidney tissue is examined under a microscope, it appears normal or nearly normal. The cause of the abnormal function typically can't be determined.
- **Focal segmental glomerulosclerosis.** Characterized by scattered scarring of some of the glomeruli, this condition may result from another disease or a genetic defect or occur for no known reason.
- **Membranous nephropathy.** This kidney disorder is the result of thickening membranes within the glomeruli. The exact cause of the

thickening isn't known, but it's sometimes associated with other medical conditions, such as hepatitis B, malaria, lupus and cancer.

- **Systemic lupus erythematosus.** This chronic inflammatory disease can lead to serious kidney damage.
- **Amyloidosis.** This disorder occurs when substances called amyloid proteins accumulate in the organs. Amyloid buildup often affects the kidneys, damaging their filtering system.
- **Blood clot in a kidney vein.** Renal vein thrombosis, which occurs when a blood clot blocks a vein connected to the kidney.

Risk factors →

Factors that can increase your risk of nephrotic syndrome include:

- **Medical conditions** that can damage the kidneys, such as diabetes, lupus, amyloidosis and other kidney diseases.
- **Certain medications.** Examples of medications that can cause nephrotic syndrome include nonsteroidal anti-inflammatory drugs and drugs used to fight infections.
- **Certain infections.** Examples of infections that increase the risk of nephrotic syndrome include HIV, hepatitis B, hepatitis C and malaria.

Treatment →

Treatment for nephrotic syndrome involves treating any underlying medical condition that may be causing nephrotic syndrome. Symptomatic treatment can be given such as management of hypertension with antihypertensives, for hyperlipidemia – statins, for oedema – diuretics, and for thrombosis – blood thinners or anticoagulants etc. Diet & life style modifications are important.

Renal failure

Kidney failure, also known as end-stage kidney disease, is a medical condition in which the kidneys no longer work. It is divided into acute kidney failure (cases that develop rapidly) and chronic kidney failure (those that are long term).

Etiology →

Kidney failure occurs when the kidneys lose the ability to filter waste from the blood sufficiently. Many factors can interfere with kidney health and function, such as:

- Toxic exposure to environmental pollutants or certain medications
- Certain acute and chronic diseases (*mentioned in nephrotic syndrome)
- Severe dehydration
- Kidney trauma

Symptoms →

Symptoms of kidney failure include the following:

- High levels of urea in the blood, which can result in:
 - Vomiting or diarrhea (or both) which may lead to dehydration
 - Weight loss
 - Nocturnal urination & More frequent urination, or in greater amounts than usual, with pale urine
 - Less frequent urination, or in smaller amounts than usual, with dark coloured urine
 - Blood in the urine

- A buildup of phosphates in the blood that diseased kidneys cannot filter out may cause:
 - Itching
 - Bone damage
 - Nonunion in broken bones
 - Muscle cramps (caused by low levels of calcium which can be associated with hyperphosphatemia)
- A buildup of potassium in the blood that diseased kidneys cannot filter out (called hyperkalemia) may cause:
 - Abnormal heart rhythms (arrhythmias)
 - Muscle paralysis
- Failure of kidneys to remove excess fluid may cause:
 - Swelling of the legs, ankles, feet, face, or hands
 - Shortness of breath due to extra fluid on the lungs (may also be caused by anemia)
- Healthy kidneys produce the hormone erythropoietin that stimulates the bone marrow to make oxygen-carrying red blood cells. As the kidneys fail, they produce less erythropoietin, resulting in decreased production of red blood cells to replace the natural breakdown of old red blood cells. As a result, the blood carries less hemoglobin, a condition known as anemia. This can result in:
 - Feeling tired or weak
 - Memory problems & Difficulty concentrating
 - Dizziness
 - Low blood pressure

- Normally, proteins are too large to pass through the kidneys, however, they are able to pass through when the glomeruli are damaged. This does not cause symptoms until extensive kidney damage has occurred, after which symptoms include:
 - Foamy or bubbly urine
 - Swelling in the hands, feet, abdomen, or face
- Other symptoms include:
 - Appetite loss, a bad taste in the mouth
 - Difficulty sleeping
 - Darkening of the skin
 - Excess protein in the blood
 - With high doses of penicillin, people with kidney failure may experience seizures

Investigations →

- Serum Creatinine, Uric acid
- Anuria or Oliguria, Proteinuria, Glycosuria
- USG, CT scan, MRI
- Renal biopsy

Treatment →

Treatment of acute disease typically depends on the underlying cause.

Treatment of chronic disease may include:

1. Hemodialysis,
2. Peritoneal dialysis, or
3. A kidney transplant.

Hemodialysis uses a machine to filter the blood outside the body.

In **peritoneal dialysis** specific fluid is placed into the abdominal cavity and then drained, with this process being repeated multiple times per day.

Kidney transplantation involves surgically placing a kidney from someone else and then taking immunosuppressant medication to prevent rejection.

Other recommended measures from chronic disease include staying active and specific dietary changes

Ayurvedic medications in Renal-insufficiency →

- चन्द्रकला रस
- श्वेत पर्पटी
- चन्द्रप्रभा वटी
- गोक्षुरादि गुग्गुलु
- यवक्षार चूर्ण
- एवर्बुबीज योग
- गोक्षुरादि क्वाथ
- पुनर्नवादि क्वाथ
- तृणपंचमूलादि क्वाथ
- पुनर्नवासव
- त्रिकण्टकाद्य घृत
- शतावरादि घृत

पथ्य आहार → पुराण शालि – मुद्ग – शर्करा – मिश्री – तक्र – गोदुग्ध – पटोल – त्रपुष – एवर्बु – खर्जूर – नारिकेल – आमलकी – जांगल मांसरस – स्वेदन – विरेचन – उत्तरबस्ति

अपथ्य आहार-विहार → उष्ण तीक्ष्ण विदाही अन्नपान – लवण – आर्द्रक – तिल – माष – विषमाशन – मद्यपान – अति श्रम – अति मैथुन

Benign Prostatic Hyperplasia (BPH)

Benign prostatic hyperplasia (BPH), also called prostate enlargement, is a non-cancerous increase in size of the prostate.

It usually occurs in men over 50 years of age.

Etiology →

The exact cause is unknown, but some hypothetical theories are there to explain etiology of BPH →

1. The hormone theory → Due to a disturbance of the ratio and quantity of the circulating androgens and estrogens.
2. The neoplastic theory → Proliferation of all elements (Fibrous, muscular & glandular) of prostate in old age.

Risk factors include a family history, obesity, type 2 diabetes, not enough exercise, and erectile dysfunction. Medications like pseudoephedrine, anticholinergics, and calcium channel blockers may worsen symptoms.

Clinical Features →

- Frequency – Urgency – Hesitancy in urination
- Difficulty in urination
- Retention of urine
- Hematuria
- Hydronephrosis
- Renal insufficiency

Investigations →

Blood routine, serum creatinine, urea etc.

Urine – culture & sensitivity test, urea, blood, pus etc.

IVP, USG, Cystourethroscopy etc.

Treatment →

1. Medical Rx →

- α blockers →
 - Tab. Terazosin 2-5 mg OD
 - Tab. Alfuzosin 10 mg OD
 - Tab. Tamsulosin 0.2 to 0.4 mg OD
 - Tab. Tamsulosin + Dutasteride 0.5 mg
- 5- α reductase inhibitors (Anti-androgenic agent) →
 - Tab. Finasteride 5 mg OD
 - Tab. Dutasteride 0.5 mg OD
- Tadalafil (often to treat erectile dysfunction, can also treat BPH)

2. Surgical Rx →

- Transurethral resection of prostate (TURP)
- Suprapubic prostatectomy
- Retropubic prostatectomy
- Perineal prostatectomy
- Preliminary vasectomy

Diseases of पुरीषवह स्रोतस

अतिसार (Diarrhoea)

निरुक्ति →

गुदेन बहुद्रवसरणं अतिसारः ।

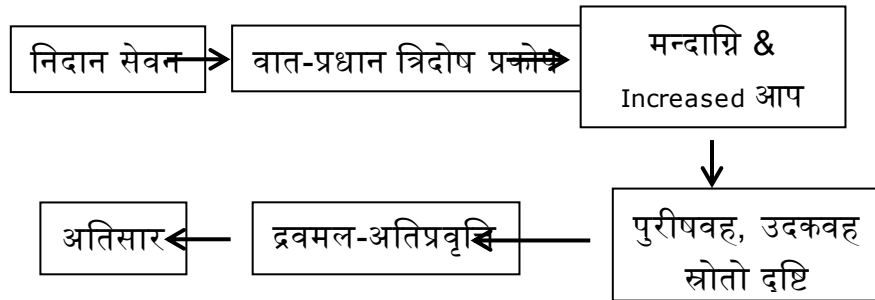
Loose stool or passing excessive watery stool through guda-marga is called Atisara.

सामान्य निदान → गुरु अतिसिग्ध-उष्ण-द्रव-स्थूल-शीत आहार – अजीर्ण – अध्यशन – विषमाशन – विरुद्धाहार – दूषित जल – मद्य – विष – भय – शोक – वेगधारण – मिथ्यायोग स्नेहन स्वेदन पंचकर्म – ऋतुविपर्यय – जलक्रीडा – कृमि etc.

विशिष्ट निदान →

1. वातज → वातप्रकृति – अतिवायुसेवन – व्यायाम – रूक्ष आहारविहार – वेगधारण
2. पित्तज → पित्तप्रकृति – कटु अम्ल लवण उष्ण तीक्ष्ण आहार – आतपसेवन – क्रोध
3. कफज → कफप्रकृति – गुरुमधुरस्निग्ध आहार – अतिभोजन – अचिन्ता – दिवास्वप्न
4. सन्निपातज → वात-पित्त-कफ त्रिदोषप्रकोपक निदान
5. & 6. आगन्तुज → शोकज – भयज

सम्प्राप्ति (Pathogenesis) →



पूर्वरूप → हृदय-नाभि-गुद-उदर-कुक्षि तोद (सूचीभेदनवत पीडा) – गात्रावसाद – अपानवायु अवरोध – विट्संग (मल-प्रवृत्ति अभाव) – आध्मान – अविपाक etc.

विशेष लक्षण →

क्र.	अतिसार-भेद	लक्षण
1	वातज अतिसार	अरुण फेनिल रूक्ष अल्पमल – सरूक् – सशब्द
2	पित्तज अतिसार	पीत नील कृष्ण दुर्गन्धीमल – तृष्णा – दाह – मूर्च्छा – गुदपाक
3	कफज अतिसार	स्निग्ध श्वेत श्लेष्मयुक्तमल – गुरुदरगुदबस्तिवक्षणादेश – आलस्य
4	सन्निपातज अतिसार	मांसधावनसन्निकाश रक्त कृष्ण हरितपीत मल – त्रिदोष लक्षण
5	शोकज अतिसार	वातज अतिसार लक्षण
6	भयज अतिसार	वातज अतिसार लक्षण

साध्यासाध्यता →

- वातज, पित्तज & कफज अतिसार are सुखसाध्य, while सन्निपातज, शोकज & भयज अतिसार are कृच्छ्रसाध्य.
- असाध्य अतिसार लक्षण →
 - पक्वजम्बू सवर्ण / यकृतखण्ड सवर्ण, कृष्ण-लोहित & स्निग्ध मल
 - घृत, तैल, वसा, मज्जा, वेशवार, दुग्ध, दधि, मांसधावन सम्मित मल
 - शवगन्धि / अत्यधिक दुर्गन्धयुक्त मल
 - सन्निपातज अतिसार (Acc. to आचार्य चरक)
 - उपद्रवयुक्त अतिसार

उपद्रव →

शोथ – शूल – ज्वर – तृष्णा – मूर्च्छा – कास – श्वास – हिक्का – अरुचि – छर्दि – मूत्राघात

अतिसार चिकित्सा सूत्र →

आमपक्व क्रमं हित्वा नातिसारे क्रिया हितः ।

अतः सर्वेतिसारास्तु ज्ञेयाः पक्वामलक्षणैः ॥ (सु.उ.40/24)

First, find out the stage of अतिसार, whether it is →

- आमातिसार (undigested food passed in stool), or
- पक्वातिसार (digested stool).

Without knowing the 'आम-पक्व क्रम' the physician cannot treat the अतिसार.

साम-निराम मल परीक्षा → When the stool is दुर्गन्धित, पिच्छिल, and अपक्वान्नयुक्त, & sinking in the water (जल निमज्जन) it should be considered as साम मल. निराम मल will not sink in the water and is devoid of above features.

➤ आम्रातिसार चिकित्सा →

“न तु संग्रहणं देयं पूर्वमाम्रातिसारिणे” (च.चि.19/15)

Do not try to stop diarrhoea when undigested food is being expelled (i.e. संग्राही & स्तम्भक औषध or कषाय रस द्रव्य are to be avoided, initially).

If stambhana chikitsa is done when ama is still inside, it can cause various diseases or complications like → दण्डालसक, आध्मान, ग्रहणी, अर्श, शोथ, पाण्डु, प्लीहा, कुष्ठ, गुल्म, उदर, and ज्वर.

आम्रातिसार चिकित्सा सिद्धान्त →

→ लंघन – पाचन

Fasting (उपवास) is the best method for आम-पाचन. If needed, पाचनीयद्रव्य सिद्ध लघु-आहार like मण्ड, पेया etc. can be given.

→ If ama still remains, administer हरीतकी or ईसबगोल or एरण्ड तैल with उष्ण जल to expel remaining dosha.

→ प्रमथ्यां मध्यदोषाणां दद्याद्दीपनपाचनीम् ।
लंघनं चाल्पदोषाणां प्रशस्तम्रातिसारिणाम् ॥ (च.चि.19/19)

- In अल्पदोषावस्था → लंघन (उपवास)
- In मध्यदोषावस्था → दीपन-पाचनीय प्रमथ्या
- In बहुदोषावस्था → विरेचन

औषध योग for आमपाचन →

प्रमथ्या → पिप्पल्यादि प्रमथ्या – ह्रीबेरादि प्रमथ्या – पृश्निपर्ण्यादि प्रमथ्या

क्वाथ → धान्यपंचक क्वाथ – नागरादि क्वाथ

चूर्ण → शुण्ठ्यादि चूर्ण – त्र्यूषणादि चूर्ण – लवण भास्कर चूर्ण

वटी → रसोनादि वटी – संजीवनी वटी

➤ पक्वातिसार चिकित्सा →

In पक्वातिसार, रक्तातिसार, and if even after आम-पाचन patient still passes loose stool due to disturbed ग्रहणी → स्तम्भन द्रव्य should be used.

e.g. कुटज – बिल्व – लोध्र – जम्बू – शाल्मली – शल्लकी – धातकी – अहिफेन

औषध योग →

गंगाधर चूर्ण – नारायण चूर्ण – कुटज घन वटी – बिल्वादि क्वाथ – कुटजाष्टकावलेह – कुटजारिष्ट – अहिफेनासव – कर्पूर रस etc.

	पथ्य	अपथ्य
आहार	पुराण शालि, विलेपी, लाजमण्ड, मसूर, आढकी, मुद्ग, मधु, जम्बू, दाडिम, बिल्व, पटोल, तिन्दुक, जीरक, धान्यक, अपक्व कदली, श्लेष्मान्तक, कपित्थ, तक्र, अजादुग्ध, कृशरा, षडंगपानीय etc.	सतीन, कलाय, माष, निष्पाव, इक्षुरस, एवार्सक, त्रपुष, अति जलपान, गुरु-स्निग्ध आहार, हरित शाक, पूग, बदर, ताम्बूल, मद्य, द्राक्षा, दधि, पिष्टान्न, चणक etc.
विहार	लंघन (उपवास), निद्रा, विश्राम etc.	स्नान, अवगाह, अभ्यंग, नस्य कर्म, व्यायाम, व्यवाय, आतप सेवन, दिवास्वप्न, स्वेदन कर्म, धूम्रपान, रात्रिजागरण, वेगधारण, क्रोध etc.

In गुदभ्रंश (prolapse of rectum) →

- चांगेरी घृत प्रयोग
- मूषक तैल प्रयोग
- स्नेह बस्ति (अनुवासन)
- पिचु प्रयोग after manual replacement of rectum

वातातिसार चिकित्सा →

- दीपन – पाचन
- वातानुलोमन
- पंचमूल सिद्ध तक्र प्रयोग
- पथ्यादि क्वाथ – वचादि क्वाथ

पित्तातिसार चिकित्सा →

- लंघन – पाचन
- तृष्णा निग्रहण with मुस्ता-पर्पटक-उशीर-सारिवा-चन्दन-किराततिक्तक-उदीच्य सिद्ध जल prepared by षडंग पानीय-विधि
- In strong patients- विरेचन to expel dosha
- अजादुग्ध प्रयोग
- पिच्छा बस्ति

कफातिसार चिकित्सा →

- लंघन – पाचन
- Eating अपक्व बिल्व फल
- कपित्थादि चूर्ण (कपित्थ फल चूर्ण with त्रिकटु, मधु and शर्करा)
- पिच्छा बस्ति (best in pittaja & kaphaja atisara)

रक्तातिसार चिकित्सा →

- तृष्णा निग्रहण with अजादुग्ध + मधु + शर्करा
- पित्तज अतिसार चिकित्सा
- शतावरी कल्क + दुग्ध / बिल्व + गुड
- कुटज क्वाथ – शतावरी घृत – पिच्छा बस्ति

सन्निपातज अतिसार चिकित्सा →

- All the V-P-K measures.

भयज & शोकज अतिसार चिकित्सा →

- वातहर चिकित्सा
- Counseling & making the person happy

पिच्छा बस्ति →

Green stalks of शाल्मली is covered with green कुश, and coated with black mud. This पिण्ड is placed over the cow-dung fire. After the mud is dried up, the stalks of शाल्मली is removed. This is triturated in खल्व-यन्त्र, and then 9 मुष्टि (1 पल) of शाल्मली is mixed with 1 प्रस्थ of शृत दुग्ध (boiled milk) and filtered.

घृत, तैल & यष्टिमधु चूर्ण are added to this milk in adequate quantity, and this mixture is used for enema (पिच्छाबस्ति) after proper oleation (स्नेहन). After coming out the enema, patient should take bath and eat rice with milk or meat soup.

Indications: पित्तज& कफज अतिसार – ग्रहणी – ज्वर – अतियोग of निरूह बस्ति etc.

प्रवाहिका (Dysentery)

निरुक्ति →

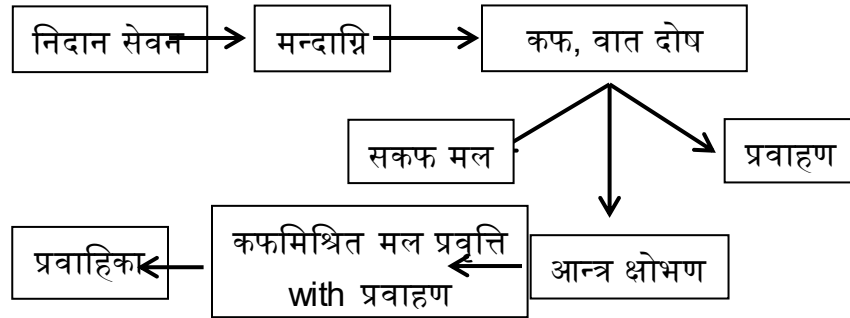
“प्रवाहमाणस्य प्रवाहिका”

When less quantity of कफयुक्त मल is passed frequently, by प्रवाहण (straining) and with difficulty, the condition is called प्रवाहिका. The cardinal symptom (प्रत्यात्म लक्षण) of प्रवाहिका is “प्रवाहण”.

निदान →

- सर्व अतिसार-निदान
- अहित आहार-सेवन
- वात-कफ दोष वर्धक आहार-विहार सेवन
- अत्यन्त उष्ण-तीक्ष्ण काल (ऋतु)
- तिलपिष्ट, मिष्ठान्न सेवन

सम्प्राप्ति →



पूर्वरूप →

हृदय-नाभि-गुद-उदर-कुक्षि तोद (सूचीभेदनवत पीडा) – गात्रावसाद – अपानवायु अवरोध – विट्संग (मल-प्रवृत्ति अभाव) – आध्मान – अविपाक etc.

सामान्य लक्षण →

- मुहुर्मुहु कफमिश्रित अल्प मल प्रवृत्ति (Mucus in stool. And frequency of defecation is more but quantity of faeces is less)
- प्रवाहण (Tenesmus)

प्रवाहिका चिकित्सा →

- प्रवाहिका should be treated like अतिसार, by considering its आम- and पक्कावस्था.
- In आमावस्था → लंघन – पाचन
If Pravahika still remains after Langhana-pachana → पाचनीय द्रव्य सिद्ध घृत or तैल सेवन, and पिच्छाबस्ति प्रयोग
स्तम्भन should not be used in आमावस्था. If आम is more, then it should be eliminated through मृदुविरेचन with दुग्ध or एरण्ड तैल etc.
- In पक्कावस्था → स्तम्भन द्रव्य प्रयोग
- In रक्तज प्रवाहिका → रक्तस्तम्भक चिकित्सा & पिच्छा बस्ति
- For pacification of प्रकुपित वात → अनुवासन बस्ति

शमनौषधि →

रामबाण रस – पंचामृत पर्पटी – कुटज पर्पटी – कुटज घन वटी – संजीवनी वटी – बृहद् गंगाधर चूर्ण – नारायण चूर्ण – कुटजारिष्ट – मुस्तकाद्यरिष्ट etc.

पथ्य आहार-विहार →

पुराण शालि – मण्ड – विलेपी – मुद्ग – मसूर – मधु – जम्बू – दाडिम – बिल्व – तिन्दुक – जीरक – धान्यक – तक्र – अजादुग्ध – कृशरा – षडंगपानीय – उपवास – विश्राम – निद्रा

अपथ्य आहार-विहार →

माष – सतीन – निष्पाव – इक्षुरस – एर्वारु – त्रपुष – अति अम्बुपान – गुरु स्निग्ध आहार – स्नान – व्यायाम – वेगधारण

अर्श (Hemorrhoids)

निरुक्ति →

अरिवत् प्राणान् शृणाति इति अर्थः । (मधुकोष)

Arsha produces severe pain & discomfort just like an enemy.

व्याख्या →

Abnormal growth like sprouts of muscles (मांसांकुर) that obstructs the passage of the rectum and anal canal (गुदमार्ग) and causing pain & discomfort like an enemy is called “अर्श”.

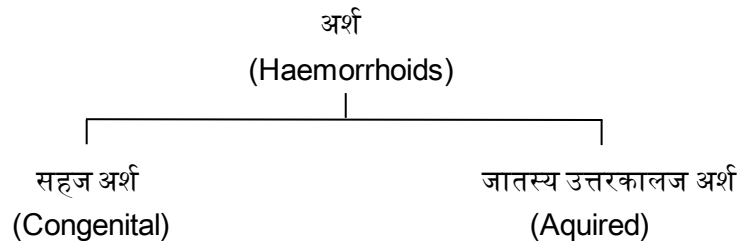
The term “अर्श” is also used to name the abnormal benign growths like polyps & warts. The sites other than गुदमार्ग are – त्वक्, नासा, कर्ण, मुख, तालु, गल, अक्षिवर्त्म, मूत्रेन्द्रिय, योनिमार्ग etc. But Acharyas has called the मांसांकुर in the गुदमार्ग as अर्श.

अर्श स्थान (Sites) →

There are 3 गुदवलि (three ano-rectal folds) in the space (अवकाश) of 4½ अंगुल of गुद-प्रदेश. These three gudavalis are the site of formation of arsha.

अर्श भेद (Types) →

(A) According to Charaka →



Nidana of Sahaja arsha →

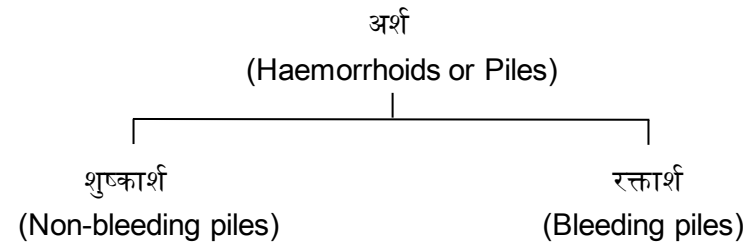
बीजविकृति (शुक्र-शोणित दुष्टी or विकृति) may take place due to following two factors →

- (i) मिथ्या आहार-विहार of मातृ-पितृ
- (ii) पाप or दुष्कर्म of पूर्वजन्म

Nidana of Uttarakalaja arsha →

- (i) Weakness of digestive power (मन्दाग्नि) due to causes of dosha-prakopa (दोषप्रकोपक हेतु)
- (ii) Accumulation of waste (मलसंचय)
- (iii) Chronic constipation (मल विबद्धता)
- (iv) Improper and prolonged sitting, and riding, driving or travelling on uneven paths
- (v) Complications of improper basti-karma
- (vi) Complications of ज्वर – गुल्म – अतिसार – ग्रहणी – शोफ – पाण्डु & वेगधारण
- (vii) निदानार्थकर रोग → अर्श-अतिसार-ग्रहणी are often of mutual causation, and can also cause each other

(B) According to bleeding nature, arsha can be classified as follows →



This classification helps in the diagnosis and treatment of the arsha, as the treatment principles of non-bleeding and bleeding piles are different.

(C) According to Sushruta → Based on predominant dosha → 6

1. वातज अर्श →

आकृति (shape) – Like सर्पप

लक्षण (symptoms) – Blackish growth, pointed tip and irregular. Patient passes hard stool with difficulty and suffers from cutting type of pain (कर्तन वेदना) in गुद, बस्ति, नाभि, कटि, पृष्ठ, and पार्श्व etc.

2. पित्तज अर्श →

आकृति – Like तिंदुक, गुंजा, बिम्बी, बदर etc.

लक्षण – Reddish, yellowish, bluish, blackish growth, bleeding & yellow discharge. Other associated symptoms are ज्वर, दाह, तृष्णा, मूर्च्छा etc.

3. कफज अर्श →

आकृति – Like गोस्तन, अंगुष्ठ etc.

लक्षण – Pale / whitish growth, moist with itching, continuous discharge of whitish colour or mixed with blood. शोफ, शीतज्वर, अरोचक, अविपाक, and गुरुता in शिरभाग etc. may occur as उपद्रव (complications).

4. रक्तज अर्श →

पित्तज अर्श वत (same as pittaja arsha)

5. सन्निपातज अर्श →

त्रिदोष संमिश्र लक्षण (mixed symptoms of all VPK)

6. सहज अर्श →

Due to शोणित दुष्टी (दुष्टार्तव) & शुक्रदुष्टी this type of arsha is produced. The pile masses are inside and difficult to see on examination, it may be dark in colour, troublesome and bend inwards, the patient is leans, eat little quantity of foods, can have less children or oligospermia, his voice is feeble, always angry, poor digestion and less strength etc.

उपद्रव (Complications) →

आध्मान – हल्लास – परिकर्तिका – शोथ – ज्वर – श्वास – कास – पीनस – मानसिक रोग – गुल्म – उदर रोग – मूत्राघात – प्रवाहिका – ग्रहणी etc.

साध्यासाध्यता (Prognosis) →

साध्य → बाह्य गुदवली स्थित & अचिरकाली (of less than 1 year) – एकदोषज

याप्य / कृच्छ्रसाध्य → द्वितीय गुदवली स्थित & चिरकाली (of more than 1 year)

असाध्य → आभ्यन्तर गुदवली स्थित & त्रिदोषज & सहज अर्श

अर्श चिकित्सा →

चतुर्विधोऽर्शसां साधनोपायः ।

तद्यथा– भेषजं क्षारोऽग्निः शस्त्रमिति ॥ (सु.चि.6/3)

4 types of chikitsa of arsha →

1. औषध प्रयोग
 2. क्षार कर्म
 3. अग्नि कर्म
 4. शस्त्र कर्म
- Less than 1 year old (अचिरकालजात) and the arsha having less symptoms or complications (अल्पदोष-लिंग-उपद्रव) can be treated with भेषज (औषध प्रायोग).
 - Soft to touch (मृदु), extended (प्रसृत), deep seated (अवगाढ) and elevated (उन्नत मांसांकुर) type of arsha should be treated with क्षारकर्म.
 - Rough (कर्कश), fixed (स्थिर), thick (पृथु) and firm or hard (कठिन) type of arsha should be treated with अग्निकर्म.
 - Slender root (तनुमूल), raised (उच्छिन्न) and moist (क्लेद) type of arsha is treated with शस्त्रकर्म.

शुष्कार्श चिकित्सा →

- स्थानिक अभ्यंग with कुष्ठ तैल – पिण्ड स्वेद – अवसेचन & धारासेक
- धूपन (fumigation) with human hairs, slough of snake, arka roots & shami leaves.
- Application of latex of खुही & हरिद्रा कल्क (paste) on the piles
- तक्रारिष्ट प्रयोग
- तक्र (Buttermilk) is best to be used in dry piles (शुष्कार्श).
- As last resort रक्तमोक्षण is adopted.

रक्तार्श चिकित्सा →

- Not to arrest bleeding initially (as the blood is impure)
- Then advice अग्निदीपक & दोषपाचक – तिक्तस द्रव्य
- संशोधन चिकित्सा according to predominant dosha
- पिच्छाबस्ति प्रयोग
- घृतभृष्ट पलाण्डु – पलाण्डु with तक्र – मसूर with तक्र

❖ तक्र प्रयोग in अर्श →

- In वातज अर्श → अनुदघृत तक्र
- In पित्तज अर्श → अर्धोदघृत तक्र
- In कफज अर्श → रूक्ष तक्र

According to बल-काल etc. तक्र should be used for a week (सप्ताह), ten days (दशाह), fifteen days (पक्ष), or a month (मास).

❖ Best drugs →

भल्लातक for शुष्कार्श
कुटज for रक्तार्श

अनुवासन बस्ति in अर्श →

If the patient of Arsha is suffering from उदावर्त, प्रतिलोम वात, अतिरूक्षता, उदरशूल, or गुदशूल, unctuous enema (अनुवासन बस्ति) should be used.

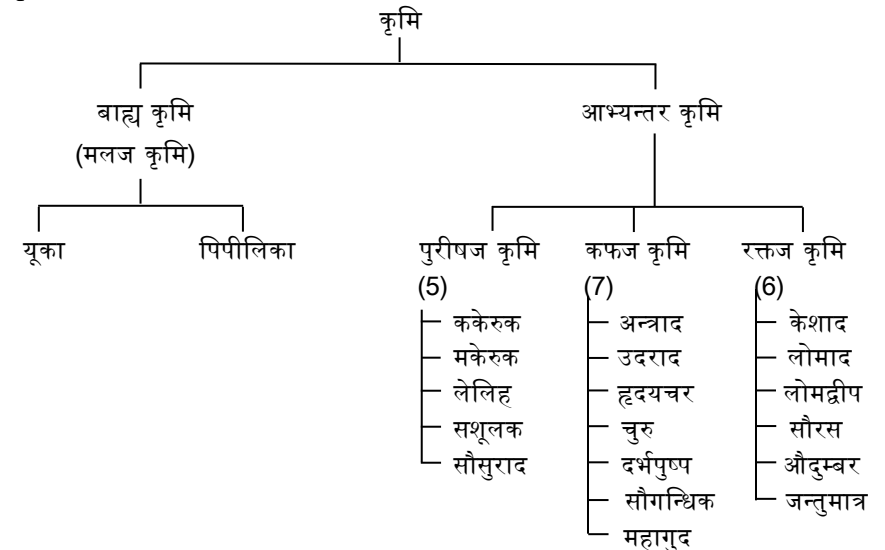
पिप्पल्यादि अनुवासन बस्ति : तैल पाक with तिलतैल, गोदुग्ध, पिप्पली, मदनफल, बिल्व, शताह्वा, यष्टिमधु, वचा, कुष्ठ, शटी, पुष्करमूल, चित्रक and देवदारु.

शमनौषधि →

अर्शकुठार रस – कांकायन वटी – त्र्यूषणाद्य चूर्ण – व्योषाद्य चूर्ण – धतूरादि चूर्ण – तक्रारिष्ट – अभयारिष्ट – दन्त्यारिष्ट – द्राक्षासव – पिप्पल्यादि घृत – गुड भल्लातक प्रयोग – कुटजादि रसक्रिया – भल्लातकादि मोदक etc.

पुरीषज कृमि (Intestinal Worms)**कृमि →**

क्रिमिः क्रव्ये मेद्यति । Those craving for मांस are called कृमि.

कृमि भेद →

सामान्य निदान →

अजीर्ण भोजी मधुराम्लनित्यो द्रवप्रियः गुडोपभोक्ता ।
व्यायामवर्जी च दिवाशयनो विरुद्धभुक् स लभते क्रिमीस्तु ॥ (मा.नि.7/4)

- अजीर्ण भोजन
- मधुर & अम्ल पदार्थ -अतिसेवन
- द्रव बहुल पदार्थ सेवन
- पिष्टमय पदार्थ & गुड अतिसेवन
- अव्यायाम
- दिवास्वप्न
- विरुद्धान्न सेवन

Along with these nidanas following are causes of बाह्य कृमि →

- अस्वच्छता (Unhygiene – not washing the body parts, and not cleaning the clothes properly)
- स्नान द्वेष (not taking bath regularly)

विशेष निदान of पुरीषज कृमि →

माषापिष्टान्नविदलपर्णशाकैः पुरीषजाः । (सु.उ.54/17)

- माष & पिष्टान्न अतिसेवन
- अम्ल लवण रस पदार्थ & गुड अतिसेवन
- पर्ण शाक अतिसेवन

विशेष लक्षण of पुरीषज कृमि →

स्थान → पक्वाशय

आकृति → Some are long, cylindrical, minute, or like wool fubers etc.

वर्ण → White, blackish, blue, green, or yellow.

प्रभाव → Intestinal worms cause following symptoms →

- अतिद्रव मल प्रवृत्ति or अतिसार (loose stool)

- पाण्डुता (anemia)
- दौर्बल्य (weakness)
- शूल (pain in abdomen)
- कठोरता (hardness of abdomen)
- रोमहर्ष (horripilation)
- गुद-प्रदेशे कण्डू (itching in anus)
- Sometimes worms are passed out with stool

कृमि-रोग चिकित्सा →

Acharya Charaka has described three principles of treatment of Krmi-roga →

1. अपकर्षण (Extraction or Removal of Krmi)

Krmi may be extracted by hand or with instruments.

By using purificatory therapies (संशोधन चिकित्सा), krmi can be removed from the body.

2. प्रकृति विघात (Breaking pathogenesis / Counteraction of manifestation of Krmi-roga by using Krimighna dravyas)

कटु-तिक्त-कषाय-क्षार-उष्ण औषध & आहार should be used to along with कृमिघ्न द्रव्य (विडंग-पारसीक-कम्पिल्लक-पलाशबीज-पूग-निम्ब etc.)

3. निदान परिवर्जन (Avoid Factors which produces Krmi)

The etiological factors of krmi like मधुर-अम्लरस अतिसेवन, गुड, शाक, विरुद्धान्नहार, दुष्ट अम्बुपान, अव्यायाम, दिवास्वप्न etc. should be avoided so that कृमि cannot produce again.

शमनौषधि →

कृमि मुद्गर रस – कृमिकुठार रस – कृमिहर रस – विडंग लौह – विडंग चूर्ण – पलाश बीज चूर्ण – पारसीकादि चूर्ण – विडंगारिष्ट – विडंगावलेह – पारिभद्रावलेह etc.

IBS

Irritable bowel syndrome (IBS) is a group of symptoms – including abdominal pain and changes in the pattern of bowel movements without any evidence of underlying damage. These symptoms occur over a long time, often years.

It has been classified into four main types depending on whether diarrhea is common, constipation is common, both are common, or neither occurs very often (IBS-D, IBS-C, IBS-M, or IBS-U respectively).

Etiology →

- The causes of IBS are not clear.
- Theories include combinations of gut–brain axis problems, gut motility disorders, pain sensitivity, infections including small intestinal bacterial overgrowth, neurotransmitters, genetic factors, and food sensitivity.
- Onset may be triggered by an intestinal infection, or stressful life event.

Signs & Symptoms →

- The primary symptoms of IBS are abdominal pain or discomfort in association with frequent diarrhea or constipation and a change in bowel habits.
- There may also be urgency for bowel movements, a feeling of incomplete evacuation (tenesmus), bloating, or abdominal distension.

- People with IBS, more commonly than others, have gastroesophageal reflux, symptoms relating to the genitourinary system, chronic fatigue syndrome, fibromyalgia, headache, backache, and psychiatric symptoms such as depression and anxiety.

Diagnosis →

- No specific laboratory or imaging test can be performed to diagnose irritable bowel syndrome. Diagnosis involves excluding conditions that produce IBS-like symptoms, and then following a procedure to categorize the person's symptoms.
- Ruling out parasitic infections, lactose intolerance, small intestinal bacterial overgrowth, and celiac disease is recommended before a diagnosis of irritable bowel syndrome is made.

Investigations →

- Full blood examination, LFT, ESR
- Stool examination (microscopy & culture)
- USG Abdomen, Colonoscopy etc.

Treatment →

- There is no specific treatment of IBS, but a number of treatments have been found to be effective, including fiber, talk therapy, antispasmodic and antidepressant medication, and peppermint oil.
- Medications may consist of stool softeners and laxatives in IBS-C and antidiarrheals (e.g., opiate, opioid, or opioid analogs such as loperamide, codeine, diphenoxylate) if diarrhea is predominant.

Ulcerative Colitis

Ulcerative colitis is a chronic condition that results in inflammation and ulcers of the colon and rectum.

The mucosa of ulcerated colon appears diffused, thickened & granular microscopically.

Etiology →

- Exact mechanism of ulcerative colitis is not clear but autoimmune is the most popular theory.
- Immunological response to antigen which may be due to the foods, drugs, cow's milk, wheat, tomatoes, orange, potatoes, eggs etc.
- Emotional influence (stress) can produce changes in the colon mucosa up to the ulceration.

Clinical Features →

- Disease is defined as 'SMS' i.e. small multiple superficial ulceration.
- (sea of ulceration / pin point ulceration)
- In 95% of cases, ulcerative colitis starts from the rectum & spreads to the colon. Hence it is known as 'Procto-colitis'.
- Cardinal symptom of ulcerative colitis is 'early morning spurious diarrhea'. Disease is characterized by passage of 15-20 motions per day mixed with blood and mucous, while sometimes without blood.
- In acute episode of ulcerative colitis →
 - High grade fever
 - Abdominal distension

- Tenderness all over abdomen
- Anemia & Shock etc.

• As late features →

- Dehydration
- Malnutrition
- Hypoproteinemia
- Metabolic acidosis etc.

Diagnosis →

- Per-rectal examination → After withdrawing finger, it is stained with mucus & blood. If rectum is involved, mucosa is diffused & thick.
- Proctoscopy → On proctoscopy, inflammatory changes can be observed in initial stage while in advanced stage pin-point ulcers are seen in rectal mucosa.
- Colonoscopy → SMS ulceration with pin-point ulcers & unhealthy mucosa between two ulcers.

Treatment →

- Hospitalization
- Antispasmodic & sedatives
- Correction of fluid & electrolyte balance with IV Fluids
- Psychotherapy
- Antibiotics (Sulfasalazine / Salazopyrin / Metronidazole)
- Steroid therapy (Oral Prednisolone / Enema)
- Immunosuppressants
(Methylprednisolone / Di-sodium cromoglycate – DSCG)
- Surgery (if needed) – Proctocolostomy

Sexually Transmitted Diseases (STD)

उपदंश (Chancroid)

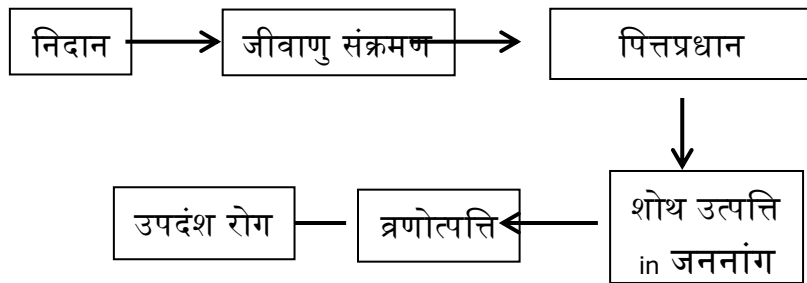
उपदंश is characterized by पीत रक्तस्राव युक्त व्रणोत्पत्ति on genital organs (प्रजनेन्द्रिय) of men and women. आचार्य चरक has explained it as ध्वजभंग.

व्रण are मृदु (soft sore), hence can be correlated with soft chancre or chancroid.

निदान →

- अति व्यवाय – अति ब्रह्मचर्य – दुष्टयोनि संसर्ग – वैश्यागमन
- अतिब्रह्मचारिणी-रजस्वला-दीर्घरोमा-कर्कशरोमा-संकीर्णरोमा-निगूढरोमा-अल्पद्वारा-महाद्वारा-अप्रिय-अकामा-अशुद्धजल प्रक्षालिता-अप्रक्षालिता योनि संसर्ग
- हस्तमैथुन – लिंग अभिघात by हस्त-मुख-दन्त-नख-शूक etc.
- लिंग-अधावन – योनिदोष – वेगधारण
- According to modern medicine, the causative organism of Chancroid is the bacteria called 'Bacillus of Ducrey' or 'Haemophilus ducreyi'.

सम्प्राप्ति →



भेद → 5

वातज – पित्तज – कफज – रक्तज – सन्निपातज उपदंश

सामान्य लक्षण →

- शोथ-शूल-रक्तिमा-स्फोट-पाक in जननांग
- मांसोत्पत्ति पश्चात् व्रणोत्पत्ति on लिंग
- पुलाकोदक सम्मित श्याम-अरुणवर्ण स्राव from व्रण
- तीव्र दाह & वेदना like अग्निदग्ध – ज्वर – तृष्णा – भ्रम – मूर्च्छा – छर्दि etc.
- If उपदंश is not treated, or in late stage it causes gangrene of genitalia that is शिश्रमणि, मेढू or मुष्क fall out.

उपदंश चिकित्सा →

उपदंशेषु साध्येषु स्निग्धस्विन्नस्य देहिनः ।

सिरा विध्येन्मेढूमध्ये पातयेद्वा जलौकसः ॥ (सु.चि.19/25)

- निदान परिवर्जन
- स्नेहन – स्वेदन – रक्तमोक्षण by venesection (सिराव्यध) or leech application (जलौकावचरण) on genital organ
- वमन & विरेचन (यथाबल)
- अनुवासन & निरुह बस्ति (in दुर्बल रोगी)
- व्रण प्रक्षालन – लेपन – रोपण – धूपन – बंधन
- प्रक्षालन with ग्राही-जन्तुघ्न, विसंक्रामण क्वाथ (पंचक्षीरी क्वाथ – त्रिफला क्वाथ – भूतघ्नचक्रिका द्राव – भृंगराज स्वरस)
- व्रण रोपण with त्रिफला मसी – जात्यादि तैल – व्रणराक्षस तैल

शमनौषधि → त्रिवंग भस्म – उपदंशगज केशरी रस – गंधक रसायन – गैरिक भस्म – कासीस भस्म – आरोग्यवर्धिनी वटी – कैशोर गुग्गुलु – चोपचीन्यादि चूर्ण – पटोलादि क्वाथ – महामंजिष्ठाद्यरिष्ट – करंजाद्य घृत – जात्यादि तैल – व्रणराक्षस तैल – त्रिफला मसी – गैरिकादि लेप – उपदंशहर मलहर

Chancroid

Also known as soft chancre and ulcus molle.

Chancroid is a bacterial sexually transmitted infection (or venereal disease) characterized by painful sores on the genitalia. It is caused by the bacterium called '**Bacillus of Ducrey**' (*Haemophilus ducreyi*).

Chancroid is known to spread from one individual to another solely through sexual contact.

Clinical Features → The symptoms may vary in men and women →

- **In Men** → Men may notice a small, red bump on their genitals that may change to an open sore within a day or two. The ulcer may form on any area of the genitals, including the penis and scrotum. The ulcers are frequently painful.
- **In Women** → Women may develop four or more red bumps on the labia, between the labia and anus, or on the thighs. After the bumps become ulcerated, or open, women may experience a burning or painful sensation during urination or bowel movements.
- The following symptoms can occur in both men and women →
 - The ulcers have a soft center that's gray to yellowish-gray with defined, or sharp, edges.
 - The ulcers may bleed easily if touched.
 - Pain may occur during sexual intercourse or while urinating.
 - Swelling in the groin may occur (i.e. lymphadenopathy of inguinal region → bubo formation)

→ Swollen lymph nodes can break through the skin and lead to large abscesses, or collections of pus, that drain.

Complications →

- Extensive adenitis may develop.
- Large inguinal abscesses may develop and rupture to form draining sinus or giant ulcer.
- Phimosis can develop in long standing lesion by scarring and thickening of foreskin, which may subsequently require circumcision.

Diagnosis →

- From bubo pus or ulcer secretions, *H. ducreyi* can be identified
- PCR-based identification of organisms, or Antigen detection

Prevention →

Chancroid spreads in populations with high sexual activity, such as prostitutes. Uses of condom, prophylaxis by azithromycin, syndromic management of genital ulcers, treating patients with reactive syphilis serology are the preventive measures of chancroid.

Treatment →

- A single oral dose (1 gram) of azithromycin, a single IM dose of 250 mg ceftriaxone, oral 500 mg of erythromycin q.i.d for seven days, or 500 mg of Ciprofloxacin b.i.d for three days.
- Treatment may include more than one prescribed medication.
- Abscesses are drained.
- Aminoglycosides such as Gentamicin, Streptomycin, and Kanamycin have been used to successfully treat Chancroid.

Lymphogranuloma inguinale

Lymphogranuloma inguinale, or Lymphogranuloma venereum (LGV), is a sexually transmitted disease caused by the bacterium called **Chlamydia trachomatis**.

Clinical Features → The clinical manifestation of LGV depends on the site of entry of the infectious organism (the sex contact site) and the stage of disease progression.

- Inoculation at the mucous lining of external sex organs (penis and vagina) can lead to the inguinal syndrome named after the formation of buboes or abscesses in the groin (inguinal) region where draining lymph nodes are located. The rectal syndrome (Lymphogranuloma venereum proctitis, or LGVP) arises if the infection takes place via the rectal mucosa (through anal sex) and is mainly characterized by proctocolitis or proctitis symptoms.
- The pharyngeal syndrome is rare. It starts after infection of pharyngeal tissue, and buboes in the neck region can occur.
- **Primary stage** → LGV may begin as a self-limited painless genital ulcer that occurs at the contact site 3–12 days after infection. Women rarely notice a primary infection because the initial ulceration where the organism penetrates the mucosal layer is often located out of sight, in the vaginal wall.
- **Secondary stage** → The secondary stage most often occurs 10–30 days later, but can present up to six months later. The infection spreads to the lymph nodes through lymphatic drainage pathways.

The most frequent presenting clinical manifestation of LGV is unilateral lymphadenitis and lymphangitis, often with tender inguinal and/or femoral lymphadenopathy because of the drainage pathway for their likely infected areas. If the route was anal sex the infected person may experience lymphadenitis and lymphangitis along with proctitis or proctocolitis. In females, cervicitis, perimetritis, or salpingitis may occur as well as lymphangitis and lymphadenitis.

- **Enlarged nodes are called buboes**. Buboes are commonly painful. Nodes commonly become inflamed, thinning and fixation of the overlying skin. These changes may progress to necrosis, fluctuant and suppurative lymph nodes, abscesses, fistulas, strictures, and sinus tracts.

Diagnosis →

The diagnosis is made serologically (through complement fixation) and by exclusion of other causes of inguinal lymphadenopathy or genital ulcers. DFA (Direct fluorescent antibody) test and / or PCR (Polymerase chain reaction) of likely infected areas and pus.

Treatment →

- Treatment involves antibiotics and may involve drainage of the buboes or abscesses by needle aspiration or incision. Common antibiotic treatments include: tetracycline (doxycycline), erythromycin and azithromycin.
- Further supportive measure may need to be taken: dilatation of the rectal stricture, repair of rectovaginal fistulae, or colostomy for rectal obstruction.

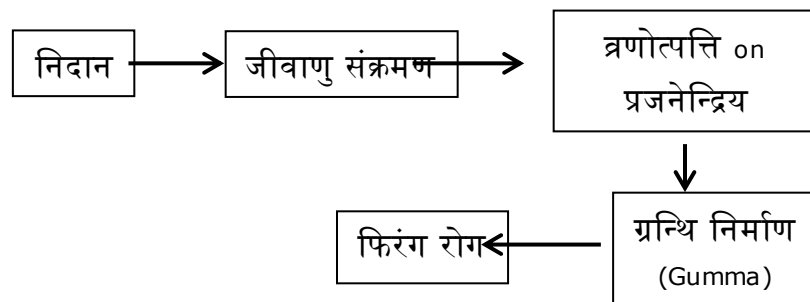
फिरंग (Syphilis)

It is a sexually transmitted disease (मैथुनजन्य संक्रामक रोग) which is spread by foreigners (फिरंगी). As it came in India due to फिरंगी, it is called फिरंग.

निदान →

- फिरंग रोग is produced due to sexual contact with foreigners (फिरंगी पुरुष or स्त्री). Thus it is आगन्तुक रोग in which दोष प्रकोप takes place after some time (7 days) of संक्रमण.
- It is an STI that is passed from one person to another through sexual contact. It get transmitted to a healthy man if he sexually contacted infected woman, and transmitted to a healthy woman if she sexually contacted infected man. If a pregnant woman is having syphilis (फिरंग) that can pass to baby, and this type of फिरंग is known as कुलज or सहज फिरंग (Congenital Syphilis).
- According to modern medicine, the causative organism of Syphilis is the bacteria called 'Treponema pallidum'.

सम्प्राप्ति →



फिरंग रोगावस्था →

1. प्रथमावस्था → This stage is a kind of बाह्य फिरंग. Only local symptoms (स्थानिक लक्षण) are found in this stage, which includes:

- व्रणोत्पत्ति on प्रजनेन्द्रिय (a skin lesion, called a chancre, appears at the point of contact, usually at glans penis or inside prepuce in men, and inside labia majora in women. Sometimes chancre may also develop on lips, breasts, tongue and fingers etc.)
- This व्रण is hard to touch and round in shape, and therefore known as hard chancre.
- After 1 to 2 weeks of व्रणोत्पत्ति, lymphadenopathy may occur in inguinal region (वंक्षण प्रदेश).
- This stage lasts for 1½ to 2 months.

2. द्वितीयावस्था → After initial व्रणोत्पत्ति, within 3 to 4 weeks second stage manifests that can last for 2 years. In this stage जीवाणु विष gets circulated and spread all over the body producing following symptoms:

- Skin lesions appear anywhere in the body. These multiple lesions are not similar in shape, size and colour. Usually body parts are affected bilaterally, and there is no itching and pain.
- Like that of external skin, lesions (ulcers) appear on mucosa of lips, tongue, palate etc. The sites where skin is always moist and those are junction of skin and mucosa such as गुदा (anus) and भगौष्ठ (labia minora), there also lesions appear that are called 'Condyloma' (genital warts).

- Lymphadenopathy in कक्षा (axilla), कुर्पर (elbow), ग्रीवा (neck) and वंक्षण प्रदेश (inguinal region).
- Apart from skin lesions and genital warts, following systemic symptoms may also appear →
 - ज्वर (fever)
 - पाण्डुता (anemia)
 - दुर्बलता (weakness)
 - शिरःशूल (headache)
 - अस्थि & सन्धिःशूल (arthralgia) in रात्रि
 - खालित्य (hair fall)

3. तृतीयावस्था → This stage manifests after 6 months to 2 years of Condyloma-stage. In this stage, ग्रन्थि are formed in त्वचा (skin), लसिका ग्रन्थि (lymph nodes), अस्थ्यावरण (periosteum), अस्थि (bones), मांसपेशी (muscles and tendons), यकृत (liver), प्लीहा (spleen), and वृषण ग्रन्थि (testis) आदि which are called 'Gumma'. These ग्रन्थि's are having following features →

- ग्रन्थि (gumma) are knotty and flat.
- Gradually they get suppurated as abscess and burst as ulcers, then thick and sticky discharge.
- Deep ulcers are formed in skin.
- If it is formed in नासा, it causes नासाभंग (Saddle Nose).
- If it is formed in तालु, it causes विदारण (Perforated Palate).
- If it is formed in मस्तिष्क or सुषुम्ना प्रदेश, it causes पक्षाघात or पंगुता.
- If it is formed in जिह्वा, it causes जिह्वा भेद (Fissureing of Tongue).

4. चतुर्थविस्था → Some acharyas consider this fourth stage as third stage itself and thus explained only three stages of Phiranga-roga. But this final stage is considered as separate because of its peculiarity that is involvement of Central Nervous System. Within 3 months after first stage, or 25 to 30 years later this stage can manifests by attack of जीवाणु-विष with occurrence of Neurological diseases:

- नाडी संस्थानीय फिरंग (Neuro syphilis)
- फिरंगजन्य खंजता (Tabes dorsalis or Locomotor ataxia)
- फिरंगज सर्वाङ्गघात युक्त उन्माद (General paralysis of insane – GPI)

सहज फिरंग →

फिरंग रोग produced in new-born baby (नवजात शिशु) due to infected mother or father (फिरंग पीडित मातृ-पितृ) is called सहज फिरंग (Congenital Syphilis).

फिरंग भेद → (i) बाह्य फिरंग (त्वकगत) – सुखसाध्य

(ii) आभ्यन्तर फिरंग (सन्धिगत) – कष्टसाध्य

(iii) बहिरन्तर्गत फिरंग (बाह्य & आभ्यन्तर) – कष्टसाध्य or असाध्य

फिरंग चिकित्सा →

- निदान परिवर्जन
- यथाबल संशोधन
- रक्तमोक्षण (जलौकावचरण) & रक्तशोधक औषध प्रयोग
- व्रण प्रक्षालन with स्फटिका चूर्ण – त्रिफला क्वाथ – निम्बपत्र क्वाथ – भृंगराज स्वरस
- व्रण शोधन-रोपण with त्रिफला मसी – व्रणघ्न मलहर – पारदादि लेप वटी

शमनौषधि → रसमाणिक्य – मल्ल सिन्दूर – आरोग्यवर्धनी वटी – मल्ला दिवटी – चोपचीनी चूर्ण – निम्बादि चूर्ण – पटोलादि क्वाथ – लघुमंजिष्ठादि क्वाथ – बृहद्मंजिष्ठादि क्वाथ – खदिरारिष्ट – देवदार्वारिष्ट

Syphilis

Syphilis is a sexually transmitted disease (STD) caused by the bacterium **Treponema pallidum**. Syphilis is most commonly spread through sexual activity. It may also be transmitted from mother to baby during pregnancy or at birth, resulting in congenital syphilis.

Types & Features of Syphilis →

(A) Acquired Syphilis → The signs and symptoms of syphilis vary depending in which of the four stages it presents (primary, secondary, latent, and tertiary).

1. **Primary syphilis** is typically acquired by direct sexual contact with the infectious lesions of another person. The primary stage classically presents with a single chancre (a firm, painless, non-itchy skin ulceration) but there may be multiple sores. Lymph node enlargement frequently occurs around the area of infection.
2. **Secondary syphilis** occurs approximately four to ten weeks after the primary infection. While secondary disease is known for the many different ways it can manifest, symptoms most commonly involve the skin, mucous membranes, and lymph nodes. There may be a symmetrical, reddish-pink, non-itchy rash on the trunk and extremities, including the palms and soles. It may form flat, broad, whitish, wart-like lesions on mucous membranes, known as condyloma lata. Other symptoms may include fever, sore throat, malaise, weight loss, hair loss, and headache.

3. **Latent syphilis** is defined as having serologic proof of infection without symptoms of disease. In latent syphilis, which can last for years, there are few or no symptoms.
4. **Tertiary syphilis** may occur approximately 3 to 15 years after the initial infection, and may be divided into three different forms:
 - i. Gummatous syphilis (or Late Benign syphilis),
 - ii. Late Neurosyphilis, and
 - iii. Cardiovascular syphilis.

Gummatous syphilis or late benign syphilis is characterized by the formation of chronic gummas, which are soft, tumor-like balls of inflammation which may vary considerably in size. They typically affect the skin, bone, and liver, but can occur anywhere.

Neurosyphilis refers to an infection involving the central nervous system. It may occur early, being either asymptomatic or in the form of syphilitic meningitis, or late as meningovascular syphilis, general paresis, or tabes dorsalis, which is associated with poor balance and lightning pains in the lower extremities.

Cardiovascular syphilis causes complication like syphilitic aortitis, which may result in aortic aneurysm formation.

- (B) Congenital syphilis** → Congenital syphilis is that which is transmitted during pregnancy or during birth. Congenital syphilis is syphilis present in utero and at birth, and occurs when a child is born to a mother with syphilis. Common symptoms that develop over the first

couple of years of life include enlargement of the liver and spleen, rash, fever, neurosyphilis, and lung inflammation. If untreated, late congenital syphilis may occur.

Congenital syphilis is characterized by Hutchinson's triad.

Hutchinson's triad →

1. **Deafness** → eighth nerve deafness
2. **Hutchinson's teeth** → centrally notched, widely spaced peg-shaped upper central incisors
3. **Interstitial keratitis (IK)** → an inflammation of the cornea which can lead to corneal scarring and potentially blindness

Investigations →

Syphilis is difficult to diagnose clinically during early infection. Confirmation is either via blood tests or direct visual inspection using dark field microscopy.

Blood tests includes:

- Nontreponemal tests are used initially, and include venereal disease research laboratory (VDRL) and rapid plasma reagin (RPR) tests.
- Because of the possibility of false positives with nontreponemal tests, confirmation is required with a treponemal test, such as treponemal pallidum particle agglutination (TPHA) or fluorescent treponemal antibody absorption test (FTA-Abs).

Prevention → Practising safe sex is the best way to prevent syphilis infection. Use condom during sex, and avoid sex with someone infected with syphilis or who has symptoms of syphilis until they have finished treatment. To prevent congenital syphilis, WHO recommends all pregnant women be tested at their first antenatal visit and again in the 3rd trimester. If they are positive, their partners should also be treated.

Treatment →

In Early infections →

- The first-line treatment for uncomplicated syphilis remains a single dose of **intramuscular benzathine benzylpenicillin**.
- **Doxycycline** and **tetracycline** are alternative choices for those allergic to penicillin; due to the risk of birth defects, these are not recommended for pregnant women.
- **Ceftriaxone**, a third-generation cephalosporin antibiotic, may be as effective as penicillin-based treatment.

In Late infections →

- For neurosyphilis, due to the poor penetration of benzathine penicillin into the central nervous system, those affected are given large doses of intravenous penicillin for a minimum of 10 days.
- If a person is allergic to penicillin, ceftriaxone may be used or penicillin desensitization attempted.
- Other late presentations may be treated with once-weekly intramuscular benzathine penicillin for three weeks.

पूयमेह (Gonorrhoea)

“पूयं मेहयतीत्यतः पूयमेहः” As there is पूयस्राव (pus-discharge) from मूत्रमार्ग (urethra) in this disease, it is called पूयमेह.

पूयमेह पर्याय (Synonyms) →

औपसर्गिक मेह – produced by उपसर्ग (संक्रमण)

आगन्तुक मेह – produced by exogenous cause – संक्रमित जीवाणु

व्रणमेह – symptoms & treatments are like a व्रण

निदान →

- It is an STI that is passed from one person to another through sexual contact. It gets transmitted to a healthy man if he sexually contacted infected woman, and transmitted to a healthy woman if she sexually contacted infected man.
- Sex with a woman who has योनिव्यापद.
- Indulging in sexual activities by menstruating woman (रजस्वला स्त्री).
- Multiple sexual partners.
- Not using condom.

लक्षण →

- मूत्रमार्गगत पूयस्राव – Pus discharge from urethra
- मूत्रमार्गगत रक्तिमा & शोथ – Redness & swelling of urethra (urethritis)
- मूत्रकृच्छ्र – Painful & burning micturition
- मूत्रमार्गावरोध – Sometimes urinary retention due to dried pus
- मूत्रेन्द्रिय कण्डू – Itching around urethral opening
- मुष्क-प्रदेश वेदना – Pain in scrotum

→ अधोजठर शूल – Pain in lower abdomen

→ औपसर्गिक नेत्राभिष्यन्द in नवजात शिशु – Ophthalmia neonatorum

→ श्रोणिगत शोथ – Pelvic inflammatory disease (PID)

उपद्रव →

In men (पुरुष) →

मूत्रमार्ग सन्निरोध (urethral stricture) – अष्ठीला शोथ (prostatitis) – वृषण शोथ (orchitis) – बस्ति शोथ (cystitis) – शुक्राशय शोथ (vesiculitis) – आमवात (arthritis) – औपसर्गिक नेत्राभिष्यन्द (conjunctivitis)

In women (स्त्री) →

मूत्रमार्ग शोथ (urethritis) – गर्भाशय ग्रीवा शोथ (cervicitis) – योनि शोथ (vaginitis) – उदरावरण कला शोथ (peritonitis) – बन्ध्यता (infertility) – आमवात (arthritis) – औपसर्गिक नेत्राभिष्यन्द (conjunctivitis)

पूयमेह चिकित्सा →

- निदान परिवर्जन
- मैथुन त्याग
- शोथहर-व्रणहर-वातानुलोमक-मूत्रल-रक्तशोधक औषध & आहार सेवन
- शिश्र / योनि प्रक्षालन with स्फटिका जल / निम्बपत्र क्वाथ / पंचवल्कल क्वाथ
- उत्तर बस्ति with त्रिफला + बबूल + पिप्पली कषाय / त्रिफला + दधि + तुथ + हिङ्गुल + मृदारशृङ्ग कषाय

शमौषधि →

त्रिवंग भस्म – रस माणिक्य – प्रवाल पिष्टी – गंधक रसायन – चन्द्रप्रभा वटी – आरोग्यवर्धिनी वटी – गोक्षुरादि गुग्गुलु – शिलाजत्वादि वटी – स्फटिकादि चूर्ण – तृणपंचमूल क्वाथ – मंजिष्ठादि क्वाथ – बबूल निर्यास – विडंगारिष्ट

Gonorrhea

Gonorrhea is a sexually transmitted infection (STI or STD) caused by the bacterium **Neisseria gonorrhoeae**. Gonorrhea is spread through sexual contact with an infected person. This includes oral, anal, and vaginal sex. It can also spread from a mother to a child during birth.

Clinical Features →

- Many people have no symptoms.
- Men may have burning with urination, discharge from the penis, or testicular pain.
- Women may have burning with urination, vaginal discharge, vaginal bleeding between periods, or pelvic pain.
- If untreated, gonorrhea can spread to joints or heart valves.

Investigations →

- Gram stain → Smear stained by Gram stain of the urethral discharge is almost 100% accurate in diagnosing in Gonorrhea in men. In women, the material should be obtained from multiple sites including the cervix, vaginal vault and urethra.
- Culture → Culture of the organism-Enriched media is used like 'Chocolate agar'.
- Fluorescent antibody test
- Polymerase chain reaction (PCR)
- Evaluate the patient for the presence of other co-existing sexually transmitted disease including Syphilis and Chlamydia.

Complications →

Skin pustules –Conjunctivitis –Septic arthritis – Meningitis – Endocarditis

In men → Epididymitis – Orchitis – Prostatitis – Urethritis etc.

In women → Pelvic inflammatory disease (PID) – Cervicitis – Urethritis – Vaginitis – Infertility etc.

Prevention →

- Gonorrhea can be prevented with the use of condoms, having sex with only one person who is uninfected, and by not having sex.
- Newborn babies coming through the birth canal are given erythromycin ointment or one drop of 1% silver nitrate in the eyes to prevent blindness from infection. The underlying gonorrhea should be treated; if this is done then usually a good prognosis will follow.[]

Treatment →

- Injectable ceftriaxone is one of the few effective antibiotics. This is typically given in combination with either azithromycin or doxycycline.
- Many antibiotics that were once effective including penicillin, tetracycline, and fluoroquinolones are no longer recommended because of high rates of resistance.
- It is recommended that sexual partners be tested and potentially treated. One option for treating sexual partners of people infected is patient-delivered partner therapy (PDPT), which involves providing prescriptions or medications to the person to take to his/her partner without the health care provider's first examining him/her.

मनस & मानसिक रोग

मन → “मन्यतेऽनेन इति मनः” / “मन ज्ञाने एवं मनु अवबोधने”

Manas or mind is defined as an object of cognition (indriya) through which one perceives knowledge.

मनस स्थान → हृदय

मनस-गुण → अणुत्वमथ च एकत्वं दौ गुणौ । (च.शा.1/19)

मनस कर्म → इन्द्रियाभिग्रहः कर्म मनसः स्वस्य निग्रहः । (च.शा.1/21)

मनस विषय → चिन्त्यं विचार्यमूह्यं च ध्येयं संकल्पमेव च ।
यत्किञ्चिन्मनसो ज्ञेयं तत् सर्वं ह्यर्थसंज्ञकम् ॥ (च.शा.1/20)

मनो-रोग सामान्य निदान →

→ अल्प अत्व

→ Vitiation of मानसिक दोष → रज & तम

→ Emotions or मनोभाव → (क्रोध-शोक-भय-हर्ष-विषाद-ईर्ष्या-मात्सर्य-काम-लोभ-मोह)

→ All of the Sharirika and Manasika rogas are caused by following three types of etiological factors →

1. असात्मेन्द्रियार्थ संयोग
2. प्रज्ञापराध
3. परिणाम (काल)

प्रज्ञापराध →

धीधृतिस्मृतिविभ्रष्टः कर्म यत् कुरुतेऽशुभम् ।

प्रज्ञापराधं तं विद्यात् सर्वदोषप्रकोपणम् ॥ (च.शा.1/102)

When a person does sinful acts because of the derangement of धी, धृति & स्मृति, it is called प्रज्ञापराध. Indulging in the improper activities knowingly is nothing but प्रज्ञापराध that leads to सर्वदोषप्रकोप.

Rabin Singh

• मनो-रोग सामान्य लक्षण →

→ उद्वेग लक्षण → अस्थैर्य (instability) – वेपथु (tremor) – असहिष्णुता (intolerance) – शीघ्रकोप (Irritability) – स्वेदबाहुल्य (Excessive perspiration) – तृट्बाहुल्य (Excessive thirst) – मुखशोष (Dryness of mouth) – भय (Fear) etc.

→ विषाद लक्षण → विषाद (depression) – उत्साहनाश (loss of enthusiasm) – आशाभंग (frustration) – अनन्नाभिलाषा (lack of interest in food) – अल्पचेष्टा (reduced activity) – अनिद्रा (sleeplessness)

• मनो-रोग सामान्य चिकित्सा →

1. दैव व्यपाश्रय चिकित्सा →

→ मन्त्र जप – औषध & मणिधारण – पूजन – यज्ञ – स्तुति – बलि – मंगल – होम – उपहार – ब्रह्मचर्य – तप – सत्य – नियम – साधुदर्शन etc.

2. युक्ति व्यपाश्रय चिकित्सा →

→ आहार – विहार – औषध प्रयोग

→ मेध्य द्रव्य → ब्राह्मी – मण्डूकपर्णी – यष्टिमधु – गुडूची – शंखपुष्पी – ज्योतिष्मति – जटामांसी – वचा – अश्वगन्धा etc.

3. सत्वाजय चिकित्सा →

Restrain mind from desire of unwholesome objects. The best approach to achieve the goal of sattvajaya chikitsa is through the propagation of: “मनसो ज्ञानविज्ञान धैर्यं स्मृति समादिभिः” (च.सू.1/52)

ज्ञान → knowledge of self

विज्ञान → professional / analytical knowledge

धैर्य → patience and confidence

स्मृति → keeping good memories

समाधि → concentration and meditation

• अद्रव्यभूत चिकित्सा →

त्रासन – भय दर्शन – विस्मापन – विस्मारण – क्षोभन – भर्त्सन – बंधन – हर्षण etc.

मनोभाव

काम (Lust / Passion) → Desire to obtain the objects.

क्रोध (Anger) → A strong feeling of annoyance, displeasure, or enmity to someone.

लोभ (Greed) → Intense and selfish desire for something, especially wealth, power, or food.

मोह (Delusion) → Intense passion, or a mistaken belief that is held with strong conviction even when presented with superior evidence to the contrary.

मद (Neurosis) → Madness or intoxication which leads to anxiety or confused state of mind.

मात्सर्य (Malice) → To desire to harm someone – ill will.

शोक (Grief / Sorrow) → Special condition of mind due to loss of desired things, failure of some tasks or some punishment.

भय (Fear) → occurs due to insecurity, failure or loss in future etc.

विषाद (Depression) → A state of low mood, feeling of severe dejection and aversion to activity due to failure or other cause.

दैन्य (Misery) → A state of feeling of great physical or mental distress or discomfort.

हर्ष (Euphoria / Joy) → Intense excitement and happiness.

प्रज्ञापराध (Violation of normal acts) → Due to derangement of intellect and memory, violating the normal procedures /acts, or doing unwholesome practice knowingly.

सामान्य चिकित्सा →

→ मनोरोग चिकित्सा

→ Especially सत्त्वावजय चिकित्सा → आश्वासन – मनोबुद्धि स्मृति संयोजन – संज्ञा प्रबोधन – मन प्रसाद क्रिया – सत्संग etc.

Increase Sattva quality of mind and decrease Rajas and Tamas by Sattvavajaya chikitsa → Restrain mind from desire of unwholesome objects. The best approach to achieve the goal of sattvavajaya chikitsa is through the propagation of: “मनसो ज्ञानविज्ञान धैर्यं स्मृति समादिभिः” (च.सू.1/52)

- ज्ञान → knowledge of self
- विज्ञान → professional / analytical knowledge
- धैर्य → patience and confidence
- स्मृति → keeping good memories
- समाधि → concentration and meditation

उन्माद (Insanity)

परिभाषा →

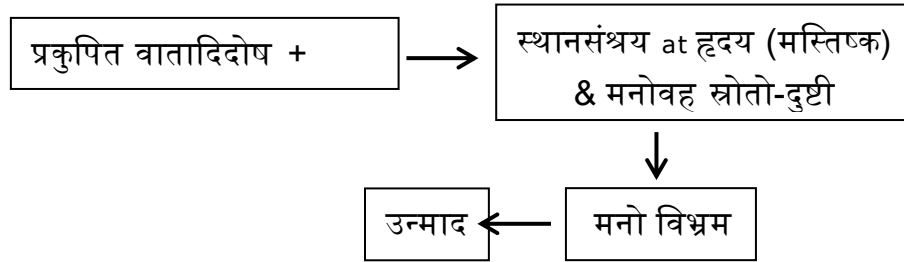
उन्मादं पुनः मनो बुद्धि संज्ञा ज्ञान स्मृति भक्ति शील चेष्टा आचार विभ्रमं विद्यात् ।
Unmada is the disease in which there is abnormality, contradiction and variation in the qualities of mind, intellect, knowledge, memory, desire, character and actions.

निदान →

विरुद्धदुष्टाशुचिभोजनानि प्रधर्षणं देवगुरुद्विजानाम् ।
उन्मादहेतुः भयहर्षपूर्वो मनोऽभिघातो विषमाश्च चेष्टा ॥

Intake of opposite or contradictory, polluted, impure food and drinks; Insult to the gods, guru and dvijas; Manoabhighata due to fear, and sudden happiness; and unwholesome physical and mental activities lead to manifestation of Unmada.

सम्प्राप्ति →



भेद → 5 → 1. वातज 2. पित्तज 3. कफज 4. सन्निपातज 5. आगन्तुज उन्माद

पूर्वरूप → (i) Feeling of emptiness in head – Loss of appetite

(ii) Restless eyes – Irregular behaviour – Excessive thinking

(iii) Exhaustion

सामान्य लक्षण →

धीविभ्रम / मनोविभ्रम (Mental disturbance) – व्यग्र चञ्चल मन (Aggressive and unstable mind) – अवबद्ध वाक (Irrelevant talk) – हृदय शून्यता (Emptiness in heart-region, or emotionlessness – no feeling of happiness or sorrow)

भेदानुसार लक्षण →

क्र.	उन्माद भेद	लक्षण
1	वातज उन्माद	At inappropriate place or without any cause laughing, smiling, dancing, singing, speaking, moving limbs, and weeping Body becomes rough, emaciated, and reddish
2	पित्तज उन्माद	Intolerance, over daring, nakedness, intimidation, running about, excessive heat, and anger Desire for shade, cold food, and cold water
3	कफज उन्माद	Sluggishness in speech and activity Liking for women and lonely places, anorexia, excessive sleep, vomiting, and salivation
4	सन्निपातज उन्माद	All three dosha lakshanas present
5	आगन्तुज उन्माद	Also known as भूतोन्माद, caused by improper observance of niyama, insult to gods, sages etc. in present life and sinful acts of past life Lakshanas are supernatural speech, valor, potency and activities; supernatural knowledge; undeterminability of the time of occurrence

8 types of भूतोन्माद →

1. देवोन्माद, 2. शापोन्माद, 3. पितृग्रहज उन्माद, 4. गन्धर्वोन्माद,
5. यक्षोन्माद, 6. राक्षसोन्माद, 7. ब्रह्मराक्षसोन्माद, 8. पिशाचोन्माद

5 types of unmada are explained by Acharya Charaka, while Acharya Sushruta has explained 6 types of Unmada →

वातज – पित्तज – कफज – सन्निपातज – मनसज – विषज उन्माद

मनसज उन्माद is produced due to mental shock or psychological shock and is characterized by irrelevant talk and unusual behavior.

विषज उन्माद is produced due to poisoning and is characterized by blackish discolouration of face and destruction of बल & इन्द्रिय.

साध्यासाध्यता →

विषज & सन्निपातज types are असाध्य. Other types are याप्य or कृच्छ्रसाध्य.

क्षीण मांस बल → असाध्य

जागरुक (unable to sleep) → असाध्य

उन्माद चिकित्सा →

→ निदान परिवर्जन

→ संशोधन चिकित्सा → स्नेहन – स्वेदन – वमन – विरेचन – अनुवासन & निरुह बस्ति – अञ्जन – अवपीडन नस्य – प्रध्मननस्य – धूमपान – धूपन कर्म

→ संशमन चिकित्सा → अभ्यंग – प्रदेह – परिषेक – अनुलेपन – त्रासन चिकित्सा

शमनौषधि →

1. रसौषधि → मात्रा = 125 - 250 mg. अनुपात = मधु

उन्माद गज केशरी रस – उन्माद गजांकुश रस – चतुर्भुज रस – प्रवाल पंचामृत

2. वटी → मात्रा = 250 - 500 mg. अनुपात = ऊष्णोदक / मधु

सर्पगन्धा घन वटी – ब्राह्मी वटी

3. चूर्ण → मात्रा = 3 - 6 g. अनुपात = ऊष्णोदक / मधु

सर्पगन्धा चूर्ण – अश्वगन्धा चूर्ण – जटामांसी चूर्ण – सारस्वत चूर्ण

4. क्वाथ/आसव/अरिष्ट → मात्रा = 10 - 20 ml. अनुपात = जल

दशमूलारिष्ट – अश्वगन्धारिष्ट – चन्दनासव

5. घृत → मात्रा = 10 - 30 ml. अनुपात = उष्ण जल / दुग्ध

हिंवादि घृत – लशुनाद्य घृत – कल्याणक घृत – महापैशाचिक घृत – सिद्धार्थक घृत

6. अगद → मात्रा = 125 - 250 g. प्रयोग → पान/नस्य/अंजन/आलेप

सिद्धार्थक अगद – सिद्धार्थकादि अगद – करञ्जादि अगद

7. अंजन/वर्ति → मात्रा = आवश्यकतानुसार

कृष्णाद्यंजन – दाव्यादि गुडिकांजन – सैध्वादि वर्ति – शिरीषादि वर्ति

पुराण घृत (best for both unmada & apasmara):

1 year old → पुराण घृत 10 years old → प्रपुराण घृत

100 years old → शतवर्ष पुराण घृत (best)

• **वातज उन्माद चिकित्सा →**

→ स्नेहपान (internal oleation)

→ If वात is कफ-पित्तावृत्त (obstructed or covered by kapha or pitta), then सस्नेह मृदुशोधन (वमन-विरेचन, if needed)

• **पित्तज & कफज उन्माद चिकित्सा →**

→ स्नेहन – स्वेदन

→ वमन for कफज

→ विरेचन for पित्तज

After संशोधन, संसर्जन क्रम should be adopted.

Again after स्नेहन-स्वेदन, when patient regains strength, then निरुह बस्ति, अनुवासन बस्ति & नस्य प्रयोग.

In case of abnormal behavior, even after shodhana chikitsa →

- तीक्ष्ण नस्य
- तीक्ष्ण अंजन
- प्रताडन (त्रासन)
- आगन्तुक उन्माद चिकित्सा →
 - घृतपान – मन्त्रोपचार
 - भूतोन्माद चिकित्सा with रतिविषयक उपहार and अर्चना, then administration of medicines.
- ❖ त्रासन चिकित्सा →
 - The patient is tied, and kept confined to a dark room devoid of iron and wooden pieces.
 - Body is rubbed with Kapikachchhu or branded with hot iron rods or burnt with hot oil or water or beaten with a hunter etc.
 - He is terrorized by the biting of snakes having their fangs removed, well tamed lions and elephants, enemies or criminals with weapons in their hands, or arresting by policemen.
 - Shouting with anger, threatening, terrorizing to the patient, and presenting gifts, exhilaration, consolidation and fear bring back the natural state of mind.

विगत उन्माद लक्षण →

प्रसादश्चेन्द्रियार्थानां बुद्ध्यात्ममनसां तथा ।

धातूनां प्रकृतिस्थत्वं विगतोन्मादलक्षणम् ॥

Clarity in perception of sense objects, clarity of intelligence, self as well as mind, and normalcy of body elements are lakshanas of the person free from Unmada.

अपस्मार (Epilepsy)

परिभाषा →

स्मृतेरपगं प्राहुरपस्मार भिषग्विदः ।

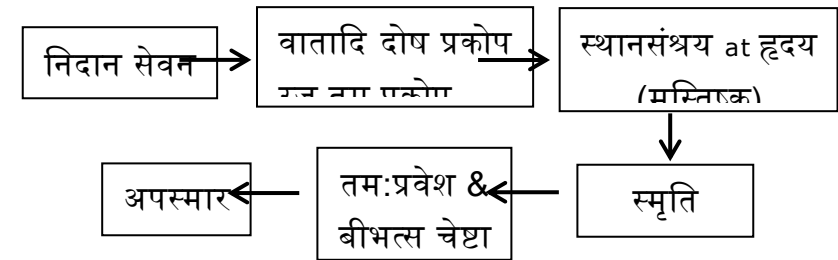
तमः प्रवेशं वीभत्सचेष्टं धीसत्त्वसंप्लवात् ॥

Loss of memory for a while, associated with a feeling of entering into darkness (unconsciousness), irrelevant body movements; due to derangement of intellect & mind are the signs of 'apasmara'

निदान →

- इन्द्रियार्थ मिथ्यायोग व अतियोग
- अहित-अशुचि भोजन
- रजस्तमो दोष वृद्धि
- अल्पसत्त्व
- चिन्ता काम भय क्रोध शोक द्वेगादि मनोऽभिघात etc.

सम्प्राप्ति →



अपस्मार-भेद → 4 → 1. वातज 2. पित्तज 3. कफज 4. सन्निपातज अपस्मार

पूर्वरूप → (i) Visualizing non-existing things

(ii) Falling down

(iii) Twitching of eyes, tongue, eyebrows

(iv) Excessive salivation

(v) Convulsive movement of hands & feet etc.

सामान्य लक्षण →

- स्मृतिनाश – Loss of memory
- तमःप्रवेश – Darkness in front of eyes
- बीभत्स चेष्टा – Irrelevant body movements
- Irrelevant mental activities
- Convulsions (Seizures)

भेदानुसार लक्षण →

क्र.	अपस्मार भेद	लक्षण
1	वातज	Tremors in the body, biting teeth, emitting froath, respiring excessively, seeing things as red & black
2	पित्तज	Yellowish discolouration of body & eyes, yellowish froath, and seeing things as yellow-red
3	कफज	Pale body & eyes, whitish froath, seeing things as white, and pt. recovers after long period of time
4	सन्निपातज	All 3 dosha lakshanas are present, it is incurable

अपस्मार प्रकोपक काल → पक्षाद्वा द्वादशाद्वा मासाद्वा कुपिता मलाः ।

In the interval of 15 days / 12 days / 1 month or when the manasika dosha along with sharirika dosha get aggravated, apasmara attacks.

साध्यासाध्यता →

अपस्मार is असाध्य in क्षीण रोगी (debilitated patient), if frequency of attacks is more, and if सन्निपातज अपस्मार is present with all symptoms.

अपस्मार चिकित्सा →

First of all, the physician should use वमन – विरेचन – बस्ति – तीक्ष्ण नस्य – तीक्ष्ण अंजन etc. to clear the obstruction of हृदय & मनोवह स्रोतस and to protect the चेतना of the patient.

संशोधन चिकित्सा →

- In वातज अपस्मार → बस्ति
- In पित्तज अपस्मार → विरेचन
- In कफज अपस्मार → वमन

संशमन चिकित्सा →

- निदान परिवर्जन
- पञ्चगव्य घृत – महापञ्चगव्य घृत – ब्राह्मी घृत – शंखपुष्पी
- अभ्यंग with सर्षप तैल processed with बस्तमूत्र (अजमूत्र)
- धूपन with पिप्पली, सैधव लवण, चित्रक, हिंगु, सर्षप, बकायन (महानिम्ब), चन्दन, & bones and nails of dog
- उत्सादन with तुलसी etc. mixed with गोमूत्र

नस्य in अपस्मार →

- नस्य with कपिला गौ-मूत्र (urine of reddish brown cow) is best to cure apasmara. Similarly, the urine of dog, jackal, cats etc. can be used.
- नस्य with भांगी, वचा and नागदन्ती triturated with गोमूत्र
- नस्य with ज्योतिष्मति and नागदन्ती triturated with गोमूत्र
- नस्य with श्वेत अपराजिता and शतावरी triturated with गोमूत्र
- नस्य with त्रिफलादि तैल cures apasmara.

आगन्तुक अपस्मार should be treated similar to उन्माद चिकित्सा.

शमनौषधि → स्मृति सागर रस – चतुर्भुज रस – सारस्वत चूर्ण – अश्वगन्धा चूर्ण – दशमूल क्वाथ – दशमूलारिष्ट – अश्वगन्धारिष्ट – सारस्वतारिष्ट – पंचगव्य घृत – महापंचगव्य घृत – ब्राह्मी घृत – सिद्धार्थक घृत etc.

मेध्य रसायन प्रयोग → Apasmara is the chronic disease related to स्मृति, which should be treated with रसायन like ब्राह्मी, मण्डूकपर्णी, यष्टिमधु, गुडूची, शंखपुष्पी, ज्योतिष्मति, जटामांसी, वचा etc.

अतत्त्वाभिनिवेश

(Attachment to unreality) also called as महागद. Vitiated dosha affects hridaya, channels connected to mind and intellect of a person (already affected with rajas & tamas) → produces confusion → attachment to unreality.

विषमां कुरुते बुद्धिं नित्यानित्येहिताहिते ।

The patient thinks or believes in the things which are not in existence.

He can't differentiate good things from bad things. He believes the beneficial things as harmful, and harmful things as beneficial.

अतत्त्वाभिनिवेश चिकित्सा →

- स्नेहन – स्वेदन – संशोधन
- मेध्य रसायन (ब्राह्मी शंखपुष्पी etc.)
- पंचगव्य घृत
- आध्यात्मिक वार्ता etc.

Anxiety disorders

Anxiety disorders are a group of mental disorders characterized by significant feelings of anxiety and fear. Anxiety is a worry about future events, and fear is a reaction to current events. These feelings may cause physical symptoms, such as a fast heart rate and shakiness.

The cause of anxiety disorders is a combination of genetic and environmental factors. Risk factors include a history of child abuse, family history of mental disorders, and poverty. Anxiety disorders often occur with other mental disorders, particularly major depressive disorder, personality disorder, and substance use disorder. Anxiety can also be caused by alcohol abuse. Caffeine, alcohol, and benzodiazepine dependence can worsen or cause anxiety and panic attacks.

There are a number of anxiety disorders including:

- Generalized anxiety disorder
- Specific phobia
- Agoraphobia
- Social anxiety disorder
- Post-traumatic stress disorder
- Separation anxiety disorder
- Panic disorder
- Obsessive compulsive disorder
- Selective mutism

Generalized anxiety disorder →

Generalized anxiety disorder (GAD) is a common disorder, characterized by long-lasting anxiety that is not focused on any one object or situation. Those suffering from generalized anxiety disorder experience non-specific persistent fear and worry, and become overly concerned with everyday matters. With chronic excessive worry, patient may suffer from restlessness, fatigue, concentration problems, irritability, muscle tension, and sleep disturbance.

Specific phobia →

A specific phobia is any kind of anxiety disorder that amounts to an unreasonable or irrational fear related to exposure to specific objects or situations. The fear or anxiety may be triggered both by the presence and the anticipation of the specific object or situation. A person who encounters that of which they are phobic will often show signs of fear or express discomfort. In some cases, it can result in a panic attack.

Few examples of specific phobias →

- Animal type – Fear of dogs, cats, rats and/or mice, pigs, cows, birds, spiders, or snakes.
- Natural environment type – Fear of water (aquaphobia), heights (acrophobia), lightning and thunderstorms (astraphobia), or aging (gerascophobia).
- Situational type – Fear of small confined spaces (claustrophobia), or the dark (nyctophobia).

- Blood/injection/injury type – this includes fear of medical procedures, including needles and injections (trypanophobia), fear of blood (hemophobia) and fear of getting injured.

Agoraphobia →

Agoraphobia is the specific anxiety about being in a place or situation where escape is difficult or embarrassing or where help may be unavailable. Agoraphobia is strongly linked with panic disorder and is often precipitated by the fear of having a panic attack. A common manifestation involves needing to be in constant view of a door or other escape route.

Social anxiety disorder →

Social anxiety disorder (SAD; also known as social phobia) describes an intense fear and avoidance of negative public scrutiny, public embarrassment, humiliation, or social interaction. This fear can be specific to particular social situations (such as public speaking) or, more typically, is experienced in most (or all) social interactions. Social anxiety often manifests specific physical symptoms, including blushing, sweating, and difficulty speaking.

Post-traumatic stress disorder →

Post-traumatic stress disorder (PTSD) results from a traumatic experience. Post-traumatic stress can result from an extreme situation, such as combat, natural disaster, rape, hostage situations, child abuse, bullying, or even a serious accident. Common symptoms include hypervigilance, flashbacks, avoidant behaviors, anxiety, anger and depression.

Separation anxiety disorder →

Separation anxiety disorder (SepAD) is the feeling of excessive and inappropriate levels of anxiety over being separated from a person or place. Separation anxiety is a normal part of development in babies or children, and it is only when this feeling is excessive or inappropriate that it can be considered a disorder.

Panic disorder →

Panic disorder is an anxiety disorder characterized by reoccurring unexpected panic attacks. Panic attacks are sudden periods of intense fear that may include palpitations, sweating, shaking, shortness of breath, numbness, or a feeling that something terrible is going to happen.

Obsessive compulsive disorder →

Obsessive– compulsive disorder (OCD) is a condition where the person has obsessions (distressing, persistent, and intrusive thoughts or images) and compulsions (urges to repeatedly perform specific acts or rituals), that are not caused by drugs or physical order, and which cause distress or social dysfunction.

OCD is a mental disorder where people feel the need to check things repeatedly, perform certain routines repeatedly (called "rituals"), or have certain thoughts repeatedly (called "obsessions"). Common activities include hand washing, counting of things, and checking to see if a door is locked. Some may have difficulty throwing things out. These activities occur to such a degree that the person's daily life is negatively affected.

Selective mutism →

Selective mutism (SM) is a disorder in which a person who is normally capable of speech does not speak in specific situations or to specific people. Selective mutism usually co-exists with shyness or social anxiety.

Common symptoms of Anxiety disorders →

In all types of anxiety disorders, patient may have few or all of following signs and symptoms →

Excessive worrying – Feeling agitated (Feeling or appearing troubled or nervous) – Restlessness – Difficulty concentrating – Fatigue – Irritability – Tense muscles – Sleep disorders (Trouble falling or staying asleep) – Panic attacks (palpitations, sweating, shaking, shortness of breath, numbness etc.) – Avoiding social situations

Treatment & Management →

Treatment includes lifestyle changes, counselling, and medications →

- **Lifestyle changes** include exercise, for which there is moderate evidence for some improvement, regularizing sleep patterns, reducing caffeine intake, stopping alcohol intake, and stopping smoking. Stopping smoking has benefits in anxiety as large as or larger than those of medications.
- **Counselling** is typically with a type of cognitive behavioral therapy. **Cognitive behavioral therapy** (CBT) is effective for anxiety disorders and is a first line treatment. Self-help books can contribute to the

treatment of people with anxiety disorders. Mindfulness based programs also appear to be effective for managing anxiety disorders.

- **Medications →**

- SNRIs are first line choices for generalized anxiety disorder.
- SNRIs – Serotonin–norepinephrine reuptake inhibitors are a class of antidepressant drugs, e.g. Desvenlafaxine, Venlafaxine, Duloxetine, Levomilnacipran etc.
*(*See examples of SSRIs and SNRIs under Antidepressants – page np. 57)*
- Bupirone, quetiapine and pregabalin are second line treatments for people who do not respond to SSRIs or SNRIs.
- There is also evidence that benzodiazepines including diazepam and clonazepam are effective but have fallen out of favor due to the risk of dependence and abuse.

Stress induced disorders

Stress induced disorders include mental health disorders that are a result of an atypical response to both short and long-term anxiety due to physical, mental, or emotional stress.

These disorders include obsessive-compulsive disorder, post-traumatic stress disorder, and acute stress disorder etc.

Stress is a conscious or unconscious psychological feeling or physical condition resulting from physical or mental 'positive or negative pressure' that overwhelms adaptive capacities. A person typically is stressed when positive or negative (e.g., threatening) experiences temporarily strain or overwhelm adaptive capacities. Stress is highly individualized and depends on variables such as the novelty, rate, intensity, duration, or personal interpretation of the input, and genetic or experiential factors.

Stress-reduction strategies can be helpful to many stressed / anxious person. The five R's of stress and anxiety reduction →

- Recognition of the causes and sources of the threat or distress; education and consciousness raising.
- Relationships identified for support, help, reassurance
- Removal from (or of) the threat or stressor; managing the stimulus.
- Relaxation through techniques such as meditation, massage, breathing exercises, or imagery.
- Re-engagement through managed re-exposure and desensitization.

Depression

Depression is a **state of low mood** and **aversion to activity** that can affect a person's thoughts, behavior, tendencies, feelings, and sense of well-being.

A depressed mood is a normal temporary reaction to life events such as loss of a loved one. It is also a symptom of some physical diseases and a side effect of some drugs and medical treatments. Depressed mood is also a symptom of some mood disorders such as major depressive disorder or dysthymia (persistent depressive disorder – PDD).

Causative factors of depression →

1. Life events →

- Life events and changes that may precipitate depressed mood include (but are not limited to): childbirth, menopause, financial difficulties, unemployment, stress (such as from work, education, family, living conditions etc.), a medical diagnosis (cancer, HIV, etc.), bullying, loss of a loved one, natural disasters, social isolation, rape, relationship troubles, jealousy, separation, and catastrophic injury.
- Adolescents may be especially prone to experiencing depressed mood following social rejection, peer pressure and bullying.
- Adversity in childhood, such as bereavement, neglect, mental abuse, physical abuse, sexual abuse, and unequal parental treatment of siblings can contribute to depression in adulthood.

2. Personality →

- People with high neuroticism indexes are at risk for the development and onset of common mental disorders, such as mood disorders, anxiety disorders, and substance use disorder, symptoms of which had traditionally been called neuroses.
- Other personality indicators could be: temporary but rapid mood changes, short term hopelessness, loss of interest in activities that used to be of a part of one's life, sleep disruption, withdrawal from previous social life, appetite changes, and difficulty concentrating,

3. Gender identity and sexuality →

- Studies have shown that those who identify as LGBT are more prone to depression.

4. Medical treatments →

- Depression may also be iatrogenic (the result of healthcare), such as drug induced depression.
- Therapies associated with depression include interferon therapy, beta-blockers, Isotretinoin, contraceptives, cardiac agents, anticonvulsants, antimigraine drugs, antipsychotics, and hormonal agents such as gonadotropin-releasing hormone agonist.

5. Substance– induced →

- Several drugs of abuse can cause or exacerbate depression, whether in intoxication, withdrawal, and from chronic use.
- These include alcohol, sedatives (including prescription benzodiazepines), opioids (including prescription pain killers etc.)

6. Non-psychiatric illnesses →

- Depressed mood can be the result of a number of infectious diseases, nutritional deficiencies, neurological conditions and physiological problems, including hypoandrogenism (in men), Addison's disease, Cushing's syndrome, hypothyroidism, multiple sclerosis, Parkinson's disease, chronic pain, stroke, diabetes, and cancer.

7. Psychiatric syndromes →

- A number of psychiatric syndromes feature depressed mood as a main symptom. The mood disorders are a group of disorders considered to be primary disturbances of mood.
- Mood disorders include major depressive disorder, dysthymia (persistent depressive disorder – PDD), bipolar disorder etc.
- Other than mood disorders → borderline personality disorder, posttraumatic stress disorder etc.

Treatment and Management →

- The physician should try to treat the patient with counseling and advising life style changes like Staying active – Restoring a regular sleep pattern – Controlling worry and reduce negative thinking etc.
- **Psychological treatments for depression** → There are several different types of psychological treatments including →
 - Cognitive behaviour therapy (CBT)
 - Interpersonal therapy (IPT)
 - Behaviour therapy
 - Mindfulness-based cognitive therapy (MBCT).

• Antidepressant drugs →

1. Tricyclic antidepressants (TCA)
 - Amitriptyline
 - Clomipramine
 - Desipramine
 - Doxepin
 - Imipramine
 - Nortriptyline
2. Selective serotonin reuptake inhibitors (SSRI)
 - Escitalopram
 - Fluoxetine
 - Fluvoxamine
 - Sertraline
3. Serotonin norepinephrine reuptake inhibitors (SNRI)
 - Duloxetine
 - Venlafaxine
 - Desvenlafaxine
4. Monoamine oxidase inhibitors (MAOI)
 - Phenelzine
 - Selegiline
 - Tranylcypromine
 - Isocarboxazid

Somatoform and mood disorders

Somatoform disorders → A somatoform disorder (or somatic symptom disorder) is a group of psychiatric disorders that cause unexplained physical symptoms. They include somatization disorder, undifferentiated somatoform disorder, hypochondriasis, conversion disorder, pain disorder, body dysmorphic disorder, and somatoform disorder not otherwise specified.

The unexplained symptoms of somatoform disorders often lead to general health anxiety; frequent or recurrent and excessive preoccupation with unexplained physical symptoms; difficult encounters with the health care system; disproportionate disability; displays of strong, often negative emotions toward the physician or office staff; and, occasionally, resistance to or noncompliance with diagnostic or treatment efforts.

Treatment →

- Psychotherapy, more specifically, cognitive behavioral therapy (CBT) can help in some of the following ways →
 - Learn to reduce stress
 - Learn to cope with physical symptoms
 - Learn to deal with depression and other psychological issues
 - Improve quality of life
 - Reduce preoccupation with symptom
- Antidepressant drugs such as SSRIs and SNRIs can also be used.

Mood disorders → Mood disorder is a group of conditions where a disturbance in the person's mood is the main underlying feature.

Mood disorders include →

- Major depressive disorder (MDD),
- Dysthymia – Persistent depressive disorder (long lasting low grade depression),
- Bipolar disorder (mania - euphoric, hyperactive, over inflated ego, unrealistic optimism),
- Cyclothymia (a mild form of bipolar disorder), and
- SAD (seasonal affective disorder).

Major depressive disorder →

Major depressive disorder (MDD), commonly called major depression, unipolar depression, or clinical depression, wherein a person has one or more major depressive episodes.

Bipolar disorder →

Bipolar disorder (BD) (also called Manic Depression or Manic-Depressive Disorder), an unstable emotional condition characterized by cycles of abnormal, persistent high mood (mania) and low mood (depression).

Seasonal affective disorder →

Seasonal affective disorder (SAD) is a type of depression that's related to changes in seasons – SAD begins and ends at about the same times every year.

Treatment & Management →

- There are different types of treatments available for mood disorders, such as therapy and medications.
 - Behaviour therapy
 - Cognitive behaviour therapy
 - Interpersonal therapy
- Major depressive disorder medications usually include antidepressants, while bipolar disorder medications can consist of antipsychotics, mood stabilizers, and anticonvulsants.
- Antipsychotics drugs →
(Also known as neuroleptics or major tranquilizers)
 - Asenapine
 - Clozapine
 - Olanzapine
 - Quetiapine
 - Risperidone
- Mood stabilizers →
 - Lithium
 - Carbamazepine
 - Lamotrigine
 - Valproate
 - Asenapine

Psychosexual disorders

The sexual problems that are psychological in origin, rather than physiological, are called psychosexual disorders.

Causes →

- It may arise due to guilty conscience, stress, anxiety, nervousness, worry, fear, depression, physical or emotional trauma, abuse, rape and so on.
- The other factors causing sexual problems are based on psychosocial and cultural aspects. Ignorance or improper sex education can be a reason.
- Relationship aspects, like power, trust or anger at partner and lack of communication and commitment to one's partner or peer may also lead to sexual disorders.

Symptoms →

- Psychosexual disorder symptoms vary for every individual and differ with gender. They may be categorized into sexual dysfunction, paraphilias and gender identity disorders.
- **Dysfunction** is a sexual problem characterized by →
 - Dyspareunia or experience of more pain during the act. Decreased desire or Sexual aversion is associated with fear, anxiety and dislike.
 - Impotence, premature ejaculation, inhibited male orgasms and vaginismus (involuntary contraction of the vagina in female) are the visible symptoms of sexual problems.

- **Paraphilias** or the disorder of sexual preference is the unusual or abnormal sexual behavior that does not follow the normal standards. This category of sexual disorder exhibits abnormal preference of sexual objects that include →
 - Fetishism (preference for inanimate objects),
 - Fetishistic transvestism (wearing clothes of the opposite sex),
 - Paedophilia (pre-pubertal children),
 - Zoophilia (animals),
 - Necrophilia (dead bodies) etc.
- **Sexual perversions** are also due to some type of psychosexual disorder and include sexual sadism, exhibitionism, voyeurism and frotteurism.
- **Gender identity disorders** exhibit variation between one's sense of sexual identity and the biological one. The sufferer shows doubt about one's own sexual orientation, difficulty to adjust to the normal lifestyle, and a desire to modify the sexual orientation to become a member of the opposite sex.

Treatment & Management →

- Sex therapy, behavioral therapy, and group therapy may be helpful to those suffering distress from sexual dysfunction.
- More serious sexual perversions may be treated with androgen blockers such as cyproterone acetate, medroxyprogesterone acetate, and GnRH modulators, or SSRIs such as Escitalopram, Fluoxetine, Fluvoxamine etc. to help restore hormonal and neurochemical balances.

भूत विद्या

भूत विद्या is a branch of the eight principle branches of अष्टांग आयुर्वेद. It is also called ग्रह चिकित्सा and deals with psychology and psychiatry (मानसिक विकार). In Ayurveda, it is used for the diagnosis and treatment of diseases resulting from unknown reasons.

Different experts have explained the word 'भूत' differently →

- Some experts say that 'भूत' means ghosts and similar evil spirits who cause abnormal psychological conditions.
- Others say 'भूत' represents microscopic organisms such as virus, bacteria that are not visible to naked eye.
- In modern terminology, it can be considered as idiopathic diseases in which the exact cause of disease is unknown.

Ayurveda also believes in the past karma or deeds as a causative factor of certain diseases. Bhuta Vidya deals with the causes, which are directly not visible and have no direct explanation in terms of tridosha.

In most cases illness is caused by the disturbance of mind, where rajas (passion) and tamas (ignorance) are supposed to be the contributing factors. These psychiatric diseases (मानसिक विकार) are a result of the affliction from the देव, असुर, गन्धर्व, यक्ष, राक्षस, पिशाच, नाग, पितृ and other demons or evil spirit.

The problems and ailments dealt under the Graha Chikitsa or Bhut Vidya can be related to modern psychiatry.

The treatment include →

- आश्रवासन or मनसोपचार → Counselling of patient.
- ग्रहविद्या द्रव्य प्रयोग → Use of certain herbal disinfectants as fumigation (धूपन) or simply keeping the sticks or leaves in the room to disinfect the atmosphere of the patient.
Examples of धूपन द्रव्य → गुग्गुलु, अगरु, वचा, निम्ब, महानिम्ब (बकायन), सर्षप, पिप्पली, चित्रक, हिंगु, चन्दन, कर्पूर etc.
- मन्त्रोच्चारण & अनुष्ठान → Chanting mantras and performing tantric rituals to pacify the affliction of evil spirits and demons.
- योग → Use of yogic therapies like ध्यान (meditation) and प्राणायाम to pacify the psychological disturbance of a patient.

मेध्य-रसायन प्रयोग

मेध्य-रसायन द्रव्य play an essential role in the treatment of psychiatric and psychosomatic diseases. The mode of this therapy involves the individual to attain sedation, calmness, tranquility or a stimulation of activities of brain.

Neurological and psychiatric disorders are generally associated with loss of memory, cognitive deficits, impaired mental function etc. Medhya rasayanas are known to have specific effect on mental performance by promoting the functions of “बुद्धि” and “मनस्” by correcting the disturbances of “रजस्” and “तमस्”. These drugs promote the Intellect (धी), Retention power (धृति), and memory (स्मृति). e.g. मण्डुकपर्णी स्वरस, यष्टिमधु चूर्ण क्षीर, गुडूची स्वरस, शंखपुष्पी चूर्ण, ब्राह्मी स्वरस, वचा, ज्योतिष्मति, जटामांसी etc.

रसायन (Rejuvenatives)

Definition of Rasayana →

लाभोपायो हि शस्तानां रसादीनां रसायनम् । (च.चि.1/1/7)

That which help in attaining the प्रशस्त रस-रक्तादि धातु (i.e. body tissues of superior qualities), by nourishing and strengthening the body, is known as रसायन.

यज्जराव्याधिविध्वांसि वयः स्तम्भकरं तथा ।

चक्षुष्यं बृंहणं वृष्यं भेषजं तद् रसायनम् ॥ (भा.प्र.)

According to Bhavamishra, रसायन द्रव्य is having following features →

- जरा विध्वंसि (जराहर) → Anti-senility (preventing old-age)
- व्याधि विध्वंसि (व्याधिहर) → Destroying the disease
- वयः स्तम्भकर (वयस्थापन) → Age-sustaining (maintain youthfulness)
- चक्षुष्य → Promoting vision (Good for eyes)
- बृंहण → Nourishment of dhatus (Increasing the bulk of body)
- वृष्य → Promoting virility (Increasing the strength)
- भेषज → Acts as quality drug

Benefits (Importance of Rasayana) →

दीर्घमायुः स्मृतिं मेधामारोग्यं तरुणं वयः ।

प्रभा वर्णं स्वरौदार्यं देहेन्द्रियं बलं परम् ॥

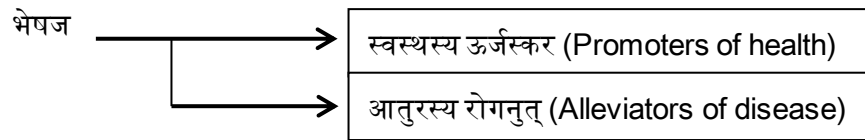
वाक्सिद्धिं प्रणतिं कान्तिं लभते ना रसायनात् ।

लाभोपायो हि शस्तानां रसादीनां रसायनम् ॥ (च.चि.1/1/7)

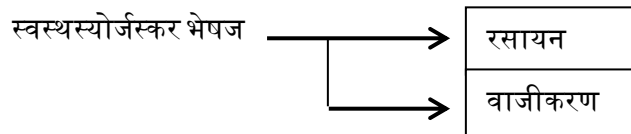
By using of Rasayana, person achieves long life, sharp memory, intelligence, optimum health, youth, luster and better colour complexion of skin, better voice, optimum strength of the body and sense organs, vaaksiddhi (what he says becomes true), popularity, brilliance etc.

Types →

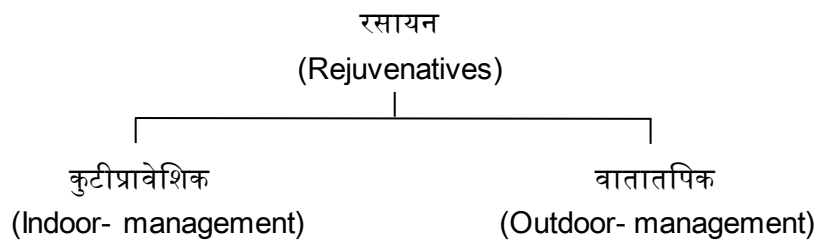
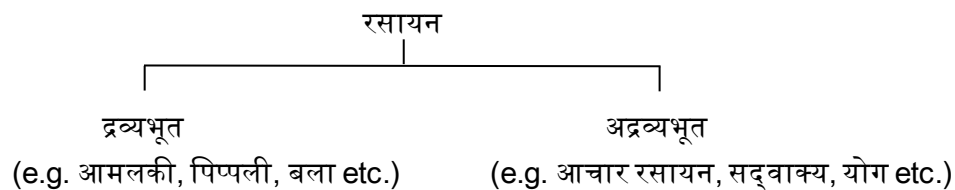
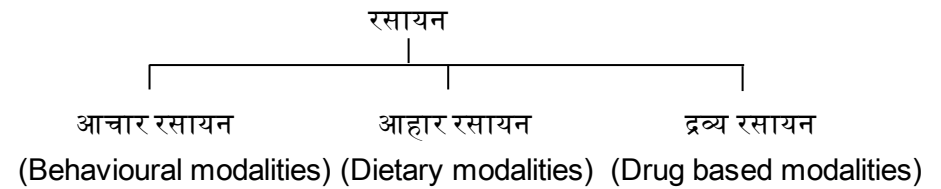
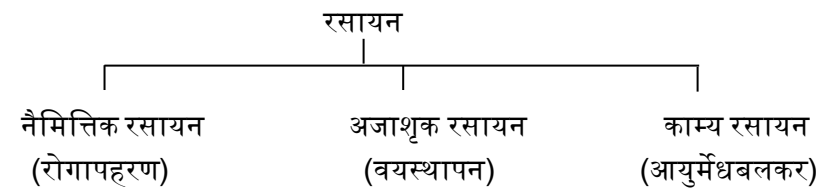
In Rasayana therapy, the Bheshja (medicine) is divided into two types →



स्वस्थस्य ऊर्जस्कर भेषज is again divided into two types →



रसायन can be classified variously, on the basis of following factors →

(I) According to mode of administration → 2 types**(II) According to dravya etc. → 2 types****(III) According to modalities → 3 types****(IV) According to achievable outcome → 3 types**

नैमित्तिक रसायन (निमित्त means cause) is also known as रोगापहरण or Curative type of Rasayana. It is used to combat or balance a specific cause responsible for the disease in the body. For example, त्रिफला रसायन for नेत्र-रोग, अर्जुन for हृद्रोग, लोहभस्म for पाण्डु, अगस्त्य रसायन for श्वास, शिलाजतु for प्रमेह, मेध्य रसायन for उन्माद-अपस्मार etc..

अजाशृक रसायन is used to maintain good health and improve the quality of life through a healthy lifestyle, diet, or exercise. It is also called as वयस्थापन रसायन. Few examples include क्षीर, घृत, अन्न, फल सेवन, योग-प्राणायाम, सद्वृत्त पालन etc.

काम्य रसायन is described as to fulfill a wish or desire or to serve a special purpose (काम means desire).

It is of three types →

1. **प्राण काम्य** → to achieve best quality of prana (life energy) in the body. e.g., आमलकी, हरीतकी, विभीतकी, नागबला, अश्वगन्धा रसायन etc.
2. **मेधाकाम्य** → enhancing the memory and intellect. e.g., मण्डूकपर्णी, यष्टिमधु, गुडूची, शंखपुष्पी, ब्राह्मी, वचा, ज्योतिष्मति, जटामांसी रसायन etc.
3. **श्रीकाम्य** → to promote the complexion and luster of a person. These rasyana drugs are devine herbs (दिव्य औषधि).

कुटीप्रावेशिक रसायन प्रयोग विधि →

- First कुटी is prepared – Kuti should be constructed where the kings and noble people live; required things are easily available, built in north east direction. Kuti should have 3 chambers (त्रिगर्भ) with small windows and thick walls.
- Patient, free from रज & तम दोष, with concentration, faith and patience enters the कुटी on auspicious day in the presence of सूर्य, शुक्लपक्ष, शुभ नक्षत्र & मुहूर्त after offering the पूजा & प्रदक्षिणा to the देवता & ब्राह्मण.
- संशोधन i.e. वमन, विरेचन, निरूह-बस्ति, and नस्य should be administered.
- पथ्य or संसर्जन क्रम should be followed.
- रसायन प्रयोग – The suitable rasayana therapy should be adopted with consideration of रोगी-रोग बल & फलश्रुति.

Importance of शोधन before administration of रसायन चिकित्सा →

Acharyas has mentioned that “As dirty clothes (मलिन वस्त्र) cannot be coloured properly, patients without शोधन (अविशुद्ध शरीर) cannot get proper result of रसायन & वाजीकरण”.

Indications (रसायन योग्य) →

- जितेन्द्रिय (who can control his sense organs)
- हितायु & सुखायु व्यक्ति (who wants to live healthy life)
- साधन सम्पन्न (who can afford the treatment)
- सावधान व्यक्ति (cautious)
- धार्मिक
- Non-addicted
- Free from mental stress
- Between 16 to 70 yrs age
- Male and female both are eligible
- शोधित & स्निग्ध

Contraindications (रसायन अयोग्य) →

- अनात्मवान (unstudied person)
- आलसी (lazy person)
- दरिद्र (poor – who cannot afford the treatment)
- प्रमादी & व्यसनी (careless and addicted persons)
- पापकृत् (evil person who indulges in sinful acts)
- भेषजापमानी (who do not respect medicine)
- पतित (characterless)
- सुश्रुषा रहित (who do not have care-taker)

‘Ideal age’ for Rasayana-therapy → ‘परिहारिणी अवस्था’ i.e. 40 + age is the ideal age for initiation of रसायन चिकित्सा, as during this stage all the dhatus reach ‘Peak’ stage (fully matured) and decline is likely to set in, if Rasayana are administered at this critical age, the dhatus may remain intact and sustain the perfect health further.

आचार रसायन (Rejuvenation by means of good conduct) → आचार रसायन deals with practice of ethics (आचार) and regimen (विहार), collectively known as 'behavioural science' to gain the excellence of सत्व-बुद्धि and चेतन (आत्मा).

सत्यवादिनं अक्रोधं निवृत्तं मद्यमैथुनात् । अहिंसकं अनायासं प्रशान्तं प्रियवादिनम् ॥
जपशौचकरं धीरं दाननित्यं तपस्विनम् । देवगोब्राह्मणाचार्यं गुरुवृद्धार्चने रतम् ॥.....
(च.चि.1/4/30-35)

- सत्यवादिनं – To be honest / trustworthy.
- अक्रोधं – To be free from anger.
- निवृत्तं मद्यमैथुनात् – To walk away from wine and women (sex).
- अहिंसकं – To be non-violent.
- अनायासं – Never be tired, but be cheerful.
- प्रशान्त – To keep cool and calm.
- प्रियवादिनं – To speak good / talk sweetly.
- जपशौचकरं – To practice mantra-chanting and is cleanliness
- धीरं दाननित्यं – To be courageous and charitable (donates & serves to poor).
- तपस्विनम् – To practice yoga, meditation etc.
- देवगोब्राह्मणाचार्यगुरुवृद्धार्चनेरतं – Devoted to Gods, cows, brahmins, sages, teachers, elders, and serving them.
- नित्यं करुणवेदिनं – Ever compassionate, merciful.
- सम जागरण स्वप्नं – Balanced sleep and awakening.
- नित्यं क्षीर घृताशिनं – Using ghee and milk regularly.
- देशकाल प्रमाणज्ञं – To be aware of place & time, and act accordingly.
- शस्ताचारं असंकीर्णं – To be well behaved and simple (transparent).
- उपासितारं वृद्धानां – Accompanying (experienced) elders.
- आस्तिकानां – To have faith in God.
- जितात्मवान् – To be self controlled, non-submissive to sensory.

Rabin Singh

मेध्य रसायन →

मेध्य रसायन promote the intellect (धी), retention power (धृति), and memory (स्मृति). They provide मेधा, आरोग्य, आयु, अग्निबल, देहबल, वर्ण, and स्वर. आचार्य चरक has mentioned four मेध्य रसायन → मण्डूकपर्णी स्वरस, यष्टिमधु चूर्ण with दुग्ध, गुडूची स्वरस, and शंखपुष्पी कल्क.

1. **मण्डूकपर्णी** → act on behaviour besides being neuroprotectives brain growth promoter, inhibits the memory impairment induced by scopolamine through the inhibition of AChE.
2. **यष्टिमधु** → it increases the circulation into the CNS system, improves learning and memory on scopolamine induced dementia.
3. **गुडूची** → possess learning and memory enhancing, antioxidant, and anti-stress action, enhances the cognition in normal and cognition deficits animals in behavioural test. It is useful for treatment of bhrama (Vertigo), in improving behavior disorders, mental deficit and IQ levels.
4. **शंखपुष्पी** → effective in chittodvega (anxiety disorders), reverses the social isolation stress induced prolongation of onset and decrease in pentobarbitoneinduced sleep, increased total motor activity and stress-induced antinociception in experimental model.

The formulation of Medhya Rasayana drugs are of two types →

1. **शीत वीर्य and मधुर विपाक** – it promotes kapha and enhances “धारण कर्म” (i.e., retention of cognition) e.g., यष्टिमधु, ब्राह्मी, शंखपुष्पी etc.
2. **उष्ण वीर्य and तिक्त रस** – it promotes pitta and enhances ग्रहण and स्मरण (i.e., grasping power and Memory) e.g., गुडूची, वचा, ज्योतिष्मति etc.

Examples of Medhya Rasayana →

मण्डूकपर्णी – यष्टिमधु – गुडूची – शंखपुष्पी – ज्योतिष्मति – ब्राह्मी – वचा – जटामांसी – अश्वगन्धा रसायन – सतत अध्ययन – वाद – सत्संग etc.

Rasayana according to Srotas →

For giving strength and increasing immunity of the Srotas or channel systems following herbal, mineral or metallic preparations can be given.

- प्राणवह → पिप्पली, मरिच and सहस्र पुटी अभ्रक etc.
- अन्नवह → शुण्ठी (आर्द्रक), लशुन, मरिच, हिंगु, शंख भस्म etc.
- उदकवह → फल स्वरस (all fruit juices), नारिकेलोदक (coconut water).
- रसवह → शतावरी, गुडूची, खर्जूर etc.
- रक्तवह → मंजिष्ठा, सारिवा, धामासा etc.
- मांसवह → अश्वगन्धा, मांसरस, स्वर्ण भस्म etc.
- मेदोवह → गुग्गुलु, शिलाजतु, व्यायाम etc.
- अस्थिवह → बबूल निर्यास, प्रवाल, मुक्ता etc.
- मज्जावह → यष्टिमधु, ब्राह्मी, शंखपुष्पी, जटामांसी, रजत भस्म, अक्षोट (walnut), बादाम (almonds), केशर (saffron) etc.
- शुक्रवह → कपिकच्छू, अश्वगन्धा, मुसली, शतावरी, घृत, क्षीर etc.
- मूत्रवह → गोक्षुर, पुनर्नवा, वरुण, नारिकेलोदक etc.
- पुरीषवह → बिल्व, हरीतकी, त्रिफला etc.
- स्वेदवह → धतूर, चन्दन etc.
- मनोवह → मेध्य रसायन – मण्डूकपर्णी, यष्टिमधु, गुडूची, शंखपुष्पी, ब्राह्मी, वचा, ज्योतिष्मति, जटामांसी etc.
- आर्तववह → अशोक, लोध्र, शतावरी etc.
- स्तन्यवह → शतावरी, मेथिका etc.
- जीवनीय गण द्रव्य → जीवक, ऋषभक, मेदा, महामेदा, काकोली, क्षीरकाकोली, मुद्गपर्णी, माषपर्णी, जीवन्ती, and यष्टिमधु.

Rasayana according to Diseases →

Eye diseases → त्रिफला – ज्योतिष्मति – यष्टिमधु – धात्री लौह

Tuberculosis → पिप्पली – लशुन – नागबला – शिलाजतु

Heart diseases → अर्जुन – शालपर्णी – पुष्करमूल – गुग्गुलु

Skin diseases → बाकुची – खदिर – गंधक – भल्लातक – तुवरक – विडंग

Joint diseases → शल्लकी – अमृत भल्लातक – रसोन

Diabetes → शिलाजतु – हरिद्रा – आमलकी

Anemia → आमलकी – पुनर्नवा – लौह रसायन

Grahani & Gulma → पिप्पली – भल्लातक

Hypertension → रसोन – सर्पगन्धा – बला – रास्ना – मेध्य रसायन

Hypotension → कुपीलु – कस्तूरी

Asthma → हरिद्रा – रास्ना – शिरीष – अगस्त्य रसायन

Allergies → हरिद्रा

Lipid disorders → गुग्गुलु – रसोन – हरीतकी

Brain & Mental disorders → ब्राह्मी – शंखपुष्पी – ज्योतिष्मति

Rasayana and organs →

Brain – मेध्य रसायन like ब्राह्मी, शंखपुष्पी etc., Heart – अर्जुन,
Kidneys – गोक्षुर and पुनर्नवा., Pancreas – मेषशृंगी and दारुहरिद्रा,
Small Intestine – त्रिकटु and चित्रक., Large Intestine – पर्पटी-preparations,
Eyes – त्रिफला.

Rasayana herbs according to Prakriti →

- वात प्रकृति – घृत, दुग्ध, अश्वगन्धा, बला, गुडूची, शंखपुष्पी, स्वर्णभस्म etc.
- पित्त प्रकृति – घृत, दुग्ध, आमलकी, च्यवनप्राश, शतावरी, भृंगराज, ब्राह्मी, रजत etc.
- कफ प्रकृति – मधु, त्रिकटु, त्रिफला, हरीतकी, ब्रह्म रसायन, बाकुची etc.

Rasayana and Stages of Life →

According to Sharangadhara – Administration of Rasayana as per Age-group for supplementation of loss →

Age-group in years	Loss of impact / Desired effect	Rasayana Recommended
1 to 10	बाल्य (Childhood)	वचा, स्वर्ण भस्म
11 to 20	वृद्धि (Growth)	काश्मरी, बला, शतावरी
21 to 30	छवि (Complexion)	आमलकी, लोह भस्म
31 to 40	मेधा (Intellect)	मेध्य रसायन like शंखपुष्पी, ब्राह्मी etc.
41 to 50	त्वक (Skin)	ज्योतिष्मति, तुवरक, सोमराजी, भृंगराज
51 to 60	दृष्टि (Vision)	त्रिफला, ज्योतिष्मति, धात्रीलौह
61 to 70	शुक्र (Semen)	कपिकच्छू, अश्वगन्धा, शिलाजतु
71 to 80	विक्रम (Power / Courage)	आमलकी, बला, महाबला, अश्वगन्धा
81 to 90	बुद्धि (Wisdom)	ब्राह्मी
91 to 100	कर्मेन्द्रिय (Motor functions)	Rasayana not effective

Purpose of Rasayana →

Rasayana should be used for three fold purpose →

1. Maintaining health,
2. Prevention of diseases and old age and
3. To aid in treatment, and to avoid the recurrence of diseases.

Sharangadhara has advised that in each group of age, every person should take specific rasayana for keeping healthy and fit.

पथ्य & अपथ्य → (During Rasayana Therapy)

पथ्य (Beneficial) →

पथ्य आहार → गोघृत – गोदुग्ध – अजा दुग्ध – मधु – शीतल जल – षष्टिक शालि – मुद्ग यूप – यव यवागु – लाजा – जांगल मांसरस etc.

पथ्य विहार → सतत अध्ययन – वाद – सत्संग – लघु व्यायाम – अभ्यंग – उत्सादन – स्नान – स्वच्छ वस्त्र धारण – बृंहण कर्म – विश्राम – अचिन्ता etc.

अपथ्य (Harmful) →

अपथ्य आहार → ग्राम्याहार → अति अम्ल – अति लवण – कटु – क्षार – शुष्क शाक – शुष्क मांस – तिल – तिलपिष्ट – माष – विरूढ अन्न (sprouted grains) – नव अन्न (newly harvested grains) – विरूढ आहार – असात्म्य आहार etc.

गुरु – रूक्ष – तीक्ष्ण – विदाही – अभिष्यन्दी आहार – मद्य – अध्यशन – विषमाशन etc.

अपथ्य विहार → विषम व्यायाम – अति व्यायाम – अति व्यवाय – दिवास्वप्न – रात्रिजागरण – वेगधारण – आतप सेवन – मारुत सेवन – क्रोध – शोक – भय – ईर्ष्या etc.

रसायन योग (Formulations) →

च्यवनप्राश

Ingredients:

- प्रधान द्रव्य – आमलकी
- 36 क्वाथ द्रव्य – बिल्व, अग्निमन्थ, श्योनक, गम्भारी, पाटला, बला, माषपर्णी, मुद्गपर्णी, पृश्नपर्णी, शालपर्णी, पिप्पली, गोक्षुर, बृहती, कण्टकारी, कर्कटशृङ्गी, भूम्यामलकी, द्राक्षा, जीवन्ती, पुष्करमूल, अगरु, हरीतकी, गुडूची, ऋद्धि, जीवक, ऋषभक, शटी, मुस्ता, पुनर्नवा, मेदा, एला, चन्दन, उत्पल, विदारी, वृषमूल, काकोली, and काकनासिका.
- प्रक्षेपक द्रव्य – मधु, तुगा, पिप्पली, त्वक, एला, पत्र, and नागकेशर.

Preparation:

- All these herbs are taken 1-1 pala each in coarse powdered form and cooked in 1 drona of water. A pottali of 500 fruits of आमलकी is immersed in this mixture to be cooked. Boil the water till it is reduced to ¼ then pottali is removed and decoction is filtered.
- Seeds of Amalaki fruits are removed and paste is made. Fibers are removed by straining it through cloth.
- Fry this paste of Amalaki in the mixture of घृत & तिल तैल till the moisture is evaporated.
- Add sugar (मिश्री) to decoction and boil to prepare syrup, to this add fried paste of Amalaki and cook till it becomes अवलेह. After cooling add मधु (honey) and प्रक्षेपक द्रव्य – चतुर्जाति (त्वक एला तेजपत्र नागकेशर).

Dose: the quantity that should not alter the food intake.

Indications: राजयक्ष्मा, कास, श्वास, क्षतक्षीण, वृद्ध, बाल, स्वरक्षय, हृद्रोग, वातरक्त, पिपासा, मूत्रविकार etc.

Benefits: Best rejuvenative and anti-aging medicine.

Rabin Singh

प्रथम ब्रह्म रसायन

Preparation:

This consists of five types of roots (पञ्च पञ्चमूल):

- लघु पञ्चमूल – गोक्षुर बृहती कण्टकारी शालिपर्णी पृश्निपर्णी
- बृहत पञ्चमूल – बिल्व अग्निमन्थ श्योनक गम्भारी पाटला
- मध्यम पञ्चमूल (पुनर्नवादि) – बला पुनर्नवा एरण्ड मुद्गपर्णी माषपर्णी
- जीवनीय पञ्चमूल – जीवन्ती जीवक ऋषभक मेदा महामेदा
- तृण पञ्चमूल – कुश काश नल दर्भ काण्डेशु

Take all 25 drugs, 10 pala (50 gms) each with 1000 fruits of हरीतकी and 3000 fruits of आमलकी, add 10 times water. Boil it to prepare decoction. Take the fruits of हरीतकी & आमलकी out and remove seeds then prepare paste of these cooked fruits. Mix decoction and paste again and add मण्डूकपर्णी, पिप्पली, शंखपुष्पी, मुस्ता, विडंग, चन्दन, अगरु, यष्टिमधु, हरिद्रा, वचा, त्वक, एला चूर्ण to it (4 pala each). Then add 1100 pala of मिश्री (sugar), 2 adhaka of तिल तैल and 3 adhaka of घृत. Cook the mixture on low fire till अवलेह (jam) is prepared. After cooling add मधु (honey) to it.

Dose: It should be taken in such quantity that doesn't alter appetite of the person that means it should get digested before the time of food.

Benefits: By the administration of this rasayana, the sages (तपस्वी) regained their youth (तरुण वय) and achieved मेधा, स्मृति & बल. One, who seeks निरापद दीर्घायु and तारुण्य, should use this rasayana.

द्वितीय ब्रह्म रसायन

Preparation:

- Take 1000 fruits of आमलकी, remove the seeds and cook them well in milk. Then dry the mixture in sun and triturate it with the आमलकी स्वरस.
- Add पुनर्नवा, जीवन्ती, बला, नागबला, शतावरी, कपिकच्छू, गुडूची, चन्दन, अगरु, ब्राह्मी, मण्डूकपर्णी, शंखपुष्पी, पिप्पली, वचा and विडंग. Triturate the mixture again with

नागबला स्वरस. Add double the quantity of mixture of मधु & घृत to it. Keep the mixture in jar for 15 days. Then add स्वर्णभस्म, रजतभस्म, ताम्रभस्म, and प्रवालभस्म.

Dosage: According to agni bala of the person and the quantity that should not alter the food intake.

After digestion of medicine, the person should take Shastika Shali with milk and ghee.

Indications: it is indicated for Kutipraveshika rasayana.

मेध्य रसायन

1. मण्डूकपर्णी स्वरस → 10-15 ml with मधु (honey) twice a day.
2. यष्टिमधु चूर्ण → 2-3 gms with क्षीर (milk) twice a day.
3. गुडूची स्वरस → 10-15 ml with equal quantity of मधु twice a day.
4. शंखपुष्पी चूर्ण → 3 gms with क्षीर twice a day.

Benefits: मेध्य रसायन promote the intellect (धी), retention power (धृति), and memory (स्मृति). They provide मेधा, आरोग्य, आयु, अग्निबल, देहबल, वर्ण, and स्वर.

हरीतक्यादि रसायन

Preparation:

- A decoction is prepared of हरीतकी, आमलकी, विभीतकी & पञ्च पञ्चमूल.
- Add to it paste prepared from the mixture of पिप्पली, यष्टिमधु, काकोली, क्षीरकाकोली, कपिकच्छू, जीवक and ऋषभक. Add विदारी स्वरस, क्षीर and घृत and cook properly.

Benefits: This is excellent tonic for old age and increases functions of all sense organs and all systems of the body.

आमलक रसायन

Preparation:

- आमलकी, हरीतकी & विभीतकी or combination of these, is tied to the bark of पलाश. This is covered with a layer of wet mud. This big ball is fried using cow dung cake fire.
- Then the mud and Palasha layers are scrapped, pulp is taken and crushed properly in khalva yantra.
- This paste is administered by adding equal quantities of दधि, घृत, मधु, तिल कल्क and मिश्री.

Use: During administration of this rasayana, person should be restrained from food, if he feels hunger, should take milk or fruits.

Benefits: By the administration of this rasayana, the sages regained their youth and lived for many hundreds of years.

आमलकावलेह

1000 Amalaki and 1000 Pippali are soaked in Ksharodaka of Palasha. Then these are dried, powdered, mixed with Ghrita, Madhu & Sharkara and kept in a ghee smeared jar. This jar is stored under ground for 6 months.

विडंगावलेह

All the ingredients विडंग चूर्ण (1 आढक), पिप्पली चूर्ण (1 आढक), सितोपला चूर्ण (1½ आढक) and घृत, तैल, मधु (1 आढक each) are mixed and then kept in a ghee smeared jar. This jar should be stored inside a heap of ashes for complete rainy season.

नागबला रसायन

Roots of Nagabala should be collected from Jangala desha, the earth of the field should be snigdha, krishna, madhura or suvarna varna. The field should be free from poison, wild animals and pavana-salila-agni-dosha., should not be

cultivated land, no ant-hill, no crematorium, no sacred temple, no residential house etc. Roots of Nagabala should be collected in the month of माघ & फाल्गुन. The roots are made to a thin paste or powder and taken with milk or honey in the morning.

इन्द्रोक्त रसायन

(First rasayana told by Lord Indra)

Regular use of drugs like ऐन्द्री, ब्राह्मी, पयस्या, क्षीरपुष्पी, श्रावणी, महाश्रावणी, शतावरी, विदारी, जीवन्ती, पुनर्नवा, नागबला, वचा, मेदा, महामेदा with milk (क्षीर) for 6 months, improves स्वर – वर्ण – बल – मेधा – स्मृति.

द्रोणी-प्रावेशिका रसायन

- The juice (स्वरस) of- ब्रह्म सवर्चला – सूर्यकान्ता – नारी – काष्ठगोधा – सर्पा – सोम – पद्मा – अजा – नीला should be taken internally and then the person enter the द्रोणी (boat-like tub) made of पलाश, after smearing his body with घृत.
- He stays unconscious for 6 months in the boat and then awakens completely rejuvenated (achieves आयु – वर्ण – स्वर – आकृति – बल – कान्ति).
- अजाक्षीर (goat's milk) should be given to drink.

केवल आमलक रसायन

After spending one year on milk-diet and surrounded by cows, one should do fasting for three days and then enter the forest of आमलकी, in the months of पूष, माघ, फाल्गुन. By chanting ॐ he should consume the fresh fruits of आमलकी.

लौहादि रसायन (आयस चूर्ण)

Red hot iron is dipped in त्रिफलाक्वाथ, गोमूत्र & क्षारोदक in sequence. चूर्ण is made and kept in घृत-smeared earthen pot inside paddy of barley (यव) for one year. After one year, it should be used with मधु according to अग्निबल.

पिप्पली वर्धमान रसायन

Intake of पिप्पली daily with गोदुध, start with 10 पिप्पली, then increase it by 10 पिप्पली/day, everyday for 10 days, then from 11th day decrease it by 10 पिप्पली/day till the consumption of total 1000 पिप्पलीs.

Increasing	Decreasing
1 st day → 10	11 th day → 90
2 nd day → 20	12 th day → 80
3 rd day → 30	13 th day → 70
4 th day → 40	14 th day → 60
5 th day → 50	15 th day → 50
6 th day → 60	16 th day → 40
7 th day → 70	17 th day → 30
8 th day → 80	18 th day → 20
9 th day → 90	19 th day → 10
10 th day → 100	
550	450
Total → 1000 पिप्पली	

Methods of Pippali Rasayana Prayoga (पिप्पली-त्रिविधप्रयोग):

1. श्रेष्ठ प्रयोग : Starting with 10 pippali, increasing & decreasing by 10-10 pippali → 1000 pippali prayoga.
2. मध्यम प्रयोग : Starting with 6 pippali, increasing & decreasing by 6-6 pippali → 600 pippali prayoga.
3. लघु प्रयोग : Starting with 3 pippali, increasing & decreasing by 3-3 pippali → 300 pippali prayoga.

Benefits of पिप्पली रसायन :

बृंहण – स्वर्य – आयुष्य – प्लीहोदरनाशन – वयस्थापन – मेध्यं etc.

शिलाजतु रसायन

- रस – अम्लरस युक्त कषायरस प्रधान
- वीर्य – नात्युष्णशीत (neither hot nor cold)
- विपाक – कटु
- कर्म – सर्वरोगनाशक (There is no disease which Shilajatu cannot cure, and when used by healthy person it promotes strength.)

4 Types of Shilajatu →

प्रकार	वर्ण	रस	वीर्य	विपाक
1. सुवर्ण शिलाजतु	रक्ताभ	मधुर तिक्त	शीत	कटु
2. रजत शिलाजतु	श्वेताभ	कटु	शीत	स्वादु
3. ताम्र शिलाजतु	मयूरकण्ठाभ	तिक्त	उष्ण	कटु
4. लौह शिलाजतु	कृष्ण	तिक्त लवण	शीत	कटु

लोह शिलाजतु is best.

All Shilajatus smell like cow's urine i.e. गोमूत्र गन्धी.

शिलाजतु रसायन प्रयोग विधि →

- शिलाजतु शोधन → Shilajatu is mixed with दुग्ध and immersed into त्रिफला क्वाथ or hot decoction of the drugs that are prescribed for alleviating the aggravated doshas. After the decoction is absorbed, decoction should be added again.
- This procedure should be repeated for 7 days.
- Use this processed Shilajatu (शुद्ध शिलाजतु) mixed with लोहभस्म with milk.
- can be used for 7 or 3 or 1 weeks as पर, मध्यम and अवर प्रयोग respectively.
- can be used in dose of 1 पल , 1/2 पल , and 1 कर्ष.

अपथ्य in शिलाजतु प्रयोग →

Avoid तीक्ष्ण गुरु अहार, and especially कुलत्थ while taking Shilajatu, because कुलत्थ has the quality of अश्मनो भेदन (cutting & breaking of stone).

Rabin Singh

त्रिफला रसायन

Acharya Charaka has explained following four rasayana prayoga of Triphala:

(I) त्रिफला रसायन-1:

Along with honey and ghee, a person should take: 1 हरीतकी after digestion of previous meal; 2 विभीतकी before food; and 4 आमलकी after food.

(II) त्रिफला रसायन-2:

A new iron vessel should be pasted with the Triphala paste (kalka) for 24 hours. This paste is administered with honey and water. After its digestion, one should take food with ghrita and sneha.

If it is administered for one year, the person can live one hundred years free from aging and diseases.

(III) त्रिफला रसायन-3:

Using Triphala churna with Yashtimadhu, Vamshalochana, Pippali, Madhu, Ghrita and Mishri is effective anti-aging rasayana.

(IV) त्रिफला रसायन-4:

Triphala along with Sarva Loha bhasma, Suvama bhasma, Vacha, Vidanga, Pippali, Lavana, Madhu, and Ghrita is administered continuously for one year. It provides health, intellect, strength and longevity.

Other important Rasayanas →

- धात्री रसायन
- अश्वगन्धा रसायन
- अमृतप्राश घृत
- अगस्त्य हरीतकी
- अर्जुन क्षीरपाक
- रसोन क्षीर
- सारस्वतारिष्ट
- शंखपुष्पी सिरप
- गंधक रसायन
- सहस्र पुटी अभ्रक

Antioxidant action of Rasayana

Rasayana drugs act as anti-oxidants. Antioxidants are the substances that reduce oxidative damage that caused by free radicals. Well known antioxidants include a number of enzymes and other substances such as vitamin C, vitamin e and beta-carotene that are capable of counteracting the damaging effect to oxidation.

A recent study shows that people whose diets had highest level of vitamin-E were 30% less likely to develop type-2 DM. In addition, researchers found that people who ate a lot of carotenoids, a type of antioxidant found in colourful fruits and vegetables, also had a lower risk of developing type-2 DM.

Oxidative damage to cells is also important phenomenon in aging process. So antioxidants are also helpful in delaying aging.

Antioxidants may possibly reduce the risk of cancer and age related macular degeneration.

Many rasayana drugs like आमलकी, अश्वगन्धा, गुडूची etc. act as antioxidants. Main constituents of आमलकी are vitamin C, carotene, and riboflavin. It is having a role in cellular oxidation reduction. They have also a role in collagen fibrin synthesis, helps in formation of fibrin; Absorption of iron. Withanolide in अश्वगन्धा also acts as antioxidant by increasing levels of three natural antioxidants like – dismutase, catalase, glutathione peroxidase.

Immunomodulatory action of Rasayana

Immunomodulator is a chemical agent (as methotrexate or azathioprine) that modifies the immune response or the functioning of the immune system (as by the stimulation of antibody formation or the inhibition of white blood cell activity).

Rasayana drugs also act as immunomodulator. Certain rasayana drugs such as गुडूची, हरिद्रा, पिप्पली, शिलाजतु etc. act as immunomodulator; strengthening of immunity is done in various ways by promoting bodily defence mechanism like increasing the WBC count, improving immune function.

In an in vitro study on immune competent cells, aqueous extract of the stems of गुडूची was found enhancing T as well as B cells responses and protected mice against experimental infections. (Sainis et al, 1998).

Administration of ब्रह्म रसायन, अमृतप्राश रसायन, and नरसिंहा रसायन were found to enhance the proliferation of lymphocytes in response to mitogens (Pradeep kumar et al, 1995)

वाजीकरण (Aphrodisiacs)

Derivation →

“वाजी अश्व” The word ‘Vaji’ is a synonym of horse (which is known for high physical strength and sexual power)

“वाजः शुक्रम्” Vaja means shukra, person who is having shukra (semen) is known as ‘vaji’, and who is not possessing it is called ‘avaji’.

Definition of Vajikarana →

अपत्यसन्तानकरं यत्सद्यः सम्प्रहर्षणम् ।
वाजीवातिबलो येत् यात्यप्रतिहितः स्त्रियः ॥

The medicine which makes the man able to have children and continue his generations; the medicine which induces sexual desire and pleasure; and by using which the man copulate with woman like a horse is called Vajeeekarana.

येन नारीषु सामर्थ्यं वाजीवल्लभते नरः ।
व्रजेच्चाभ्यधिकं येन् वाजीकरणमेव च ॥

The drugs and foods which make a man capable of performing sex many times, like a horse, are called ‘Vajeeekarana’.

वाजीकरण पर्याय (Synonyms) →

वाजीकर – वृष्य – कामोत्तेजक – पुंसत्व प्रद – शुक्र बल प्रद

- वाजीकर – that which makes the man powerful like a horse
- वृष्य (शुक्र वर्धक) – increases the quantity and quality of semen
- कामोत्तेजक – sexual stimulant – enhances libido in man or woman
- पुंसत्व प्रद – enhances sexual competence
- शुक्र बल प्रद – increases shukra (semen) and bala (strength)

Rabin Singh

Aim (प्रयोजन) of वाजीकरण सेवन → (or Benefits)

वाजीकरणमन्विच्छेत् पुरुषो नित्यमात्मवान् ।

तदायत्तौ हि धर्मार्थौ प्रीतिश्च यश एव च ॥

पुत्रस्यायतनं ह्येतद्गुणाश्चैते सुताश्रयाः । (च.चि.2/1/3-4)

According to Charaka, one should use वाजीकरण (aphrodisiacs) for →

→ Achieving धर्म, अर्थ (wealth), प्रीति (pleasure), & यश (fame).

→ Producing a good progeny (सन्तानप्राप्ति).

According to Sushruta – the three benefits attributed to vajikarana →

→ प्रीति – Pleasure, or affection to women

→ अपत्य – Producing offspring (children)

→ बलप्रद – Strength in the body

Importance of fertility →

निःसन्तान पुरुष निन्दनीय : A person with no progeny (without child) is useless like a tree with no fruits, having one branch with no shadow. A man without child is like an idol made of grass wearing the grad of a man, he is like a lamp in sketches (paintings), like gold without any qualities of gold, and like a dried lake.

A person, who has many children (बहुप्रजः), is having बहुमूर्ति, बहुमुख, बहुव्यूह, बहुक्रिया, बहुचक्षु, बहुज्ञान and बहु आत्मा. He is मंगली (auspicious), यशवान (praise-worthy) धन्य (blessed), वीर्यवान and बहुशाखा युक्त.

प्रशस्त फलप्रद शुक्रगुण (Qualities of semen that is capable of reproduction) →

बहलं मधुरं स्निग्धमविसं गुरु पिच्छिलम् ।

शुक्लं बहु च यच्छुक्रं फलवत्तद् असंशयम् ॥

1. बहल – thick,
2. मधुर – sweet (by inference),
3. स्निग्ध – unctuous,
4. अविसं – not having foul smell,
5. गुरु – heavy,
6. पिच्छिल – slimy,
7. शुक्ल – white,
8. बहु – profuse (in sufficient quantity)

Classification of Vajikarana dravyas →

Broadly vajikarana dravyas or the vajikarana therapy is of four types →

1. शुक्र जनक (शुक्रल) – That increases semen
e.g. अश्वगन्धा, मुसली, शतावरी, शर्करा, शिलाजतु, कपिकच्छू etc.
2. शुक्र प्रवर्तक – That enables flow of semen
e.g. स्त्रीस्पर्श, संकल्प, मदिरा, अहिफेन, अग्निजार (अम्बर), अकरकरा etc.
3. शुक्र-स्रुति वृद्धिकर – That increases ejaculation
e.g. माष, भल्लातक, कपिकच्छू (कौंच बीज), क्षीर etc.
4. शुक्र स्तम्भक – That causes prolonged erection of penis
e.g. जातिफल (जावित्री), अहिफेन (अफीम), केशर, अकरकरा etc.

Indications (रसायन योग्य) →

- जितेन्द्रिय (self-controlled, or who can control his sense organs)
- अपत्यार्थी (who wants good progeny)
- काम सुखार्थी
- उपभोग सुखार्थी
- वीर्य वर्धनार्थी
- पुत्रार्थी
- अश्ववत् मैथुनार्थी
- बल-वर्ण-स्वरार्थी
- क्लैब्य
- अल्प वीर्य
- विलास पुरुष
- धनवान
- रूपवान
- शोधित
- Between 16 to 70 years of age

Contraindications (रसायन अयोग्य) →

- Below 16 years of age (बाल)
- Above 70 years of age (वृद्ध)
- चिन्तित
- व्याधित (रोगी)
- उपवासित
- अतिव्यवाय श्रान्त
- दुरात्मा
- अविवाहित
- ब्रह्मचारी
- साधु-संत-तपस्वी
- अशोधित

वाजीकरण सेवन-विधि (Procedure) →

- शोधन is essential before Vajikarana therapy is administered.
- पूर्वकर्म – Depending upon the strength of the person, Shodhana therapies should be administered, before the commencement of Vajikarana-chikitsa, like a dirty cloth doesn't get properly coloured even if it is dyed, similarly in an uncleaned body the aphrodisiac remedies do not produce the desired effects.
- प्रधानकर्म – After Shodhana, with consideration of age, strength, prakriti and needs of the individual person, a suitable vajikarana drug or formulation should be administered with suitable anupana and pathya for a certain duration.

And the person should be observed carefully for the effects and adverse effects.

→ पश्चात्कर्म – The person, after therapy should be exposed to वाजीकरण भाव like beautiful parks, ponds, mountains, pleasing women, scents, flavours, garlands, friendly companions (वाजीकरण मित्र-मण्डली), body-massage (अभ्यंग), bathing (स्नान), wearing clean and perfumed clothes etc., and he should follow पथ्य-अपथ्य.

वाजीकरण आचार विचार →

A person gets vigor, and he becomes fit for sexual intercourse, if he is having following आचार-विचार →

- नित्य घृत-क्षीर अशन – consumes ghee and milk regularly
- निर्भीक – free from fear (i.e. courageous)
- निर्व्याधि – free from any disease (i.e. healthy)
- युवा – youthful (i.e. who is young, or living a youthful life by rasayana-vajkarana and pathya ahara-vihara sevana)
- संकल्प प्रवण – willing to have sex (i.e. who has proper libido)

पथ्य & अपथ्य → (During Vajikarana Therapy)

पथ्य (Beneficial) →

आहार → गोघृत – गोदुग्ध – दुग्ध विकार – षष्टिक शालि – गोधूम – यव – सक्तू – मिष्ठान – पक्व फल – फल स्वरस – शुष्क फल (मृद्विका-अक्षोट-बादाम etc.) – खर्जूर – मधुर रस प्रधान द्रव्य – हरित शाक – जांगल मांसरस etc.

विहार → वाजीकरण मित्र मण्डली – वाजीकर स्त्रीस्पर्श – मनोऽनुकूल विहार – मनोऽनुकूल गीत वाद्य यन्त्र – अभ्यंग – स्नान – स्वच्छ & सुगन्धित वस्त्र धारण etc.

अपथ्य (Harmful) →

आहार → अति कटु – अति अम्ल – अति लवण – रुक्ष अन्न सेवन – गुड विकार – असात्म्य विरुद्ध आहार – अनशन (उपवास) etc.

विहार → अतिव्यवाय – अयोनि मैथुन – दिवास्वप्न – चिन्ता – शोक – भय etc.

स्त्री सर्वप्रधान वाजीकरण →

A beautiful and youthful woman is the best aphrodisiac.

वृष्यतमा स्त्री → (Woman suitable for sexual intercourse)

अतुल्यगोत्रा वृष्या च प्रहृष्टा निरुपद्रवाम् ।
शुद्धस्नाता व्रजेन्नारीमपत्यार्थी निरामयः ॥ (च.चि.2/1/15)

A person who is healthy and who desires to have child should copulate with a woman who is →

- अतुल्यगोत्रा – of a different clan
- वृष्या – sexually attractive
- प्रहृष्ट – excited
- निरुपद्रव – free from any disease
- शुद्धस्नाता – after her menses and / or after taking bath

लक्षण of वृष्या (वृष्यतमा स्त्री) →

- सुरुपा – beautiful
- यौवनस्था – youthful
- वश्या – attractive or adorable
- शिक्षिता – educated
- नृत्य संगीत गीतादि कला प्रवीण – expert in dance, music and other arts

वाजीकरण द्रव्य (Drugs) →

आहार → दुग्ध – घृत – मांसरस – षष्टिक शालि – गोधूम – सक्तू – माष – लशुन – पलाण्डु – कूष्माण्ड – हरिद्रा – दाडिम – मृद्विका – अक्षोट – खर्जूर – बादाम etc.

विहार → वाजीकरण मित्र मण्डली – वाजीकरण आचार विचार – स्त्रीस्पर्श – मनोऽनुकूल विहार – मनोऽनुकूल गीतवाद्ययन्त्र – अभ्यंग – स्नान – स्वच्छ & सुगन्धित वस्त्र धारण etc.

औषध → कपिकच्छू – अश्वगन्धा – शतावरी – मुसली – कोकिलाक्ष – गोक्षुर – पुनर्नवा – विदारीकन्द – भल्लातक – केशर – शुद्ध शिलाजतु – स्वर्ण भस्म – अश्वक भस्म etc.

वाजीकरण योग (Formulations) →

बृंहणी गुटिका

Ingredients:

क्वाथ द्रव्य : 3-3 पल each शरमूल – इक्षुमूल – काण्डेशु – इक्षुवालिका – शतावरी – क्षीरकाकोली – काकोली – विदारीकन्द – कण्टकारी – जीवन्ती – जीवक – ऋषभक – मेदा – वीरा – बला – ऋद्धि – गोक्षुर – रास्ना – आत्मगुप्ता (कपिकच्छू) – पुनर्नवा & 1 आढक माष (उडद) & 1 द्रोण जल

कल्क द्रव्य : 2-2 पल each यष्टिमधु – द्राक्षा – आत्मगुप्ता (कपिकच्छू) – शतावरी – पिप्पली – खर्जूर – आमलकी

प्रक्षेपक द्रव्य : शर्करा – वंशलोचन – पिप्पली – मरिच – त्वक- एला – नागकेशर

Method of preparation:

The kwatha and kalka are cooked with 1 आढक दुग्ध & 1 द्रोण घृत as per घृतपाक विधि. Then प्रक्षेपक द्रव्य are added and गुटिका are prepared.

वाजीकरण पिण्ड रस

Ingredients: शर्करा – माष – वंशलोचन चूर्ण – गोधूम चूर्ण (wheat flour) – दुग्ध – घृत

Method of preparation: उत्कारिका (पुआ) is prepared of above ingredients.

These are cut into pieces, एलादि चतुर्जात द्रव्य are sprinkled on these pieces, and then squeezed and mixed with कुक्कुट मधुर मांसरस. This is known as वाजीकरण पिण्ड रस.

In the same way, other पिण्ड रस can be prepared with मयूर, हंस or तित्तिर मांसरस.

Benefits: It promotes बल & वर्ण. Man becomes exceedingly excited.

वाजीकरण घृत

2.56 kg each of newly harvested माष and कपिकच्छू (Mucuna pruriens), and 160 gm each of जीवक (substitute Pueraria tuberosa) – ऋषभक (substitute Pueraria tuberosa) – वीरा (Aloe vera) – मेदा – ऋद्धि – शतावरी – मधुक – अश्वगन्धा are added in water and boiled to prepare decoction.

640 gm Ghee – 6.4 Litre Milk – 640 ml juices each of विदारी and इक्षु are added and the mix is cooked on mild fire. This Ghee preparation should be mixed with 160 gm each of शर्करा – मधु – वंशलोचन – पिप्पली.

The dose is 40 gm before meal.

वृष्य पिप्पली योग

This product is prepared by frying 30 पिप्पली fruits in 40 gm each of तिल तैल (Sesamum) and घृत (clarified butter), pulverizing and then mixing with Sugar and Honey. The food should consist of Shashtik rice and Ghee.

वृष्य पायस योग

This is good aphrodisiac. The rice cooked with milk is called पायस. The माष (black gram) and षष्टिक शालि (rice) are cooked in the decoction of गोक्षुर (Tribulus terrestris), and विदारी (Pueraria tuberosa), with four times milk and plenty of ghee

वृष्य मधूक योग

One who uses 10 gm मधूक (Madhuka indica) powder mixed with घृत, मधु and क्षीर becomes continuously impelled with sex urge.

वृष्य रस

1. चटक (गौरेया) मांस cooked in तित्तिर मांसरस
2. तित्तिर मांस cooked in कुक्कुट मांसरस
3. कुक्कुट मांस cooked in मयूर मांसरस
4. मयूर मांस cooked in हंस मांसरस

These मांसरस (meat soup) are again fried with घृत and अम्ल द्रव्य, मधुर द्रव्य, एला, or शर्करा are added as per taste.

अपत्यकरी षष्टिकादि गुटिका

- The षष्टिक शालि is soaked in milk, then crushed and grounded with milk. The paste is squeezed in a cloth to collect its juice.
- To this juice, गोदुग्ध and आत्मगुप्ता बीज चूर्ण are added and boiled.
- While boiling, the decoctions (क्वाथ) of माष, बला, मुद्गपर्णी, माषपर्णी, जीवन्ती, जीवक, ऋद्धि, ऋषभक, काकोली, गोक्षुर, यष्टिमधु, शतावरी, विदारीकन्द, द्राक्षा and खर्जूर are added.
- At the end of boiling, powders (चूर्ण) of वंशलोचन, माष, षष्टिक शालि, and गोधूम are added to make it semisolid.
- Add मधु and शर्करा to make it sweet.
- Pills (गुटिका) are prepared and fried with घृत.

Dose: Depending upon अग्निबल.

Benefits: Even an old man becomes capable of procreating many children and he does not get exhausted during sexual intercourse.

वृष्य क्षीर

Ingredients: क्वाथ द्रव्य : 1-1 पल each खर्जूर मस्तक – खर्जूर – क्षीरकाकोली – शतावरी – मधूक – मृद्विका – कपिकच्छू & 1 आढक जल

Method of preparation: Decoction is prepared by boiling above ingredients till 1/4th part is remaining. 1 प्रस्थ क्षीर (milk) is added and boiled again till the milk part remains.

Use: This वृष्य क्षीर should be taken with षष्टिक शालि mixed with घृत.

वृष्य घृत

The कल्क of जीवक, ऋषभक, मेदा, जीवन्ती, यष्टिमधु, खर्जूर, द्राक्षा, पिप्पली, शुण्ठी, शृंगाटक, and विदारी is cooked with 4 parts of घृत, 4 parts of दुग्ध, and 16 parts of जल till only घृत remains.

This वृष्य घृत is mixed with ¼ part मधु & शर्करा.

वृष्य शतावरी घृत

1 कुडव शतावरी कल्क is cooked with 1 प्रस्थ गोघृत and 10 प्रस्थ गोदुग्ध till only घृत remains. This should be taken with मधु, शर्करा and पिप्पली.

माष

Black gram is one of the best वाजीकरण द्रव्य (aphrodisiac drugs). It can either be used directly, or indirectly, by drinking the milk of the cow fed on black-gram leaves.

माषादि पूपलिका

The flours of माष, गोधूम, शालि, षष्टिक शालि and powders of कपिकच्छू, विदारी, इक्षु and शर्करा (मिश्री) are mixed and kneaded with milk to make dough for making pancake balls. These pancake balls are rolled in to pancakes and fried in Ghee to make Poopalika. These poopluka are to be taken with milk.

Other important Vajikarana yogas →

- वानरि गुटिका – कपिकच्छू, शर्करा
- मकरध्वज रस – पारद, गंधक, स्वर्ण
- कामेश्वर मोदक – अकरकरा, पारसिक यवानी, धतूरे बीज
- अश्वगन्धादि चूर्ण – अश्वगन्धा, विदारी, गोक्षुर
- अमृत भल्लातक – भल्लातक, त्रिफला, त्रिकटु
- मूसलीपाक – मूसली, शुण्ठी, शतावरी
- केशरपाक – केशर, त्रिकटु, अकरकरा, गोक्षुर
- शुद्ध शिलाजतु (शिलाजीत)